Sun Pharma Adv. Res (SPARC)

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P. J. Towers,
Dalal Street,
Mumbai - 400 001.

Ref: Scrip Code: NSE: SPARC; BSE: 532872

Dear Sir/Madam,

Sub: Transcript of the Investor Presentation for Update on Clinical Programs and R&D Pipeline

Further to our letter s dated September 29, 2022 and October 13, 2022 bearing reference no s. SPARC/Sec/SE/20 22-23/056 and SPARC/Sec/SE/2022 -23/060 respectively , please note that the Investor Presentation was made as per the scheduled time. Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, w e hereby share the copy of the Transcript of the said Investor Presentation. The same will also be made available on the website of the Company.

This is for your information and dissemination.

Yours faithfully,

For Sun Pharma Advanced Research Company Ltd.

Dinesh Lahoti
Company Secretary and Compliance Officer
ICSI Membership No. A22471

Encl: As above

"Sun Pharma Advanced Research Company Ltd. (SPARC)
Update on Clinical Programs and R&D Pipeline Conference
Call"

October 13, 2022

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ANAGEMENT : MR. ANIL RAGHAVAN – CHIEF EXECUTIVE OFFICER
DR. SIU-LONG YAO – HEAD, CLINICAL DEVELOPMENT
DR. NITIN DAMLE – CHIEF INNOVATION OFFICER
DR. VIKRAM RAMANATHAN – HEAD, TRANSLATIONAL DEVELOPMENT
MR. CHETAN RAJPARA – CHIEF FINANCIAL OFFICER
MR. JAYDEEP ISSRANI – SENIOR GENERAL MANAGER , BUSINESS DEVELOPMENT

Moderator: Ladies and gentlemen, good day, and welcome to SPARC' s Update on Clinical Programs and R&D Pipeline . As a reminder, all participant lines will be in the listen - only mode , and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call , please signal an operator by pressing '*' then '0' on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Jaydeep Issrani. Thank you , and over to you, sir .

Jaydeep Issrani: Good evening , ladies and gentlemen. I am Jaydeep Issrani . On behalf of SPARC, I welcome you all to SPARC' s Annual Update on Clinical Programs and R&D Pipeline .

We have our CEO , Mr. Anil Raghavan and members of the SPARC 's senior team on the call today.

I hope that you have received the presentation that was sent out some time ago. The slides are also available on our website , www.sparc.life . The present ation will be similar to what we have been following in the previous versions , i.e. will walk you through the presentation, and then open the call for questions.

Before I hand it over, I want to mention and remind you all that our discussion today includes forward -looking statements that are subject to risk s associated with our business that may cause actual results to differ from those projected in the presentation.

I will now hand over the call to our CEO, Mr. Anil Raghavan. Over to you , Anil .

Anil Raghav an: Thank you , Jaydeep , for the introduction and the opening comments . Good morning and good afternoon, everybody, if you're joining from the US or Europe . A very warm welcome to the 12th edition of our Pipeline to engage and make this such a special event on our calendar. So, welcome back to this important call a nd thank you for your time.

As Jaydeep mentioned, we have the management team on the call today. I n the interest of time though we don't plan to do detailed introductions.

Slide #3 has the agenda for the day. We will start with brief comments on our strategy and some important program update s, including a brief snapshot of the portfolio

performance and the listing of upcoming catalysts. Subsequently, we will dedicate most of our presentation today to provide additional color on six opportunities with four NCE clinical programs plus one important biologics platform with IND visibility and a new first -in-class N CE asset in dermatology.

Dr. S iu-Long Yao, who h eads our clinical development , will cover the clinical projects , Dr. Nitin Damle, our Chief Innovation Officer , will provide an update on the biologics platform and the MUC -1 ADC , and Dr. Vikram Ram anathan , who h eads our Translational Development , will speak to us about an interesting collaboration with Johns Hopkins University on a potentially first -in-class intervention in alopecia areata which is the autoimmune form of hair loss. We will conclude the presentation with brief comments from our CFO, Chetan Rajpara, on our financial performance and cash situation.

So, with that, let's start . I'm going to start with Slide # 4, which is a ni ce set of numbers summarizing the state of our business. This is meant more of a scorecard reflecting the current outcomes of our journey, navigating different shades of risk starting with our 505(b)(2) days to building an organization which can now take real bet s on new targets and complex modalities. Please allow me to take a few minutes to go over these numbers .

On the commercial or close to market end of our portfolio, we have five assets .

Elepsia, which is a high dose levetiracetam pill is commercialized through a US CNS specialty company called Tripoint Therapeutic s.

And Xelpros, which is our BAK -free formulation of a first-line glaucoma drug, latanoprost , is with Sun Pharma in the US.

Two more NDAs are under review and another one is in planning. First one , phenobarbital benzyl alcohol - and propylene g lycol -free formulation is under review with FDA with this PDUFA date coming up next month.

And the second one , brimonidine once-a-day eye drop goes by the project name PDP-716 was submitted recently. P DP-716 and ophthalmic steroid SDN -037, are both licensed to Visiox Pharma as we disclosed earlier.

These programs which are either in the market or have had successful clinical outcomes in the late stage trials, go a long way in validating the SPARC model. In

addition to the cash, these experiences did offer substantial learning in terms of building com petencies to conceive and convert full development program.
The second pillar on the slide shows six ongoing clinical trials leveraging three NC Es. These assets offer really significant commercial opportunities, particularly vodobatinib in Parkinson’s Dise ase and more broadly across other neurodegenerative conditions . Vibozilimod in dermatology is in a spectrum of autoimmune disorder s.
The primary development space for the CNS leg of v odobatinib centers on a bunch of diseases driven by the neuronal toxicit y of aggregated alpha s ynuclein. Parkinson's disease is the largest slice of that pie in terms of prevalence . Lewy Body Dementia or LBD, and Multi System Atrophy or MSA , are the other two components of the spectrum . LBD is quite a substantial disease in terms of the total disease burden, and while MSA is not a large patient pool, the unmet need is quite high. It doesn't have an appropriate standard of care currently.
Vodobatinib can offer a potentially disease modifying intervention across alpha synucleinopathies once we establish a clinical proof -of-concept , which is very consequential for patients across the globe with these debilitating conditions.
On vibozilimod , the next generation of S1PR 1 agonist are becoming class alternative to JAK inhibitors in dermatology after the re cent FDA safety warning for the J AK as a class . That was quite an important development, and may be one of the primary drivers of Pfizer buying Arena Pharmaceuticals . We are among the few players developing S 1Ps in dermatology with A rena and BMS being the other two.
On the third column is on our preclinical effort . We intend to bring two more projects to clinic in the next 18 months . Alopecia Areata program later this year if everything goes on plan and MUC- 1 ADC next year. But more importantly, we have seen our operating capabilities and external collaboration mature to a point where we feel comfortable taking early stage risk.
I want to leave you with the following thoughts on this slide. SPARC has validated its operating model with a significant number of assets across commercial , late , and mid-state clinical development , and early stage programs . So, in that sense, SPARC offer s one of the more validated translational bridges to navigate the opportunity to innovate from India for patients across the world. And more importantly, we are

approaching some meaningful catalysts in the next 12 -to-18-months for our stock and that's what we intend to cover in the rest of the deck.
I am going to use the next couple of slides to give some additional color on a few points I just made .
Slide #5 please. On capital efficiency , as a matter of strategy, we went after lower incremental innovation initially . A positive outcome from that approach was the non -dilutive capital it has generated to fund more complex science and high- value opportunities we built in the last few years . As you can s

ee in this graph here, SPARC
has spent upwards of $500 million up to the end of FY '22 running our operations in the last 15- years and building what we have today.
Fresh equity rights issue s in 2012 and 2016 contributed around $79 million in t otal, while preferential issues brought in the rest. In this perio d, we invested around $277 million from our internal revenue accruals .
Two things are worth noting besides the substantial non- dilut ive cash flows used to create the model and the portfolio build- out. These cash flows are primarily coming from our 505(b)(2) products plus some platform licensing transactions and some service revenues in addition to one upfront payment on vibozilimod . The economics of all the N CE or NBE programs are fully retained with in SPARC , with the exception of vibozilimod . So, the non- dilutive funding mentioned in this slide happened without diluting the more promising programs.
Secondly, our journey has been marked by substantial promoter commitment . The lion's share of fresh equity infusion mentioned here in this slide for SPAR C has come from our promoter group , and that's a huge show of confidence on the SPARC story and a demonstration of commitment to the vision.
Now, Slide # 6. We've been talking about our portfolio pivot from incremental to more riskier NCE opportunities in the past few years , with the rationale and nature of that

transformation many times . Early on that pivot, we focused on validated targets like BCR-ABL in chronic myelogenous leukemia and S 1PR1 agonism in autoimmunity. But we've now moved to the next phase in that evolution with increased confidence in taking early -stage risk position in novel biology . These numbers on the chart here are worth taking note of. A lmost 60% of our portfolio now has first-in-class potential. The

composition of the portfolio from a modality mix standpoint continue s to evolve. Almost 1/5 of our programs now are outside of the traditional , straightforward new chemistry , and that component is growing faster. They either represent complex modalities like antibody dru g conjugates, or other conjugated entities of bio logics . This transformation wouldn't have been possible without our robust external innovation initiative. Most of our ideas involve substantial partnering components either as traditional development collab orations or for sourcing important development competencies, which are difficult to find or build in India . In this process , our own identity has shifted quite a bit towards being a translational engine which looks to access exciting science and differentiating competencies from wherever we can find them from groups across the world.

And secondly, the confidence in our portfolio decision- making has evolved quite a bit. Some of that is driven by the discipline d portf olio review process that we have followed to take Go/No -Go decision s, and some of it is driven by the changing emphasis on certain parts of the process i tself like the identification of key experiments which can give us early signals of proof -of-mechanism, or early signs of potential safety and efficacy, or a conscious decision to invest substantially in largish reproducible global trials in the Phase -II stage itself .

While we are still very much a work in progress, it's important to say that we'v e taken deliberate steps to mitigate the risk as much as possible, while embracing early stage risk as a matter of business reality that we're dealing with. A large part of that process is our relationship with external advisors, and the space that we prov ide to them to input into our decision -making process.

So, with that, let's go to Slide # 7 for a few examples . I want to spend some time on three programs here, vodobatinib in PD , ADC using antibodies a gainst MUC -1 alpha

/beta junction , and SCD -153 for a novel immunological target to treat alopecia areata.

Vodobatinib is probably the first seri ous effort to track the oxidative stress response in neurodegenerative diseases. The toxic cascade of event s initiated by A bl-mediated oxidative stress response is affected through a complex web of interrelated events involving on one side , the aggregation of intercellular alpha synuclein and on the other side compromis ing of multiple protein clearance pathway .

Our decision to move to a clinical proof -of-concept study was driven by critical pieces of evidence for the impact of disrupting this cascade with a highly potent, but s uper - selective c -Abl in hibitor gathered through several important experiments done at some of the best labs in this area globally, like Dr. Ted Dawson at Johns Hopkins University , or the Ann Romney Neurology Institute at Brigham and Women’s Hospital, or Atuka, Inc. in Canada, in addition to of course our own internal experiments . So, the t otality of evidence coming from these experiments g ave us the confidence to move to clinic. Our initial focus in the clinical study was to establish an appropriate clinical dose, meeting the brain exposure targets indicated by the preclinical program and confirm the safety of the proposed doses. Once we achieved that, we moved to a clinical proof -of-concept study, that's large and global enough to provide a comprehensive and reproducible proof -of-concept for the mechanism. So, that's what I was talking about de -risking early -stage risk taking through a deliberate translation al framework.

Moving on to the second project here on the slide, here, again, you can see several parts of the same strategy at play. MUC -1 has been a high interest cancer target for ADC and other tumor -targeting programs because of its high tumor -specific expression. Our strateg y was built on fo ur antibodies from Biomodifying against a set of unique α/β junctional epi tope s looks to avoid the trap of peripheral floating MUC - 1 which led to failure of earlier attempt s. So, that gives us the differentiated opportunity to deliver high potency cytotoxic payloads and other targeted agents using the ADC construct.

On a separate note, we are very excited about ADCs as a modality, which ha s clearly become mainstream on the back of clinical successes of product s like Trodelvy and Enhertu recently . These successes, and several others have demonstrated the stability and scalability of clinical system, and safety windows for the commonly deployed payload. In fact, substantial activity using clinical ADC framework, as well as this novel construct involving target ed agents or immunological intervention and MUC -1 program and other programs in preclinical mix , give us an opportunity for unique targeting in oncology.

And finally, on this slide on the right column SC D-153, which we are disclosing for the first time here. SCD-153 is being evaluated through an option- to-license agreement

with a highly accomplished group at Johns Hopkins and leveraging a compound with significant anti -inflammatory propert y.

As you will see later on in Vikram slides, we have developed substantial data and are in the process of establishing a topical formulation with appropriate safety m argin . We are hopefully headed towards an IND in the early part of 2023.

In both these preclinical programs, you can see key elements of a model playing out in terms of an attempt to leverage novel biological insights, willingness to learn from external growth, and robust early translational work to set up clinical programs appropriately. We believe that 's the right approach and offers the responsible path towards growing the portfolio wi

th potentially attractive and clinically relevant asset s.

So, let me now pivot to certain near -term cons ideration before I hand over the call for progress and that 's cash runway on Slide # 8. We have an expected spend of around $60 million in a year . Numbers for the outer y ears as you can imagine are estimates, but we are more or less in the ballpark here. In terms of cash flo w, we expect warrant conversions of up to $9 3 million by end of December 2022. In addition, we will have certain milestones and royalties from existing and upcoming commercial products and technology licenses . Without taking into account royalties and mile stones, we have a cash runway that will take us to the end of next financial year FY '24.

Slide #9 please. Now, let me spend few minutes on the slide to give you some ideas around Sezaby which is our benzyl alcohol -free phenobarbital formulations for treatment of neonatal seizures . Starting with a little more context before covering project status on the slide . The currently market ed product s did not go through the current approval process requiring a clear demonstration of safety and efficacy as they' re in medical practice before the current process was established . FDA formulated a plan to sunset such unapproved products if a company obtains an approval through the due process. The current policy therefore aims to remove all unapproved products from t he market w hen such approval s are given on the basis of fresh evidence presented . The policy also offers certain market exclusivity benefit s to incentivize companies to generate new data.

Phenobarbital injectable is an unapproved product and is primarily used for treating newborns experiencing seizure episode s. In addition, current unapproved phenobarbital formulations contain benzyl alcohol or propylene glycol, which many regulatory agencies across the globe removed because of the potential toxicity .

We filed a new drug application for phenobarbital in February 2022. The FDA has granted a priority review and pediatric rare disease designation, which i f confirmed can offer seven years of market exclusivity. If approved, the SPARC product can become potentially the first approved phenobarbital, which can enjoy the associated market exclusivity , pending certain agency determinations subsequent to the approval. SPARC may be eligible for certain other incentives associated with it, depending on the outco me of the ongoing review. We are in advanced stages of evaluating our options for commercialization of this product in the US. The PDUFA data as I mentioned is in November 20 22. We hope to become an exclusive provider of the first approved formulation of p henobarbital in the US. We 're looking forward

to working with an appropriate commercial partner to grow the franchise
responsibly. We can come back to this in more detail if you have questions during our
Q&A.
Now, let's spend a few minutes on Slide # 10 to go over the upcoming catalyst s for the
company . The first column discusses the commercial or l ate stage clinical program .
We had an excellent start for Elepsia with Tripoint , though, these are still in very early
days. The early performance gives a s ignificant confiden ce to scale up for commercial
success for Elepsia.
Xelpros is our second product in this category, which is with Sun Pharma in the US.
We are working very closely with the Sun team to continue to build the franchise.
In addition, we expect to see commercialization of Sezaby a bit later in this financial
year and brimonidine or PDP -716 in the next financial year. We're still discussing the
NDA submission for SD N-037 with Visiox .
So, on the commercial side, we have a few levers which we can use to generate
additional cash from royalties and regulatory milestone payments in the short

-to-
medium term.

On the clinical side, vodobatinib CML program which is in a pivotal study is actively
recruiting now, tho ugh slower than we would like , given the rare indication that we're
pursuing . We are seeing excellent efficacy for protocol eligible patients in the study,
Siu will go over this in a bit . We are looking to file NDA in 2 024.
PROSEEK is our Phase -2 PD trial for vodobatinib and we have crossed the 70%
recruitment mark a few weeks back, and we are pushing forward on all cylinders to
comp lete the accrual by the end of this financial year. And that sets up an important
data even t in the coming financial year. Vodobatinib as you know is also in a single
center study in Lewy Body Dementia at Georgetown University , which is expected to
read out around the same time.
Vibozilimod is in two Phase -2 trials in atopic dermatitis and psoriasis. We're r amping
up the site infrastruct ure significantly with additional sites in Europe and Latin
America. Finally , SCO-120 is in Phase -1 dose escalation stud y in patients . We hope to
get early efficacy signals as early as next year for the oral SERD program. We are
carefully navigating the space though given the complex nature of the SERD field right
now , and we will set up a Go/No -Go event for this program basis the quality of the
efficacy signal s we receive going into next year.
And on the preclini cal side, I have spoken in detail about S BO-154, that's a MUC -I ADC
and SCD-153 in alopecia areata, so I won’t go into more details on that , but both
projects are on track to go into INDs by financial year ' 24.
Finally, we will continue to look for additional assets for partnering . We will look for
exciting early science which synergizes with our current basket, but we will be
conscious as it has the potential to float out clinical spend going into next year and
even beyond. So, our interest would be primarily driven by strategic portfolio
considerations and not opportunism.
Now , on to Slide # 11, m y last slide in the deck. This chart is the pipeline summary. I
guess we've gone through most of this in a fair bit of detail. It offers indication , current
development status plus next set of data milestones. And we can come back to th is
again for the Q &A.

So, with that, thanks again for your time today. I will now hand over the call to Dr.
Siu-Long Yao who h eads clinical development for additional d etail on fo ur clinical
programs.

Dr. Siu -Long Yao: Thank you, Anil. Again, m y name is S iu Yao and I oversee clinical development. The
next set of slide s go over SCO-088 for the treatment of chronic myelogenous
leukemia. So, with S CO-088, we believe that we have a safe, effective option for the
treatment of heavily pretreated , last-line patients . This Slide #14 summarizes the
clinical development program for this drug. The program started with a healthy
volunteer single ascending dose and food effect study in 40 subjects and then
proceeded to a multiple ascending dose study in patients. Currently, we're in the
midst of a pivotal study involving chronic phase , accelerated phase and blast phase

CML. As noted in the first bullet in the lower right -hand part of the slide, this is last line therapy. So, patients are required to have disease that is refractory and/ or intolerant to greater than or equal to three prior tyrosine kinase inhibitors, one of which must have been ponatinib. For this stud y, we have sites in the US , Belgium and there's a host of other sites listed on the slide and you can refer to it .
The next Slide # 15 gives you some of the data we've seen from the multiple ascending dose study in patients. This slide shows the major cytog enetic response rates that we've observed, and bre

ak s it down into the status of the entering patients at baseline on the top bar, and then what happened to them following treatment on the bottom bar. As you can see, 71% of patients came into the study wit h uncontrolled disease. Only 29% had a major cytogenetic response at baseline . Following treatment with vodobatinib however, there was a marked increase in the proportion of patients with a major cytogenetic response a s you can see in the lower bar to over two-thirds of the patients had disease that was responding to treatment.
Next Slide # 16 shows results shows major molecular response rates following treatment. Again, the top bar represents the status of patients at baseline just prior to enrollment a nd the bottom bar represents results observed following treatment with vodobatinib . You can see that the re has been a major molecular response, which represents a three- log reduction in the number of cancer cells, has increased from 2.8% prior to treatment to 43% following treatment.

Finally, slide #17 here depicts a swimmer ’s plot that gives you a feel of what we're observing overall with vodobatinib. On the x -axis is durat ion of treatment, which is a measure of progression -free survival , and on the y- axis in each row are the results for individual patients . Patients are treated until they no longer respond and the point at which response stops is depicted by a dark circle. The duration depicted on the x -axis is substantial , or it can be substantial, with time ranging out to 65 months or almost five and a half years. A swimmer ’s plot is somewhat of an unequal assessment as it doesn't show the maximum benefit. Because people a t the top of the graph have only experienced treatment for a short period of time. So, you don't know if they're going to continue to respond like some of those at the bottom of the graph. This was a dose escalation study so the doses at the bottom of the graph are generally lower than those at the top. Nonetheless, you can see that there have been some dramatic responses, w ith some patients at the lower doses benefitting for well over five years now.
Slide #18 goes over our current plans for the program. We'll be providing additional updates at the Goldman Conference in October in France, and at the Annual Meeting of the American Society of Hematology (ASH) in New Orleans in December. We're happy to say that our abstract for the ASH Meeting has been selected for an oral presentation for the third year in a row now, and results from the pivotal study are anticipated in fiscal year '24.
This slide transitions to the use of vodobatinib for neurodegenerative disea ses. The mechanism of action here is relatively unique, I want to point that out and that we are trying to modify the disease course rather than just treat symptoms, like the existing therapies do.
Slide #20 is here to remind you about the underlying science supporting the role of Abl and neurodegenerative diseases, including Parkinson's disease. As noted in the bullets on the left, Abelson kinase is expressed in all parts of the brain and has a pivotal role in promoting neurodegeneration, specifically unde r conditions of toxic stress, Abl causes cells to die in order, for example, to rid the body of defective or malfunctioning cells .
Slide # 21 goes over results from the preformed fibril model of Parkinson's disease in mice. So, let me walk you through this slide. In this model, toxic aggregates of synuclein, a putative basis of Parkinson's disease are injected into the brain to cause

Parkinson's disease in mice. The top two graphs, summarize results from mice turning around at th e top of a pol e on the left graph, and then descending the pol e on the right graph. In each graph, tim

e is on the y- axis, and different treatments are on the
x-axis. The first two groups in each case or in each graph consists of control treatments, where d isease has not been induced in the mice.
The third co lumn in each case is the result you get when you induce disease in the mice . You can see that the amount of time required to turn on top of the pole a nd then descend the pol e increases in those third columns, because the mics are unsteady and they had disease induced in them.
When you treat with a low or high dose of v odobatinib represented by the fourth and fifth columns respectively, the time for the mice to complete the tas k decreases, and it's essentially the same as the unaffected mice represented by the first two columns in each graph. Similar results occur when you look at the strength of grip of the mice as depicted in the two graphs in orange shading at the bottom of t he slide .
On the y- axis in these graphs, you have grip strength, and going across the x -axis, you have the same treatment as before. Again, the first two columns represent unaffected mice, whereas the third column represents affected mice not given any treatment. Columns four and five in each graph represent the results from treatment with vodobatinib either a low or high dose . You can see that higher doses of vodobatinib are able to protect the mice from the insult of the preformed fibrils that cause Parkinson's disease in the mice.
So, next Slide # 22 is from another model of Parkinson's disease , but in rats. Here , a gene encoding a mutant misfolded toxic version of alpha synuclein, originally derived from a person with Parkinson's disease is expressed in the right half of the brain. The left side of the brain is left alone as a control to tell you what things would look like in an unaffected brain, that's not subject to treatment with a toxic disease inducing alpha synuclein. So , following treatments the whole brain is removed and evaluated for cell death by using a radioactive probe. Since in each case, the left side of the brain is normal, you want to compare the right side which has been treated to cause disease with the left normal side each time when you look at the graph. T he first group of left and right is a control , just to make sure that the virus used to deliver the alpha synuclein isn't itself causing some type of unanticipated effect. You can see that the left and right columns there in the fir st group ma tch so the delivery system is not

causing any real effect. The second group shows you what happens if you induce disease without any treatment. The left side is normal, and the right side, which has been treated with the t oxic synucle in shows mu ch fewer functional cells as represented by the lower bar or column.
Groups three, four and five represent increasing doses of vodobatinib that, again, you are comparing the right disease br ain in each case to the normal left brain f or each group . You can see that by the time you get to the high dose of vodobatinib shown in the rightmost group, the left and right brains are matching, suggesting that vodobatinib is essentially completely blocking the effects of the disease -inducing mutant alpha synucle in.
Now, we're on Slide # 23. And this summarizes the study design of the ongoing PROSEEK clinical study, evaluating the effect of vodobatinib in Parkinson's disease. There are three arms here, consisting of placebo and a low and high dose of vodobatinib . The t otal sample size is 504 split between the three arms, and p art one is the big study, whereas p art two is an extension where all subjects received vodobatinib . As summarized in the bullet on the right there, there are currently 77 active sites and over 70% of the study has been enrolled.
Slide 24 summarizes progress with a Lewy Body Dementia study that we're
co

llaborating on with an investigator at Georgetown University in the United States. Lewy Body Dementia is a disease that is thought to have a cause sim ilar to Parkinson's disease . So, you can see as noted in the figure and bullets, this is 45 patient study primarily designed to evaluate safety, tolerability and biomarker results in this population and we're now about halfway through the study.
This slide just summarizes the main milestones for this program. Again, there's the Phase -2 PROSEEK study and the Lewy Body Dementia study, both of which are maturing in fiscal year 2024. I'm now going to transition to vibozilimod or a selective S1PR1 agonist, which we believe will be a safer alternative to JAK inhibitors for the treatment of dermatologic disorders.
Slide # 27 and I'm going to walk you through some of the bullets here. This provides an overview of vibozilimod along with some context rega rding the therapeutic area and existing molecules. So, overall, the goal of our program is to develop an oral

standard of care agent for the treatment of dermatologic disorders. And as you may

know, the existing oral agents such as methotrexate or apremila st have limited efficacy and /or significant toxicity concerns, and they're generally unable to reach the efficacy seen with the injectable biologics. Our goal is to have a best -in-class sphingosine agonist with efficacy and safety that can approach that of the injectable biologics.

So, going through the first bullet on the left, as you may know, fingolimod was a pioneer in class and served as a proof -of-principle that targeting this pathway can be effective in autoimmune diseases. Fingolimod however has sig nificant cardiovascular limitations and sometimes even overnight inpatient monitoring can be required in some patient populations. Older drugs in the class are approved for non- dermatology indications, but they don't always provide selectivity against the S1 PR3 receptor for example, which can lead to hypertension, macular edema, shortness of breath, and even cancer. Th is and other safety concern s have largely limited the ability to explore the use of this class in dermatologic disorders.

The last bullet on the left there just reiterate s that everyone is looking for an oral agent that treat psoriasis with activity close to that of the biologic. For a while, it was hoped that the JAK inhibitors, for example, would be able to fulfill that goal , but the recent addition of black box warnings has resulted in roadblocks to those agents. Th e sphingosine agonist s, actually have a pretty extensive history of safety now, and hence people are moving to them into this space with some renewed vigor when possible. I've touched on several of the bullets on the right there . They give more details specifically about vibozilimod .

Additional thing s to know about vibozilimod include that we have clear preclinical and early clinical validation and corresponding study that we're in the midst of clinical trials in atopic dermatitis and psoriasis , and we've seen preclinical synergy with other mechanisms, such as the IL -23 pathway in inflammatory bowel disease.

Slide #28 summarizes the design of the ongoing Phase -2 study in psoriasis. In this study, 240 patients are randomized amongst three doses of vibozilimod and placebo and treated for 16 -weeks to obtain results on PASI75 , the primary endpoint for the study. This is basically depicted as a second column on the sli de.

There are a series of subsequent re -randomizations to provide insight into things like the durability of response on therapy, and the durability of response following

cessation of therapy , and these are represented by the subsequent columns in the slide. The study has just star

ted and approximately 20% of the subjects have been randomized.

Slide #29 represents the Phase -2 of the atopic dermatitis study. This study also consists of 240 patients randomized to three different doses of vibozilimod or placeb o. The primary endpoint also occurs at week -16, although , of course, it's different , it's EASI -75 instead of PASI -75 reflecting differences in their disease. This study is ongoing and involves the US, EU and Latin America.

This Slide # 30 transitions to our brain penetrant program targeting mutant estrogen receptor in the treatment of breast cancer .

Slide # 31 gives you some background and rationale for estrogen receptor antagonists program. The first bullet points out that fulvestrant , is the main Selective Estrogen Receptor Degrader available for patients fa iling first line therapy, but it's limited by the mode of administration , that is intramuscular and inactivity against mutations at clinically practical doses. Elacestrant , a newer estrogen receptor antagonist with activity against immune forms of this receptor , met its primary endpoint in a Phase -3 study for providing proof -of-concept for this approach. It's important to note however, that other similar drugs in the class have not a lways correspondingly succeeded. The success seem to be molecule dependent.

The last bullet just points out that most of these hormonal therapies are now given in combination with other treatments, but that other treatments have not usurped the role of these estrogen receptor degraders and antagonists.

Slide #32 goes over our current clinical program for SCO -120, which completed both single ascending and multiple ascending dose escalations in volunteers , and we're at/or near exposures that we expect to be efficacious at this point. So, far, the drug has been generally safe and well tolerated. In the middle of the slide there, we want to confirm the tolerability of these doses in patients and see if we can escalate a bi t more in Part 1 of a patient study that's already in progress for patients who have failed at least one prior endocrine therapy and no more than three prior chemotherapies. Our main focus will be to confirm the safety and pharmacokinetics we saw in volunt eers , in patients . Subsequently, we will focus on obtaining preliminary efficacy information and several cohorts listed at the bottom right of the

slide. These include patients with ESR1 mutations and those resistant to aromatase inhibitors, resistant to a romatase inhibitors and fulvestrant as well as those with brain metastases Finally, slide #33 is just a short summary of our plans for this program. We anticipate a readout from the ongoing study I described in the previous slide in fiscal year 2024, followed by initiation of Phase -2 study and a potential NDA submission in fiscal year 20 27.

And with that, I'd like to turn the presentation over to my colleague , Dr. Nitin Damle, who will walk you through some exceptional data with one of our upcoming biologics projects. Nitin?

Dr. Nitin Damle: Thank you very much, S iu, and good evening, everyone. M y name is Nitin Damle, and I would like to provide an update on our first biologic therapeutic program SB O-154. During our presentation to this audience a year ago, we had described S PARC's initiative to invest in biologic therapeutics and introduce our st rategy to explore antibody drug conjugates as anti -cancer therapeutics. SB O-154 is the first product of such exploration and represents an antibody drug conjugate , in which a humanized IgG1 antibody with a high affinity for MUC- I alpha /beta heterodimer, is linked to a potent cytotoxic drug that preferentially kills dividing cells when delivered to tumors via antibody drug conjugates. The cytotoxic drug used as a payload in S BO-154 is clinically and commercially validated

for clinical use in different types of cancers.

The tumor target that we have been interested in focusing on is human Mucin -1, also known as MUC -1, as shown on slide 36. M UC-1 is a glycoprotein overexpressed on the surface of a wide variety of carcinomas and had been the focus of immunothera pies over the last two decades . Most of those efforts were clinically unsuccessful in large part due to the presence of a high level of circulating MUC -1 carrying the epitope recognized by anti -MUC- 1 antibodies used in those studies. Mature MUC -1 antigen d isplayed on a cell surface as shown on this slide is a heterodimer of extracellular MUC -1 alpha and a transmembrane subunit MUC -1 beta. In this M UC-1 alpha subunit is the one that gets shed and can be found in large quantities in the sera of cancer patients. Such shed MUC -1 in circulation can intercept anti-MUC -1 antibodies used denying them the opportunity to bind to tumor cells resulting in th eir therapeutic failure.

The M UC-1 alpha / beta epitope targeting antibody used by us in creating S BO-154 was originally in -licensed as a mouse antibody and was subsequently humanized at SPARC and further covalently linked to a cytotoxic drug to create S BO-154 and used in our preclinical evaluation . One of the key reasons for the use of anti -MUC -1 antibody for intracellular drug delivery is that any antibody bound to M UC-1 is rapidly internalized, and thu s is ideally suited for intracellular delivery of potent cytotoxic agents.

The next Slide # 37 shows that red fluorescence humanized anti -MUC -1 antibody, bound to M UC-1 on the cell surface , is rapidly internalized over time and inside the tumor cells. And thus, is not recognized by green fluorescence secondary antibody outside the cells into m ice red fluorescence anti -MUC -1 antibody, continues to appe ar as red fluorescence, whereas the antibody that is still on the cell surface and has not been internalized, is able to be recognized by green fluorescence secondary antibody to create a yellow -orange fluorescence as you see in this slide. In contrast, similar red fluorescent Rituximab , targeted at human CD -20 and use d as a non - binding control antibody does not show any fluorescence, indicating its lack of tumor binding and internalization.

In th e next two slides, I would like to share with you anti -tumor efficacy of SB O-154

against high MUC -1 expressing human pancreatic carcinoma xe nografts , established
in immunodeficient mice. In this evaluation , pancreatic carcinoma x enografts were
first establ ished prior to the initiation of systemic therapy w ith MUC -1 targeted ADC
SBO-154. Similar ADC created using Rituximab was used as a non- binding control in
all evaluations.
Slide #38 shows that S BO-154 causes dose dependent inhibition of growth for the
xenografts and at the highest dose causes shrinkage of the established tumors.
We further evaluated anti -tumor efficacy of SB O-154, i n a model in which xenografts
of the same pancreatic carcinoma were allowed to grow to large tumor masses,
accounting for up t o 5% of the body weight .
As shown in slide #39, SBO-154 was able to cause regression of such pre -existing large
tumor masses of M UC-1 expressing tumors. In contrast, non -binder ADC of
Rituximab , allowed for further uninhibited growth of such large tumors. Thus SBO-
154 exhibits the ability to cause strong growth inhibition in a MUC -1 overexpressing

carcinoma model. We have undertaken detailed anti -tumor efficacy assessment of
SBO-154 in various other carcinomas that express varying levels of M UC-1. These
assessments would allow for greater appreciation for the potential of SB O-154 as an
anti-cancer biologic therapeutic.
In light of the above preclinical proo

f -of-concept and as shown in the next Slide # 40,
we have advanced SB O-154 program to the preclinical de velopment stage with the
intent to file IND in the US in in the financial year ' 24.
With this update , I would like to st op and hand over further discussion to my
colleague, Dr. Vikram Rama nathan. Vikram?
Dr. V. Ramanathan: Thank you , Nitin, and good morning, good afternoon or good evening to you all based
on where you're located.
My name is Vikram Ramanathan, and I'll give you an update on SCD -153. SCD-153 is
an NCE that we're working on with potential for use in autoimmune disease called
Alopecia Are ata. SCD-153 is a topical agent for this disease which has a significant
unmet medical need.
Slide #43 gives you some background on the disease. Alopecia Areata is an
autoimmune disease that causes loss of scalp hair and clumps, and it's a
psychologically very debilitating disease. This occurs because the hair follicles, which
are normally protected from the effects of patrolling immune cells lose their so -called
immune privilege. So , on the upper left is a diagram of a healthy human hair follicle.
At the base is the bulb of the follicle. Immune cells are present, but the bulb of the
follicle is normally immune from their effects. To the right of it , is a depiction of a
diseased hair follicle in alopecia areata. Some changes are immediately apparent . The
hair has fallen off, and there's a big swarm of immune cells present at the base of the
follicle.
The text on the right summarizes the changes in the disease. The hair follicle rapidly
progresses from growing or an agen phase to the transition or cata gen phase and then
eventually to the resting or telogen phase.
Secondly, there's a collapse of the normal immune privilege in the bulb. The culprit
CD4+ and CD 8+ T cells infiltrate the area at the base of the follicle. In particular, it's a

subset of the CD 8+ called NKG2D positive CD8+ cells that cause the damage.
However, the hair follicle structures and stem cells are preserved, suggesting that hair
growth is possible in principle if the hair follicle reverts to its original state.
Finally, the picture in the bottom left shows how the disease manifests in real life.
And this remains an unmet medical need at this time. Our S CD-153 is a topical agent
that seeks to reverse these immune changes and allow normal hair growth.
So, if you move to Slide # 44 now, you're looking at in vivo studies in a tel ogenic hair
growth model. The hair on the back of the mouse is clipped at eight and a half weeks
of age. The right side is treated with topical SCD- 153 and the left side is left untreated.
The top row shows that vehicle treatment does not result in hair growth. The images
in the second -row show that the treated area on the right side of the mouse shows
hair growth, but not the untreated left side. In the bottom row, we see the effects of
the treatment of the JAK inhibitor tofacitinib .

In summary in this model, SCD -153 stimulates robust hair growth after two doses given on alternate days . It promotes re -entry of the hair follicle into the antigen growth phase, possibly by activation of stem cells at the base of the hair follicle.
So, moving on to Slide #45 now, there is evidence that the topical SCD -153 also stimulates hair growth in an immune model of alopecia areata. C3H/HeJ mice are a strain that spontaneously develop a lopecia areata in about 20% of the cases. The figure on the left shows pictures before and after topical treatment.
The box in green shows good hair growth on the back of the treated area. The head region was not treated and as expected did not show hair grow th. On the right are images of the alopecic skins of these mice when seen under the microscope. In the untreated skin, there is a strong staining for the culprit CD8+ immune T -cells ,

and this
staining is greatly reduced in the treated skin. We also have separate evidence for reduction of inflammatory cytokines in th is treated skin.
In summary, we are working on a potential topical treatment for alopecia areata. We have separate data which we have not shown here that the compound suppresses inflammatory cytokines in vitro in cell culture, and that here we have shown you a data that show that it promotes hair growth in two animal models of disease.

Finally, slide #46 indicates that the IND enabling tox studies are underway, and we will be filing an IND soon on the completion of th ese studies in the early part of 2023. We look forward to sharing an update on this effort in the coming time.
This concludes my part of the presentation and I would like to hand over the baton to our CFO, Mr. Chetan Rajp ara for his Financial Update . Chetan?
Chetan Rajpara : Thank you Dr. Vikram. Good evening , everyone. This is Chetan Rajpura, CFO at SPARC . I plan to go over SPARC financials and cash position at a high level.
Slide #48. During FY '22, total income was at Rs. 144 crores, equal to US $19.3 million , while total expenses were at Rs. 347 crores, equal to $ 46.6 million , resulting into a net loss of Rs. 203 crores , equal to US $27.3 million . FY'22 income was lower as compared to FY'21, as previous year income included an upfront non- recurring rec eipt of US $20 million from SCD -044 licensing d eal.
Let me update you on our financial results for first quarter of FY '23. For Q1 FY'23, total income was at Rs. 29 crores, equal to US $3.7 million, while total expenses were at Rs.111 crores, equal to US $14.4 million , resulting into a net loss of Rs. 82 crores, equal to US $10.7 million .
Slide #49, as you may be aware company raised Rs. 1,112 crores , equal to US $148 million in July ' 21 by way of a preferential issue of convertible warrants . The company has already received Rs. 409 crore s, that is US $55 million , being 25% payable on application as well as conversion of warrants . The balance sum of Rs.703 crores , equivalent to US $93 million , is expected to be received by end of December ' 22 upon the conversion of all warrants b y the investors . The company has a line of credit in place for Rs. 250 crores , equivalent to US $31 million from the parent company , in addition to the bank facilities for Rs.245 crores , equivalent to US $31 million . Bank facilities for Rs. 183 crores, equal to US $23 million were utilized as on September 30, 2022, which is planned to be repaid in full before March ' 23.
The company has obtained shareholders’ approval at the last AGM for raising sum of up to Rs. 1,800 crores , that is US$225 million by way of issu ance of securities.

The company is in process of licensing the late stage clinical ass et, which will generate the additional liquidity. For FY '23, approximately , 1/3 of our expenses are budgeted for the clinical costs. We are aggressively managing our cost s and working to control our non -clinical expenses.
That's all from me today on the financial update. A big thanks to all for joining the call. I will now hand over the call to Jaydeep for facilitating the Q &A.
Jaydeep Issrani: Thank you, Chetan, and we will now open the call for Q&A.
Moderator: We will now begin the question -and-answer session. We have the first question from the line of Kunal Randeria from Nuvama . Please go ahead.
Kunal Randeria : Sir, my questions are around vibozilimod . Firstly, you are pitching this as an alternative to JAK inhibitor, right, because of safety, signals in JAK inhibitor. But recently, TYK2 inhibitor Sotyktu was approved by the FDA. It is probably the first

product without the safety warning that has limited

the use of JAK inhibitor. I am just
wondering how to see your product versus Sotyktu ?
Anil Raghavan: Thanks, Kunal, for that question. I think you may be referring to the recent approval
of BMS 's TYK2 product which works through the JAK pathway, but the long -term
safety data for the product is yet to be awaited. So, if you look at the JAK as a class,
the concerns came through when Pfizer came out with five -year data for their first
product. So, T YK2 in that sense, is in very early days to conclude if the safety profile
is established. So, I think we need to wait for long -term safety data to have a final
verdict on the safety of that class.
Kunal Randeria: The reason I ask was, I don't think there is an y boxed warning that typically comes
with other JAK inhibitors, so I thought maybe physicians might be more amenable to
using this.
Anil Raghavan: For sure, I mean, at the moment, they are not treating it as a straightforward JAK inhibitor because it works
through TYK2, even t hough if you look at the downstream
has a similar pathway. In that sense, there are apprehensions about how that is going
to play out in the long -term safety trial.

Kunal Randeria: My second question is again on S1PR1 receptor. Maybe fingolimod could be going
generic in a couple of years , and it' s a m ulti-functional S 1P receptor agonist . So, I'm
just wondering, what advantage would your product have over fingolimod ?
Anil Raghavan: If you look at fingolimod , it is primarily approved and used in multiple sclerosis . And
we are seeing the second generation of S1P R agonists also coming into the multiple
sclerosis. But our development of the S1P1 program is in dermatology , and we are
not going after the S1P1 pathway in multiple sclerosis. But having said that, if you
look at our data on both the agonistic potency and the extent of internalization as we
have disclosed in previous presentation, vibozilimod is probably a best -in-class agent ,
and we are currently running clinical trials, both in psoriasis and atopic dermatitis.
And our data from those trials will dictate the competitive efficacy of our product
against some of the other S 1P1 programs which are actively explore d in dermatology.
Dr. Siu -Long Yao : I just want to add to Anil 's comment there, that we also believe that we have a very
clean compound compared to some of the others. So, because of the great potency,
we're hoping to have a better therapeutic index, which would give us an advantage
over the existing compounds.
Kunal Randeria: There have been a lot of S1P1 products in the market for several years now. So , just
wondering why the companies are not pursuing clinical trial in the dermatology
space ?
Anil Raghavan: If you actually look at the dermat ology field, which ha s now begun to f orm. There are
three programs in clinical development including ours. Arena Pharmaceuticals, which
is now Pfizer , is actively pursuing this , and BMS recently entered not with ozanimod ,
but with a different agent, which is in dermatology, which requires a very clean safety
profile as S iu mentioned. So, this field is becoming active, especially after the JAK
experience, both in atopic dermatitis and psoriasis . We may probably see this even
expanding beyond that in the dermatolog y space .
Moderator: We have our next question from the line of Narottam Garg from Chanakya. Please go
ahead.
Narottam Garg : My question is on vodobatinib . You mentioned that you would complete the
recruitment for the program by the end of this financial year. And given that, the first
study is a 40 -week long study, if I'm not wrong, so, would that be fair to say that the

first read out for the Phase -2 will probably be by the end of the next calendar year,

which is CY' 23, i s that how one should t hink about it ?
Anil Raghavan: So, the expectation that we are setting through definition and also our previous
presentation is that we will have top line data readout in the second half of financial
year '24. We have n't made a more precise expectation setting in terms of specific
order for data readout, but our expectation is that we will complete the accrual by
this year and then nine months will give you the top line data which is towards the
end of 202 3 or early part of 2024 calendar year.
Moderator: We have our next question from the line of Jay esh Gandhi from Harshad Gandhi

Securities. Please go ahead.
Jayesh Gandhi : Sir, can you provide me with peak sale expectation for PDP -716 and SDN -037?
Jaydeep Is srani: Since the assets are commercialized by our partners, we don't provide a forecast or peak sales expectation for our products.
Jayesh Gandhi: If you can help me with out -licensing fee that we have got from Visiox Pharma for these two programs ?
Jaydeep I ssrani: We've got 10% equity in Visiox , but that's subject to the approval from SEBI . And we are eligible for additional regulatory milestones on the filing and approval of the program.
Anil Raghavan: In addition to the milestone and the 10% equity, we are also eligible for lo w double - digit royalties in this program.
Jayesh Gandhi: We are in process of out -licensing phenobarbital also. Any ballpark number if you can provide ?
Anil Raghavan: Not at this point, I mean, we are in active negotiations on phenobarbit al licensing. We hope to conclude a transaction soon as the PDUFA date for the program is coming up in November of this year. Our hope and expectation is that we will be able to conclude a transaction before the PDUFA outcome , so that, we can go into a launch as soon as possible. So , at the moment, we're not in a position to give you details on the transaction that is shaping up.

Moderator: We have our next question from the line of Ma nish Jain from Gorma lOne LLP. Please go ahead.
Manish Jain : On CML , how are we comparing to asciminib in particular? And later, I'll go to PD on vodobatinib .
Anil Raghavan: Hi, Manish. Thank you for that question. In CML, if you look at the setting that we are pursuing, and the data that you have seen in this presentation, is different from what Novartis has pursued for their registrational trials and subsequent trial. What we have here is last line setting , these are patients coming off ponatinib, failing three lines of tyrosine kinase therapy, and one of which needs to be p onatini b. Asciminib trial was in two lines of failure against b osutinib . So, in that sense, both the severity of resistance and the challenges in t he composition of the resistance is significantly more, and there are no last line trials in this setting. So , we cannot directly compare the two programs.
Manish Jain: So, in terms of additional patients to be recruited on this program, on Slide # 14, the numbers highlighted those are enrolled. How many more to get enrolled?
Anil Raghavan: I think as per the original FDA discussions, the target was in the range of roughly 50 patients for registration submission, but that's something which we hope to discuss with the Agency once we go back to them towards the end of this year given the rare disease nature of this indication.
Manish Jain: On PD essentially, what kind of risks do you foresee more than looking at the quantum of peak sales , what are the things th at can actually determine the sales potential of the product , what are the risk factors or performance factors , if you can give a little bit of insight on that?
Anil Raghavan: We are in a Phase -2 proof of mechanism study for the Parkinson's disease, where we are exploring the viability of novel hypothesis . As y

ou've seen in this presentation and also in earlier presentations, we are pursuing a fairly large and robust data set and being fairly transparent in terms of why we think it's a reasonable shot to take . At the same time, the translational risk of failure in this area is quite substantial. So, the translatability of these preclinical models into clinical outcomes, which we will see once we have a top line readout from the program. I think I w ould rate that as one of the more significant risk items. And if we are successful, and if we get statistically

relevant difference between treatment arms and placebo, the extent of that difference in terms of both h ow much of slowing down of the deterioration of th e MDS UPDRS score plus, how long you can keep patients from having to need symptomatic therapy which is levodopa , they're going to be important factors in this program .
Manish Jain: Given that vodobatinib is quite significant , so, what are our manufacturing plans here to ensure that there is no slippage or delay led to manufacturing concerns?

Anil Raghavan: At the moment, we are not looking at diversifying the manufacturing base with a Phase -2. This is an active consideration for us going into P hase -3. And once we have success, we will definitely expand the manufacturing base with this given both the scale or potential scale of the product plus the potential value impact on our portfolio. We will not spare any effort in terms of creating multiple manufacturing options for this, but that's more of Phase -3 question as against the Phase -2 question.
Moderator: We have our next question from the line of Girish Bakhru from Orbi Med Advisors LLC. Please go ahead.
Girish Bakhru : Just continuing on v odobatinib, a ctually on the PD side, just wanted your assessment of the risk benefit ratio, which I think FDA has been focusing on when they are looking at new treatments coming in this class. So, from what all approvals have been coming in PD, we have not seen any major breakthrough so far, and so many different pathways are being tried, like biologics or alpha synuclein and all these things. So, where do you think FDA will focus on Phase -2 readout if it comes to that ?
Anil Raghavan: I think as you mentioned, we have seen a real dearth of options here, especially after the failure of some of the a lpha synuclein targe ted antibody programs. F rom our standpoint, looking forward to FY '24 for readout of the PROSEEK program , what is going to be important is the extent of activity that we see and the toxicity profile that we see. I mean, when we look at our program so far, we have around 350 - 355 patients and we are not seeing as significant or alarming signals on safet y so far unlike some of the antibody programs both in AD and PD, which was, I guess the basis of your question, people have seen significant level of neuro -inflammation. We haven't seen any alarming signs on toxicity. But when I say this, I'm talking about just blinded visibilit y of safety data. So, a real sense of where we stand in terms of risk benefit, can only be commented upon once we have an opportunity to see the data from this

fairly large as in 505 patients study next year, that will be an important proof -of-concept moment for this hypothesis also is not just for us, as to your earlier point in terms of the dearth of options. If you look at everything that's going on in sporadic PD as in non-genetic PD, Abl inhibitors is probably the most advanced and probably the more vi able hypothesis that is being explored at the moment.
Girish Bakhru: Just to clarify, you're not pursuing PD psychosis?
Anil Raghavan: No, in PROSEEK trial we are recruiting very early stage Parkinson's patients. They haven't experi

enced symptomatic dopamine altering therapies like levodopa. So, in that sense, what we are trying to catch is early stage neuro degeneration to the extent that's possible, and try and slow down the progression of disease, so that the trajectory of our patients, as in patients under treatment, will be different from the trajectory of patients under placebo. And by slowing down the disease substantially, your hope is that you can modify the disease over a course of longer period and keep patients off the need to have symptomatic therapy, which creates s ubsequent complications like dyskinesia, etc.
Girish Bakhru : I think we discussed this last time . You said that there's been of course a lot of interest around this asset a nd several parties are probably loo king at it. So , if you could just give some sense on is there a potential of monetizing this s oon after positive Phase -2 read out?
Anil Raghavan: At the moment, we're committed to kind of seeing the Phase -2 clinical trial through and we are resourced to do that. And it opens up a lot of strategic options to SPARC both in terms of encashing partially , or kind of staying on with the asset . Those are all options that we will evaluate as we go into next year. But unfortunately, I don't have a firm guidance on th at right now.
Girish Bakhru: Last one on SCD -153. I couldn't get . Is the asset also a tinib here for alopecia?
Anil Raghavan: The lead indication for that i s alopecia areata.
Girish Bakhru: I mean, there is already like two , three tinibs like Olumiant I think got approved recently.

Anil Raghavan: This is not a kinase inhibitor. This is not a tinib . This is a novel mechanism of action and we haven't disclosed mechanism of action for IP reasons in this presentation.
Girish Bakhru : But any comment on, how even I don't know if Olumiant is expected to do well or what, but I notice a very high unmet need, b ut with your mechanism action do you

think could be far better than this.
Anil Raghavan: That is probably premature for me to take up a position on this and we are in late preclinical setting at this point a nd we will closely watch this space a nd as we accumulate more data and go into clinic, I'm sure w e will be in a position to kind of articul ate the relative performance factors for both these classes.
Moderator: We have our next question from the line of Ke tan Gandhi from Gandhi S ecurities. Please go ahead.
Ketan Gandhi : Sir, can you please throw some light on h ow many program s do we have fast track approval , particularly LBD, PD and C ML?
Anil Raghavan: LBD and PD are not in registrational trials, they are in Phase -2 trials , and in CML, we don't have fast track, but we have an orphan drug designation and path to an accelerated outcome , in the sense , surrogate endpoint, which needs to be confirmed with confirmatory trials later on. So , it's not a priority review or fast track as you indicated.
Ketan Gandhi: Where is Xelpros and Elepsia manufactured at present ?
Anil Raghavan: Xelpros and Elepsia, the manufacturing base is Halol .
Ketan Gandhi: Phenobarbital and brimonidine, where we plan to manufacture?
Anil Raghavan: We have two si tes for phenobarbital. We haven't disclosed the manufacturing strategy for the product. Once we have a finalization of the commercial partner, we will be in a position to communicate that .
Ketan Gandhi : What would be the timeline for p henobarbital to reach the p eak sales potenti al?
Anil Raghavan: So, again, we're expecting an outcome from our current review process by next month. And as you see, this is a conversion of a DESI product, I mean, in the sense

that there are unapproved products in the market a nd the DESI exclusivity works like once you

have an approval and using due process, FDA has a commitment to take out the unapproved products in the market. And that decision to enforce the DESI exclusivity is driven by certain determinations the Agency need s to make d uring the course of the first year or so, in a sense, the robustness of the manufacturing plan and the unmet need So , there is a certain set of determinations that the agency need to make in terms of the timing of enforcing that exclusivity. W e don't want to set a stipulat ive expectation in terms of specific time for seeing the unapproved product going out of the market. But the estimation of the peak sales is going to be linked to that event.
Moderator: We have our next question from the line of Harit h Ahamed from Spark C apital. Please go ahead.
Harit h Ahamed : My first question is on Xelpros, which is partnered with Sun Pharma. It's been like three years since launch. So , how has been the ramp up so far a nd have we kind of reach ed the peak sales or peak R x for this product ?
Jaydeep Is srani: Thank you for the question, Harit h. As I mentioned earlier, this is a program partnered with Sun Pharma, and they w ould be providing an update on the sales at appropriate time or whenever they disclose about the sale s potential . So , we wouldn't be commenting on where or how much they are doing. Sun may provide guidance, as they have been doing in the past about the program.
Harith Ahamed: On PDP 716 a nd SDN -037, just curious as to Sun Pharma is not the partner for these assets given ophthalmology is one of their focus in specialty segments and we've also partnered with them on Xelpros . If you could comment a bit on the capabilities at Visiox and your expectations in terms of how they plan to take these assets forward ?
Anil Raghavan: For each of these out -licensing commercialization decisions, you go through a fairly robust process in terms of identifying a partner , who gives us the best shot of maximizing the deal from the product. And in this case, also, we have gone through a process and we had multiple players involved in terms of interested parties, and Visiox was probably the most aggressive b oth in terms of the investments that they're committing and the qual ity of the team that they're bringing to the table, and their track record in terms of having succeeded in other areas , not in ophthalmology , I mean, this is their first ophthalmology effort. So , there were several factors, which

gave us confidence in terms of the potential trajectory of the product with this proposal. And they also, as Jaydeep mentioned earlier, gave us an opportunity to have contingent share of value in addition to a traditional structure. So, it's a traditional structure plus access to a slice of value that this product may create. So , in that sense, both on commercial terms and other dimensions of the proposal, both in terms of capability , investments , committed, etc. , that is a very healthy proposal , and that's the reason why we went to Visiox .

Harith Ahamed: On phenobarbital, can you g ive some color on the other therapies in the neonatal seizures indication against which phenobarbital will be competing and given that we are very close to the PDUFA date , any comments on the interest level in the assets from your licensing discussions with potential partners?

Jaydeep Issrani: Currently , there are no approved agents for treatment of neonatal seizures . Phenobarbital as you would know is the standard- of-care currently in the US and other markets. And the second alternative is injectable levetiracetam. So, those are the two agents , but both of them are unapproved therapies being used by the physicians. So, that's the answer to your first question. And on t he interest, of course, we have been in discussion with potential partners

, and we're trying to close this at the earliest possible so that the partner would have enough time and a longer period for commercializing the as set during the exclusivity period t hat we may get after the approval.

Harith Ahamed: On SCD-044, you mentioned that you're expecting completion of Phase -2 in FY '24. So , any timelines that we can expect for Phase -3 trials and maybe filing any target ?

Anil Raghavan: So, the practice that we are following on these partner ed programs is that , we're basically leaving the disclosure on the execution of those programs to the licensee since both parties are dealing with the same set of investors . We haven't in this presentation indicated a timeline . But , as you know, we are in two trials in atopic dermatitis and psoriasis and Sun would be in a position to set expectation in terms of the registration trajectory of the product.

Moderator: We have our question from the line of Manish Jain from G ormalOne, LLP. Please go ahead.

Manish Jain : So, essentially, I just wanted to know on phenobarbital . My assumption is it will primarily be a hospital setting product. So, typically, how many sales reps would be required for such a kind of product?

Jaydeep Issrani: The anticipation that this is going to be a hospital product is correct. It will primarily be used in the ICU setting or neonatal centers, which are not many, few in US. And the number of reps or the sales team will be dependent on the partner that we engage with and their strategy to commercialize the asset. So , I will not be able to give you an exact number, but it should be in the similar ballpark range as any other asset targeting neonatal centers.

Manish Jain: Because, at some point of time, I had a broader question from this which I have been raising with you is for such kind of products, at what stage , does it make sense for SPARC to actually go out and try and do the marketing on its own rather than being dependent on an external party?

Anil Rag havan: Thank you, Manish. I think that's an interesting question. Your question is do you actually try and optimize it for a smaller product which requires a smaller scale of effort or do you go for a bigger product? Because the extent of bandwidth that we need to spend at management levels and also the distraction that it may cause for the rest of the portfolio, is going to be considerable. So, that's an interesting question and an active question in terms of what would be an appropriate time to look at ot her options for commercialization. But, at this point, we have concluded that phenobarbital may not be that opportunity.

Manish Jain: Given that we are now getting into very interesting biologics like SBO -154 and others, what's going to be our manufacturi ng strategy there?

Anil Raghavan: At this point, SPARC has no plans to invest and build manufacturing capabilities for these novel modalities or for that matter traditional modalities also. So, our manufacturing strategy is to broad -base the manufacturing or supplier base and de - risk as much as possible. So, in this case also, we will continue to build up multiple options for manufacturing and as we kind of start preparing for the IND next year for this, we are already evaluating multiple external vendors for the MUC -1 ADC program.

Moderator: As there are no further questions, I would now like to hand the conference over to Mr. Jaydeep Is srani for closing comments. Over to you, sir.
Jaydeep I ssrani: Thank you, Yash, and thank you, everyone for joining today for the call. We tr ied to answer the questions you had, but in case there are any additional questions or follow on questions, you may reach out to us and we will respond to you through e -mail.
Thank you o

nce again for joining the call today.
Moderator: On behalf of Sun Pharma Advanced Research Company Limited, that concludes this conference. Thank you for joining us and you may now disconnect your lines.

Call: Nov 2023

SPARC/Sec/SE/2023-24/069 November 10, 2023

National Stock Exchange of India Ltd.,
Exchange Plaza, 5th Floor,
Plot No. C/1, G Block,
Bandra Kurla Complex,
Bandra (East), Mumbai – 400 051. BSE Limited,
Market Operations Dept.
P. J. Towers,
Dalal Street,
Mumbai - 400 001.

Ref: Scrip Code: NSE: SPARC; BSE: 532872

Dear Sir/Madam,

Sub: Transcript of the Investor's call pertaining to the update on Clinical Programs and R&D Pipeline

Please find attached herewith a copy of the transcript of the Investor's Call pertaining to the update on Clinical Programs and R&D Pipeline held on November 02, 2023.

The aforesaid transcript shall also be available on the website of the Company.

Yours faithfully,
For Sun Pharma Advanced Research Company Ltd.

Kajal Damania
Company Secretary

"Sun Pharma Advanced Research Company Ltd. (SPARC) Update on Clinical Programs and R&D Pipeline Conference Call "

November 02 , 202 3

MANAGEMENT : MR. ANIL RAGHAVAN – CHIEF EXECUTIVE OFFICER
DR. SIU-LONG YAO – HEAD CLINICAL DEVELOPMENT
DR. VIKRAM RAMANATHAN – HEAD PRECLINICAL DEVELOPMENT
DR. NITIN DAMLE – CHIEF INNOVATION OFFICER
MR. CHETAN RAJPARA – CHIEF FINANCIAL OFFICER
MR. JAYDEEP ISSRANI – HEAD BUSINESS DEVELOPMENT , CORPORATE COMMUNICATION & INVESTOR RELATIONS

Page 2 of 40
Moderator: Ladies and gentlemen, good day and welcome to SPARC'S Update on Clinical Programs and R&D Pipeline Conference Call .
As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing '*' then '0' on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Jaydeep Issrani from SPARC . Thank you and over to you, Mr. Issrani .
Jaydeep Issrani : Thank you, N eerav. Good evening, ladies and gentlemen. My name is Jaydeep Issrani , and I Head the Business Development and Investor Relations at SPARC.
On behalf of SPARC , I welcome you to the SPARC 's Yearly Update on Strategy and Program Updates .
I'm joined by our CEO, Mr. Anil Raghavan, and members of SPARC' s senior management team for the call today.
As we have done in the past, we will be using a similar format for our discussion today , that is , the presenters will walk you through the slides after which the call will be open for questions and discussion.
The prese ntation for today's discussion was shared earlier. If you have not received, you can download the presentation from our website, which is www.sparc.li fe.
Before we start today's discussion, I would like to remind you that our discussion today includes forw ard-looking statements that are subject to risks and uncertainties associated with our business. Hence, actual results may be different from those projected in today's presentation.
I will now hand it over to Mr. Anil Ra ghava n for his presentation . Over to you.

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Anil Raghavan: Thank you very much Jaydeep for kicking this off.

Hello everyone. It's my pleasure to welcome you all to SPARC's 13th annual portfolio update. As always, we are immensely grateful for your continued engagement and support. This means a lot to us. Thank you, for taking the time.

I am sure, most of you ha ve been regulars here and you know the routine. We have the SPARC's management team on this call today. Most of us are on these calls for some time. But we have a new addition. That's Dr Venkat Palle who joined us as the head of Drug Discovery and Pre -clinical Development late last year. Dr Palle comes with an impressive trail of achievements and we are lucky to have him leading the charge in discovery and preclinical development.

In terms of our agenda today, we will begin by covering certain important elements of our strategy. I will also give a bit of the 'lay of the land' of our late stage portfolio and provide some guidance on what to expect in the short to medium term. After which we will spend much of our time today on three things… Dr Siu -Long Yao will talk about our clinical programs, that's Vodobatinib in Parkinson's disease and CML, followed by Vibozilimod in Atopic Dermatitis & Psoriasis. Dr. Vikram Ramanathan and Dr. Nitin Damle will provide updates on the progress of two important programs wh ich we introduced last year, SCD -153 and the SBO -154. We will conclude the presentation with brief comments on our financial situation and cash flows from Chetan.

So, with that, let's get going with slide 5 please

Here we have a recap of the shifts that we have taken in the last few years from being a primarily delivery system focused company to an approach which embraced the risk profile of a modern biotech fully. In the sense,

Page 4 of 40
towards a portfolio of assets holding significant promise of improving the standards of care and making an impact on the lives of patients we hope to serve while carrying multiple layers of risks. But our model differs from the contemporary biotech business model on many fronts. We have spoke n

about these differentiating factors at length in the past

, Our captive
operations, Low cost of failure, Breadth of our portfolio, Compelling founder vision/financial commitment and so on. We have also spoken about our efforts to narrow our therapeutic fo cus and sharpen our execution. So, I don’t plan to go over these foundational elements of our pivot again today.

But I would like to highlight two other points before we go. In the last few years, we have made a significant push towards strategic partner ships with tier 1 academic research systems globally to access high quality science and ideas early. We can clearly see the impact of that sustained effort in terms of collaborative programs moving to Clinic now. That is an important validation for a key t enet of our strategy. We also have additional early stage, undisclosed pre -clinical programs in collaboration with external marquee institutions. A productive bridge to external science has always been part of our vision and differentiation strategy. We ar e hopeful that the bonds and competencies we have built in establishing these structures are going to be extremely useful in further building out our portfolio. We really look forward to that.

I also want to touch upon an element which is not specificall y called out on this slide. That’s our changing modality mix and the maturation of our biologics group. We have completely moved out of NDDS programs which was our mainstay in years past. Traditional small molecule chemistry is a significant share of our e ffort and will continue to be so. We think small molecules will remain an important part of the therapeutic tool kit because of the larger swathe of intercellular targets that can be addressed with appropriately designed small molecules. We have made explo ratory investments in building an in -house biologics capability ‘ground up ‘under Dr

Nitin Damle’s leadership. That effort is now turning real with its first asset, an antibody drug conjugate, approaching its clinical entry in 2024. But more importantly we have built a competitive competency which can engineer antibody therapeutic constructs, be it traditional naked antibodies, drug conjugates or bispecific or multi specific antibodies and scale them to clinic. We are also leveraging our understanding from the ADC effort to create small molecule drug conjugates which is an important emerging therapeutic class. Additionally, we have several exploratory efforts in newer modalities and are taking some really meaningful bets around pushing our modality mix even further. We look forward to bringing additional programs from the biologics basket for sure and hopefully even from newer modalities in the short to medium term.

So, in recap, I want to re -emphasize the point I made during our call last year. For investo rs with an appropriate risk appetite and an ability to grapple with translational risk, SPARC offers a compelling story. And that story is getting more real now than at any point in the past as we have several important catalytic events coming up in 2024. So, let’s go to the next slide to check out what is in store.

Slide 6 please

This slide has a straight forward listing of a series of data and phase transition events set up for the next calendar year. Some of them will have the ability to move the SPA RC valuation substantially based on data either way. So, let’s go over the list.

First up is the PROSEEK interim analysis. We are doing an administrative interim analysis which will give us the primary endpoint and secondary endpoints trends for 442 pati ents plus additional biomarker trends. The important business objectives of this administrative process are two -fold, first to initiate potential licensing discussions before the public release of the

full data set and secondly, to initiate planning and se t up activities for the regulatory interactions and potential phase 3 trials of Vodobatinib in neurodegener

ative disorders. We plan to maintain the study blind for everyone outside of a small group covered under a very strict confidentiality commitment.

We expect two more important program milestones in Q2, of 2024. SCD -153, will have its first data read out which will help us develop an initial understanding of the potential safety profile of the product. Also, more importantly, an ability to move to MAD studies in patients which can give us a lot more information regarding target engagement and efficacy markers plus some early indication of potential therapeutic dose.

We will also close out the patient enrolment of our Atopic Dermatitis program of Vibo zilimod in the second quarter.

In the third quarter – PROSEEK will read out fully. As you can see, that is probably the most important data catalyst for the company currently and certainly most keenly watched. That will lead to setting up the End of Phas e 2 meeting with the FDA to agree on the registrational path for Vodobatinib in PD. We will look to initiate the phase 3 program in Q4 and start exploring the fuller opportunity set which I will touch upon in a moment. We will also look to conclude our par tnership strategy for the asset. PROSEEK will also give us clarity in terms of our resourcing requirements and options. So that's a big one.

We have three significant milestones setup for the last quarter of the calendar. Our first biologics IND. Our ant ibody drug conjugate program is currently tracking to that plan. We will then have phase 2 data from the Atopic Dermatitis study. From a SPARC perspective that would set the stage for additional milestone payments from our licensee + we can initiate the

FDA consultation and pivotal trial planning for AD. Finally, we will also have the MAD studies of SCD -153 getting started in Q4 24.

We have only listed the data or milestone events from our larger programs here. We will also have additional milestones like the refiling of Brimonidine and potential progress in enforcing our exclusivity for Sezaby. I will come back to those programs towards the end.

Before we go to the next slide, I want to summarize all these very briefly. 2024 will see several big moments for SPARC including opening the lid on a potentially game changing program for the company and the field. I can't overstate the significance. So, I want to spend the next few slides to fully unpack the true meaning of these milestones for SPARC and its shareholders.

Let's go to slide number 7

First the big news. PROSEEK achieved its recruitment target late last month. We set out to recruit 504 evaluable patients. We ended up with 513. Siu will give you more color in terms of both the state of the study and expectations, but I want to recognize the team and the effort that went into completing the accrual of this study which is one of the largest Phase 2b studies currently active in early stage Parkinson's disease globally. We started this trial in full swing when health systems around the world began to shut down because of the pandemic. Covid created enormous challenges in prosecuting this program, but we are genuinely happy to be here and want to thank patients, their families, our investigators and site staff around the world including some re ally passionate physicians in India who supported this program wholeheartedly and our execution partners and a large team at SPARC who drove this program with enormous zeal. As I mentioned in the previous slide, we expect the interim analysis completion by end of Q1, 2024 and a full readout and Phase 3 initiation soon thereafter. And that indeed is something to look forward to.

So why are we so excited.

PROSEEK will give us a definitive proof of concept for one of the most important hypotheses in the ne urodegenerative diseases field. that's c -Abl inhibition as an approach for neuroprotection. The role of excess

ive reactive
oxygen species and the redox imbalance it creates in triggering a cascade of events that results in neuronal toxicity and death has b een identified as a potentially intervenable pathway across the neurodegenerative spectrum for some time now. Dr Ted Dawson of Johns Hopkins and several other groups around the world provided compelling mechanistic and pharmacological evidence supporting t he c-Abl story. But this hypothesis has never been fully tested as the field lacked an appropriately specific agent with the relevant pharmacological properties, particularly good blood barrier penetration at safe doses. That's the promise of Vodobatinib. A sub nano molar inhibitor of c -Abl, ultra -specific to Abl 1 and Abl 2 that crosses the brain sufficiently to ensure adequate coverage with a peripheral safety profile which makes clinical exploration in the neurodegenerative context feasible. And importan tly that promise was delivered by Vodobatinib in preclinical and in early clinical studies. The totality of evidence we have from everything we have done so far in this program was directionally consistent and supported translation.

In that sense, PROSEE K promises the first real validation for the c -Abl pathway in a sufficiently sized global trial which has the right patient setting i.e. pre -L DOPA patients with a DAT confirmed PD diagnosis, and right duration to see the effect beyond the placebo impact, 40 weeks in part 1 followed by an LTE period of another 40 weeks. I want to talk about the broader implications of a positive outcome in terms of opportunities it presents. But before that, from an operational standpoint, our immediate

priorities coming ou t of PROSEEK is the finalization of a late stage development plan including our partnership strategy, getting regulatory concurrence with FDA and other agencies around the world and initiation of its first registrational program. These will be the most imp ortant execution priorities for SPARC next year if we see the data we hope to see with PROSEEK. Now let's go to slide 8 for a bit of a big picture view.

Next slide please.

The way to look at the opportunity PROSEEK validates, is to look at various layers of the hypothesis and see the context in which it is tested. At its core, PROSEEK gives direct validation to the relevance of Abl inhibition and oxidative stress response modulation in Parkinson's disease. So, if inhibition of c-Abl means bending of the degenerative curve in a significant manner, that not only slows the progression during the initial, pre 'symptomatic therapy' phase which PROSEEK targets, but can also extent the period of effective management under dopamine therapies. And at the starting phase of the spectrum, biological understanding of prodromal PD or precursor conditions like constipation and REM sleep disorder can offer additional opportunities to intervene early and intervene proactively. So, the inner core of opportunity for Vodobat inib & other selective, brain penetrant Abl inhibitors is to become a background therapy across the PD treatment continuum.

The next layer comprises a set of diseases characterized by α Syn aggregation like the Multi System Atrophy and Lewy Body Dementia. In these indications several pathological hallmarks of PD are at play, but also some important differences. Like the area of the brain that gets affected. We believe that a compound like Vodobatinib which can moderate oxidative stress response and get distributed uniformly across the brain is an agent worthy of testing in these immediate adjacencies. That basket from an epidemi ological and inadequate SOC standpoint, offer some really large opportunities.

Then there is an outer ring of possibilities driven by other culprits like activated Tau, A Beta, SOD, etc. in diseases like Alzheimer's, & ALS. Our early studies in IPSC deri ved neurons indicated so

me of those possibilities and we
plan to continue studying Vodobatinib and its promising backup series in these conditions.

So, summarizing these two slides, we have reached a significant and important milestone with the completio n of the enrolment target for PROSEEK. We expect substantial data -flow which not only validates our hypothesis, but has the potential to teach us a lot more in terms of the disease biology, heterogeneity of the disease & response, correlation of several im portant candidate biomarkers, and so on. That sets up multiple patient settings which can potentially make Vodobatinib the backbone of the standard of care across the neurodegenerative continuum. And that is a huge deal. And we are understandably super exc ited about it. Now let's move on and examine the key "what if" question, which I am sure all of you have in your mind. Let's go over it.

Can we have slide 9 please

Much of the current biotech landscape consists of companies created to test one interest ing hypothesis. Investors and teams move on, either cash out on success or move onto something else on failure, the next shot. That's the typical story. We are architected somewhat differently with a substantial internal discovery, translational & clinical development capability with a cost of failure advantage. We are designed to spread the risk across an actively managed portfolio, learn from our successes and failures, strive to improve the quality of our competencies and deliberately build OPTIONALITY a s we come to critical decision points like the one that we are approaching now.

And we believe we do have meaningful optionality. Let me take a minute to explain.

SPARC's optionality has two coordinates. At a program level and at a competency level. At p rogram level, we have multiple options and combinations to pivot to if we need to. Immunology offers two high value options in Vibozilimod and SCD -153. We expect to see additional value unlocking data from Vibozilimod in the coming year. We will also see t he data build up for SCD -153. We have a broader set of options to go to in Oncology, where the highest value bet is the Muc -1 platform which can generate multiple products using the ADC and bispecific constructs. Vodobatinib CML offers a relatively lower v alue, but significantly surer option to leverage the asset if we unfortunately fall short in neuro.

But I think it is also equally important to consider the optionality provided by our competencies. SPARC represents one of the more sophisticated translat ional engines attempting to innovate for the world from India. Across the value chain, from the ability to develop assets in multiple modalities, to self -sufficiency in various pieces of translational development in multiple therapeutic areas and the abili ty to design and execute clinical programs globally, we have had substantial learning from our successes and failures. That's one of our biggest assets differentiating our program. I will go over a few specific points on our program hedges in the next coup le of slides. But let me say this in summary. Investors evaluating SPARC should look at our portfolio as one substantial opportunity for standard of care altering innovation in a large unmet area – that is Vodobatinib – backed up by a larger number of dece nt hedges across immunology and oncology, with many of them reaching important developmental milestones in the near future

Now let's go over slide 10 to take a look at the immunology programs

I have already spoken about these two programs in my earlier comments. Just a few high -level observations. On Vibozilimod, proof of mechanism for SIPR1 agonists exist from competing clinical programs in dermatology and a wider swathe of indications outside of dermatology. As previously disclosed, we have reached target pharmacodynamic levels at safe doses, that is well

correlated with efficacy in this class. We have completed pooled analysis of the safety data from 125 patients across Atopic Dermatitis and Psoriasis and managed to remove most of the extensive cardiac monitoring initially required in the trial. We will go over this in more detail during our clinical segment. We are now aggressively scaling the program with significant expansion to Europe and Canada. We are also adding new sites in the US. From a competitive positioning standpoint, in Atopic Dermatitis and Alopecia Areata, the oral standard of care currently consists of JAK inhibitors which carry a black box warning. Both Vibozilimod and SCD -153 has the potential to be much safer options in AD and AA. On a separate note, both these programs offer excellent examples of our strategic partnering program. Vibozilimod, as you may know, was developed in partnership with a French biotech, Bioprojet and SCD-153 is an ongoing collaboration with a two very accomplished groups from the US and Czech Republic. We started our collaboration with these groups as a joint development effort with an option to license their IP. I am happy to announce that SPARC exer cised the option to license the Intellectual Property supporting this program. You may have seen our press release today announcing the definitive licensing agreement. That allows us to move forward with SCD -153 as a fully owned SPARC program.

Next slide , that's slide no 11 explores additional pages we can turn to in oncology.

CML component of Vodobatinib was always meant as a backup for the Parkinson's program. We have had significant regulatory interactions recently about the nature of the data packag e the agency would like to see

for registration. In line with FDA's broader strategic shift towards encouraging companies to explore marketing approval in earlier settings, FDA suggested a comparative trial in 2L failure patients as against the last line, that's the post ponatinib setting. We will go over additional detail on the emerging design of the expected study in the clinical session. We are also re -evaluating the costs and timelines before deciding to go it alone or partner at this stage. But Vodoba tinib in CML represents a surer hedge, surer because of clinical validation, and alignment with FDA on a path to registration. We will have clarity and readiness to move forward when we reach that decision point post PROSEEK.

We have discussed the ADC pr ogram earlier also and Nitin will do a deeper dive later. But let me say this, we see enormous opportunities in Antibody or small molecule targeted preferential delivery to cancer cells. These platforms can be leveraged for the delivery of a wide set of pa yloads ranging from small molecule traditional cytotoxins, highly potent targeted therapies with limited safety margins, RNA therapeutics, etc. Muc -1 gives us a differentiated platform which can open up multiple product opportunities. Our oncology effort a lso carries significant interest in certain other target classes, like new synthetic lethality pairs or exploring ways to intervene in the RNA process.

Before I look to summarize, want to talk briefly about how our competency has evolved and how it offer s another level of risk mitigation. Please go to slide 12.

As I said, we don't look at SPARC just as a portfolio of programs, though they are the key drivers of current value. A significant part of our proposition is our process which is refined over a l ong period of time, imbibing the learning from our successes and failures. We believe that SPARC development process in terms of targets we seek to address, validations that we insist, developability considerations around the nomination of an asset,

clinic al strategy and execution, adoption of biomarkers, and our overall development tool kit, all have come a long way since we spun out of Sun. Equally importantly our

portfolio management process evolved into a rigorous, objective methodology which puts a pre mium on establishing and enforcing smart data stage -gates which allow us to kill unviable programs early, cheap and completely. So, SPARC is not just a bet on a high value portfolio, but on a system and competency which has been evolving and invested into over a long period of time. I hope you share our excitement and confidence beyond the here and now, which is certainly exciting, but also for the long run.

Let's go to Slide 13. We will have additional comments on cash flow and burn from Chetan towards t he end. But want to make a few observations on additional cash events outside of Vodobatinib and other earlier stage programs we spoke about here.

Our partners have successfully launched Elepsia, Xelpros and Sezaby in the US. Elepsia is commercialized by our partner Tripoint and we have achieved early run rates indicative of a robust growth trajectory. Unfortunately, we have had supply disruptions from the Halol import alert. We have managed to find an alternative vendor and are in the process of finalizi ng our plans for the relaunch. Sezaby, that's the Benzyl Alcohol free formulation of Phenobarbital, got approved in the US last November. Sun, our commercialization partner has launched Sezaby in the US during Q1, 2023. As you may know this product has a 7 -year exclusivity and enforcement of that exclusivity is a key value driver for the growth of this franchise. We are working diligently to make the case with the agency and other stakeholders in order to remove unsafe options from the market. Let me not go into specifics on the numerous initiatives we are currently undertaking to safeguard the interest of patients and caregivers, we are hopeful that we can make progress on this important objective in the near term. And on PDP-716, as we have communicated ea rlier, we have received a complete

response letter from the FDA indicating certain deficiencies with the external API manufacturing plant. There were no additional clinical data requirements indicated at this time. We have already identified an alternative API source and are in the process of planning a resubmission in 2024. Finally, as we mentioned earlier, we expect the top line and initiation of the phase 3 program in Atopic Dermatitis for Vibozilimod, that triggers a cash milestone for SPARC. Plus, we b elieve that there may be opportunities to commute future revenues from this program to generate additional cash to support the development programs if that is required post PROSEEK readout.

In the next two slides, 14 and 15, we have summarized the portfo lio and key milestone expectations. We don't plan to go over them. Except for one program, which is our oral SERD, Bexirestrant. We took the program into dose escalations in patients. But based on data from competing programs, it was becoming clear that ef fective clinical differentiation for this class as whole is becoming a challenge. With small effect sizes, the program needs very large clinical studies and may not be the best of use of capital for SPARC. So, our portfolio review, as we disclosed earlier, down -prioritized the program late last year. We can go into additional detail if required during Q&A.

With that, let me transition the call to Dr Siu -Long Yao who heads up our

Clinical Development group for the next leg of the presentation. He will cove r the clinical stage programs in a bit more detail. I look forward to seeing you later for Q&A. Thanks again for your time. Over to you Siu.

Siu-Long Yao: Thank you, Anil. My name is S iu-Yao Long and I'd like to add my warm welcome, especially to the many who have accompanied us over the years. I oversee clinical development and I'll walk you through our major clinical programs . So, slide 17, the first program I'll discuss is Vodobatinib, o ur c-Abl inhibitor of P

arkinson's disease.

Slide 18 please. This slide gives you some basic background on Parkinson's Disease. There are a lot of people affected, approximately 7 million across the world and the prevalence is growing. There are anticipated to be 14 million affected by 2040. In fact, the growth of the disease is outpacing total population growth.
The therapy we have is one where we believe we can modify the disease, and actually change its trajectory or course, rather than just treating its symptoms.
The pictorial at the bottom of th e slide is there to give you a general overview of Parkinson's Disease. I'd like you to focus on the middle row first. There, you have the disease starting from a prodromal stage on the left and progressing to diagnosis, maintenance treatment, complex trea tment and finally, palliative treatment.
On the other rows, both above and below the middle row, there are major clinical events depicted that need to be managed.
As Anil mentioned, we are initially studying Vodobatinib in the prodromal/diagnosis stage b ut, if it works, there is the potential for all stages of disease.
Slide 19, please. This slide is a reminder of the design of the Phase 2 proof of concept study named PROSEEK. The study consists of 3 arms of placebo, a high dose of V odobatinib and a low dose of Vodobatinib. Part 1 is the key part of the study where subjects are treated for 40 weeks and the effects of treatment on the primary endpoint of Part 3 of the UPDRS scale is assessed. The study is fully enrolled and we anticipa te having some initial interim data in March, 2024.
Part 2 of the study is a long -term extension where subjects who initially received placebo, along with subjects initially treated with 384 mg, receive

additional treatment with 384 mg for an additional 9 months. Subjects randomized to 192 mg will continue that dose during the extension. Approximately 87% of eligible Part 1 patients have rolled over to Part 2 and completion of the study overall is anticipated in May, 2025.
Slide 20 g ives you an idea of the distribution of patients across the world. On the y axis is the number of patients from each region, and going across the bottom on the x axis is the region. Over 40% of the patients have come from the US.
Some additional detail ab out what we've seen so far is summarized in the other bullets on the right. There were grade ¾ events in about 6% of patients and GI events and rash were the most common AEs. We do have a DSMB, which is unblinded to treatment allocation, and reviews all sa fety data in an ongoing fashion and they have not recommended any changes in the conduct of the study following 6 interim reviews.
Slide 21 summarizes some of our biomarker plans in the study. The biomarker subgroup in the study consi sts of over 150 of the 504 patients in the study, which translates into over 50 patients per arm that will have biomarker data. What I just told you, though, is a simplification. In fact, some of the biomarkers will be obtained in all subjects but, in gene ral, we are targeting to get 50 subjects in each arm with all biomarkers.
Biomarkers that will be obtained upon study entry and exit include dopamine transporter scanning and skin alpha synuclein content. Target engagement will be assessed by c -Abl and CR KL statuses, and neuronal death will be assessed through neurofilament light. Pharmacodynamics will be assessed through various downstream targets. We have smartphone -

based measures of patient functional status.
Slide 22 provides a su mmary of some of the concomitant studies in the program. There is an ongoing carcinogenicity study and we plan to perform

a relative bioavailability study along with some drug -drug interaction studies to support Phase 3 plans. A hu

man ADME study is also pl anned.
Finally, in slide 23 there is a summary of some of other related alpha synuclein based disorders that could benefit. There are the other classical synucleinopathies such as dementia with Lewy bodies and multiple system atrophy along with the prodromal synucleinopathies such as primary autonomic failure and REM sleep behavior disorder.
On the right is a Western blot that summarizes the activity of Vodobatinib in a model of Alzheimer's Disease. Molecular weight is depicted on the y axis and treatments used are displayed as columns. In this case, treatments were done in duplicate so there are two rows for treatment with control DMSO and 2 columns for treatment with Vodobatinib. As you may know, Tau is the protein of interest in Alz heimer's Disease so I'll ask you to focus on the phospho -Tau and Total Tau rows in the middle of the blot. As you can see, treatment with Vodobatinib significantly diminishes both, suggesting that treatment with Vodobatinib may be effective for Alzheimer's Disease.
Slide 24 transitions to our program with Vodobatinib in chronic myelogenous leukemia.
On slide 25 is a summary of the commercial landscape for CML. The graph on the left consists of rate per 100,000 on the y axis and year going across the x axis. The top curve is the incidence of the disease and you can see that it is steadily increasing over time. At the same time, the rate of death from CML is decreasing so that, overall, the prevalence of the disease, or patient population, is increasing.
In fact, the current market is valued at 3.5 billion US dollars. The graph on the right gives you an idea of the major drugs used in the treatment of CML and their respective market shares.

Slide 26 summarizes more recent data we've seen with Vodobatinib in CML. The figure you see is a summary of all the patients that we have efficacy data on. The rows represent the disease status of the patient population either at baseline in the top row or fo llowing treatment with Vodobatinib in the second row. Percent of the population is represented on the x axis. As you can see, most patients were not in response at baseline. Following treatment, over 40% of the patients achieved responses, which is really remarkable in this patient population, many of whom were resistant to ponatinib. Responses were very durable as you can see in the last bullet where median time on drug was over 32 months.
Slide 27 provides some preclinical data that supports our plans to do an earlier line study with a larger market potential. On the y axis you have tumor volume in mouse xenografts and on the X -axis is time in days. The top curve represents results obtained with vehicle control and the line below that is tumor size following treatment with nilotinib, a representative 2nd generation TKI for CML. You can see that there is some control of tumor size, but treatment with Vodobatinib, either at a lower or higher dose in the bottom 2 curves, is much better. T his, along with other, data suggest that we can beat existing 2nd generation treatments in earlier lines of therapy and capture a larger market share.
In slide 28, we've had discussions with FDA and they are actually quite supportive of getting an initial approval in an earlier line of therapy. This is consistent with their project Frontrunner initiative. Our plans are summarized in the diagram below. We are planning a randomized study against one of the 2nd generation treatments for C ML with a primary endpoint of major molecular response.
Slide 29, please. The remaining slides summarize Vibozilimod, our sphingosine agonist, for the treatment of psoriasis and atopic dermatitis.
Slide 30, pleas e. As I mentioned, the 2 initial indication

s we are targeting for
Vibozilimod are psoriasis and atopic dermatitis. The prevalence of psoriasis

in the US is approximately 8 million and the market is dominated by biologics. Though there are biosimilars, pene tration has been limited to date.
For atopic dermatitis, the US prevalence is even higher, approaching 18 million. Though JAK inhibitors are available, they are associated with severa l black box warnings such as thromboembolism , cancer, major adverse cardiac events and death.
Slide 31 summarizes the design of the Phase 2 dose ranging study for psoriasis. There are 4 arms consisting of placebo and various doses of Vibozilimod. The primary endpoint is at 16 weeks which is Part 1 of the study. In Part 2, we will evaluate the effectiveness of switching to higher doses if efficacy is suboptimal. Part 3 is a safety extension. Currently, 15 sites are active in the US and 3 sites are active in Europe.
Slide 32 summari zes the atopic dermatitis study. Again, there are the same 4 arms but this time we've eliminated some of the re -randomizations. The primary endpoint is also at Week 16 and there are 18 US sites and 15 European sites that are active.
Slide 33 summarizes major events for the atopic dermatitis study consisting of a futility analysis in the 2nd quarter of 2024, an interim analysis for the primary endpoint in 4th quarter of 2024 and study completion in 2nd quarter of 2025.
That's my last slide. At this point I would like to hand you over to my colleague, Vikram, who will walk you through some very nice preclinical data with one of our up and coming drugs.
Vikram Ramanathan : Thank you, Siu, and good evening everyone. My name is Vikram Raman athan and I oversee Preclinical Development at SPARC, and in the next few minutes I will be giving you an update on our compound SCD -153 which is being developed for Alopecia Areata. We are happy to share that the IND was filed

in India recently, and we ha ve now received approval from DCGI to conduct Phase 1 clinical studies.
Slide 35 Alopecia Areata is a disease where the body's own immune system attacks the hair follicles and thereby causes hair loss. There is a large patient population that suffers from the disease and current treatments are inadequate. JAK inhibitors have recently been approved but they carry an FDA black box warning for cancer, stroke, and death. The other alternative is steroids but these are also not preferred because they require in tradermal injections and have serious side effects. The pictures show how the disease manifests. It disproportionately afflicts the younger population <30 years of age and causes them significant psychological stress. There is clearly an unmet medical need .
Slide 36 gives some background on the compound SCD -153 which is being developed as a topical therapy for Alopecia areata. On the left is shown a healthy hair follicle. The hair follicle has so called immune -privilege which means that it is shielded from the immune cells in the vicinity. The AA diseased follicle is represented on the right. In this case, the immune privilege is lost and there are now disease -causing T -cells present at the base of the follicle, in what is called a swarm of bees phenomenon. These T cells secrete inflammatory cytokines and damage the follicle and the so hair falls out. SCD -153 inhibits this inflammatory process and can counter the disease. Importantly SCD -153 has been designed as a topical agent, and so systemic exposure and systemic side effects would be minimized.
Slide 37 shows preclinical data in an immune cell driven mouse model of AA which spontaneously develops hair -loss. The disease here is caused by attack by the CD8+ cytotoxic T cells which is also the culprit in the human disease. The graph on the left shows the Hair growth index on the Y axis and

time of treatment of SCD -153 on the X -axis. Applying SCD -153 on the skin three -times -a-week results in the animals recovering hair growth compared to the vehicle treated an imals. We also studied other dose strengths and

regimens of SCD -153, and have seen meaningful hair growth in those cases also.
The hair growth can be visualized in the photographs. The hair growth index is derived from software -based analysis of photogra phs. The animals start out with similar level of disease at Week 0. The animals in the top row on the right received vehicle for 16 Weeks and showed continued disease and no hair growth. Animals in the bottom row on the right received SCD -153 applied to th e skin for 16 Weeks and show good hair growth. We have also examined the skin samples under a microscope and found that the disease - causing T -cells are reduced after SCD -153 treatment. So, these data demonstrate good single agent therapy. Importantly, ther e is room now to also look combination therapies with SCD -153 for hair growth, and we have initiated combination studies with low dose JAK inhibitors.
Slide 38 shows quantitative PCR based measurement of gene expression of inflammatory markers in skin of t he AA diseased mouse skin. These are the interferon gene signature associated with AA in humans. The Y -axis shows the relative fold increase in the mRNA gene transcript levels above the baseline. In each set, the first bar is the level in healthy mice, the y are low as they lack disease. The second black bar in each case is the fold -increase in the diseased mice and the difference between the first and second bar is the disease -associated change. In each set, the last two bars are from the diseased mice that have received SCD -153 treatment on the skin, and a reduction in the transcript levels of these genes is evident. These data provide a mechanistic basis for how SCD -153 causes hair growth in this disease model. The markers shown here attract the disease -causing T cells to the site, and these then initiate the disease. We should particularly note the increase in the chemokine CXCL9, 10 and 11 in this slide; and we will come back to the same CXCL9, 10 and 11 after a couple of slides.
Slide 39 summarizes the current status. Following Toxicology and Safety Pharmacology testing the IND was recently filed with the DCGI. We now

have approval for the First -in-human Phase 1 study. The Phase 1 study will be initiated in India in Q4 of 2023. The Single ascending dose study will be a randomized double -blind vehicle -controlled study and as shown in the figure we will have 5 ascending dose cohorts. As usual, there will be inhouse assessments up to 48 h post dose and subsequent follow up. The primary objective is safety an d tolerability and plasma PK of SCD -153 will also be studied. This will be followed by multiple ascending dose study.
Slide 40 is the last slide on SCD -153. We are now evaluating the potential for SCD-153 in another autoimmune skin disease: vitiligo. Vitil igo is a condition where skin pigment is lost, and white patches develop. The chemokines CXCL9, 10 and 11 which we discussed just a couple of minutes ago, also attract the vitiligo -causing immune T cells to the site in the epidermis. These then secrete inf lammatory cytokines that target and destroy the melanocytes which are the pigment cells. Inhibition of this cycle by SCD -153 suggests that the drug has potential in this disease as well. Vitiligo also causes a large psychological burden for the patient and available options are few. So, we are in the process of testing this possibility in preclinical models.
To recap, we have shown that topically applied SCD -153 promotes hair growth in a spontaneous immune mouse model of Alopecia Areata, and SCD-153 inhibit s inflammatory cytokine gene expression levels in the underlying skin of these mice. An IND has been filed with t

he DCGI, and the
Phase 1 study will soon commence in India with results due by the middle of CY2024.
I will now hand over to my colleague Dr Ni tin Damle who will share an update on our work in the Anti -body drug conjugates area. Nitin…
Dr. Nitin Damle: A very good afternoon to you all. I am Nitin Damle and I oversee the Biologics function at SPARC. I will be providing update on SPARC's first an tibody -drug conjugate, SBO -154. Antibody -drug conjugate (ADC) field has exploded in the last 6 years during which 11 novel ADCs were approved by the US FDA as meaningful treatment options for cancer patients. As a result, the ADC

strategy has blossomed into a commercially viable therapeutic strategy for

strategic investments and the ADC market is projected to cross $25 billion in sales by 2028, with 24% increase in the compound annual growth rate during the next 5 years as shown on the slide 42.
During the last investors' call, we had introduced SBO -154 as the first biologic therapeutic from SPARC and described preclinically its antitumor therapeutic potential. SBO -154 is a humanized antibody drug conjugate of microtubule disrupting cytotoxic payload c apable of potently inhibiting growth of actively dividing tumor cells. SBO -154 is designed to bind with high affinity to a broadly expressed tumor -associated antigen, MUC1. The cytotoxic payload and its linker, both have been clinically validated and are represented in 5 already marketed ADCs.
MUC1, the tumor associated antigen target, is synthesized as a single protein that undergoes autocatalytic proteolysis into MUC1 alpha and MUC1 beta prior to its display on the cell surface as a heterodimer of MUC1 alpha and MUC1 beta subunits. While MUC1 alpha is entirely extracellular in its display, it is tightly but noncovalently bound to the transmembrane MUC1 beta subunit that holds the entire MUC1 protein on the cell surface. The region in MUC1 where MUC1 alpha binds to MUC1 beta is recognized as the SEA domain as shown in the MUC1 cartoon on slide 43. MUC1 alpha subunit includes a variable number of tandem repeats (VNTR) of a 20 amino acid long peptide sequence against which a large number of monoclonal antibodies had been made and evaluated as na ked antibodies, radio - immunotherapeutics or ADCs during the past 25 years or so.
None of these therapeutics managed to provide meaningful clinical benefit in cancer patients, in a large part believed to be due to the presence of cell - free or shed MUC1 al pha antigen both in circulation and also in the intratumoral compartment where it could effectively compete with MUC1 antigen on the tumor cell surface, intercept the anti -MUC1 antibody therapeutics, and thus denying them the opportunity to bind to tumor -

associated MUC1 to cause therapeutic activity. Our ADC, SBO -154, is a high affinity binder to the cell membrane proximal SEA domain of MUC1, and shown as circled in the MUC1 cartoon on slide 43. Unlike MUC1 alpha domain containing VNTRs, the SEA domain o f MUC1 is not actively shed or shed in very small quantities so as not be able to compete with cellular MUC1 SEA and intercept the MUC1 SEA -targeted therapeutic entities such as SBO -154. We have further confirmed that sera from cancer patients show minima l presence of MUC1 SEA while continuing to show the sizable presence of MUC1 VNTR antigen. Hence the shed MUC1 SEA is far less likely to interfere with the binding of SBO -154 to MUC1 expressing cancer cells.
The next slide, slide 44, shows the relative strength of antitumor activity of SBO-154 evaluated against large xenografts of human carcinomas expressing different levels of MUC1 SEA on their cell surface. Whenever the expression of MUC1 SEA is high, SBO -154 is able to cause regression of pre - existin g tumor xenografts. On the other hand, low MUC1 SEA expressing xenografts, although showed appreciable gr

owth inhibition or disease
stabilization, these xenografts failed to regress upon treatment with SBO - 154. In contrast, an isotype -matched nonbinding control ADC of CD20 - specific rituximab failed to show any growth inhibition in these evaluations. Thus, the antitumor benefit conferred by SBO -154 is preferential towards high MUC1 SEA antigen expressing tumors.
Our own immunohistochemical assessment of the expression of MUC1 SEA in patient -derived tumor biopsies and human tumor tissue microarrays suggests high level expression of MUC1 SEA in a wide variety of carcinomas, and thus, given opportunity, SBO -154 may be able to provide meaningful therapeutic activity in cancers with high expression of MUC1 SEA.
The SBO -154 program is currently in the preclinical development phase with the focus on manufacturing of the targeting antibody and the payload linker, and further the assembly of the ADC, SBO -154. We hope file the IND with the US FDA in the second half of calendar year 2024. While we

advance this program through its preclinical development, as shown on Slide 45, we have engaged the US FDA via the INTERACT meeting to seek

guidance for various IND -enabl ing preclinical development activities as a
prelude to the formal Pre -IND meeting in 2024 ahead of the IND submission.
We are looking forward to the FDA response sometime before the end of
this month.
I shall now hand over the discussion to SPARC's CFO, Chetan Rajpara.
Chetan…….
Chetan Rajpara: Thank you, Dr. Damle.
Good evening, everyone. This is Chetan Rajpara, CFO at SPARC.
I plan to go over the SPARC financials and cash position, at a high level.
Slide No. 47 please…
During FY23, Total Income was at Rs. 250 Cr (USD 31.1 MN), while Total
Expenses were at Rs. 472 Cr (USD 58.8 MN), resulting in to a Net Loss of Rs.
223 Cr (USD 27.7 MN). FY23 income was higher compared to FY22 on account
of upfront and milestone payments received for out -licensing SEZABY and
higher royalties on certain products.
Let me update you on our financial results for the first quarter of FY24.
For Q1 -FY24, Total Income was at Rs. 34 Cr (USD 4.2 MN), while Total
Expenses were at Rs. 129 Cr (USD 15.8 MN), resulting in to a Net Loss of Rs.
95 Cr (USD 11.6 MN).
Slide no. 48 please…
As you may be aware, the Company has raised Rs. 1,112 Cr (~USD 148 MN) in
July 2021, by way of a preferential issue of convertible warrants.
The Company has received Rs. 703 Cr (~USD 93 MN) in Jan -2023 agai nst the
conversion of all warrants by investors. With this, the entire proceed of the
Preferential Issue (i.e. Rs. 1,112 Cr) stands received.
Cash and cash equivalents as at September 30, 2023 was Rs. 363 Cr (USD
44MN).

The Company has sanctioned bank faci lities for Rs. 175 Cr (~USD 21 MN) in
place, in addition to a line of credit for Rs. 250 Cr (~USD 30 MN) from the
parent company. Utilization of limits as at September 30, 2023 is NIL.
The Company has obtained shareholders' approval at the last AGM for rai sing
a sum up to Rs. 1,800 Cr (~USD 220 MN) by way of fresh issuance of securities.
For FY24, approx. 30% of the expenses are budgeted for the clinical costs. We
are aggressively managing our costs and working to control non -clinical
expenses.
That's all from me today on the financial update. A big thanks to all for joining
the call. I will now hand over the call to Jaydeep for facilitating the Q&A.
Jaydeep Issrani: Thank you Chetan, we will now open the call for Q&A.
Moderator: We'll now begin the question -and-answer session. The first question is from
the line of Ketan Gandhi from Gandhi Securities. Please go ahead.
Ketan Gandhi : So, on Slide 9, can you please help us understand Vodabatinib in CML
recalibrating to a changing regulatory and market landscape?
Anil Raghavan: I think what this refers to is a couple of significant changes in the way FDA is
reviewing cli

nical programs in oncology . I mean , if you go back to the
prevailing paradigm in oncology for many years, the first re gistration window
for a program is usually the last line setting. You have to basically test a
compound in the last line setting. FDA is now for many classes actively
encouraging sponsors to go to earlier lines through comparative trials as
against an open label study in the last line setting. And this initiative is called
the project "Frontrunner." S o that is one important regulatory change which
has consequences or implications for our program. The second significant
paradigm shift is the way in which the agency looks at the acceptable doses.
In the earlier parad igm, FDA always allowed, going up to the maximum
tolerated dose and then coming down a step from maximum tolerated dose
and using that as a dose for clinical exploration of the program. But now
they're looking for at least some randomized data to identify the minimally
efficacious dose. And this is coming from recent experience across several
classes where approved products then manif esting significant level of toxicity

in clinical practice. So , both these as implications for us in terms of the path
that we are following based on earlier set of recommendations on the agency
in a prior consultation and what the agency would like us to d o at this point

and that is the reset that we are alluding to.
Ketan Gandhi: So, to understand it better, it will cost us more and it will cost us more time also, is my understanding, right?
Anil Raghavan: Let me take a step back . See what happens is two -fold. One, the current expectation for a registrational program in Vodabatinib is completing a last line study. And that last line study may require fewer patients, but they are far more difficult to recruit patients . The earlier line study would require more patients, but may be easier to recruit cohort. As I said in my comments, we are evaluating the cost and timeline implications and that will become a decision point post PROSEEK . Once we have results from PROSEEK whether we will pursue CML, is a strategic decision that we need to take and we will also decide whether we pursue this alone as ourselves or in partnership . I also want to highlight one other aspect of this change , that is , this will be a comparative study as in there will be an active compar ator in this program as against an open label study where there is no comparator . So, in that sense, there is an opportunity for the cost to go up for sure. The ti meline implications will need to be assessed based on the design.
Moderator: Next question is from the line of Ishita Jain from Ashika Stock Broking. Please go ahead.
Ishita Jain : So, my first question is on the PROSEEK trial. In terms of the primary endpoint, we're measuring change in baseline on the MDS UPDRS scale. Can you quantify that ? How many points of change would take us succes sfully to the finish line?
Anil Raghavan: I think Dr. Siu-Long Yao can give a little bit more detail on the MDS UPDRS part-3. But, if the question is about the design of the trial and what would be a meaningful change from a clinical perspective , the study was designed

against the n atural h istory data that we have from multiple natural history studies, particularly the MDS and Michael J. Fox Foundation study. So , in that setting, if you take part-2 plus part -3 together, it's a 6.5 -point deterioration over a nine -month period, which is the ba seline deterioration that we assume , and the 35% improvement of over that is deemed clinically significant. And that is proportionately split between part -2 and part -3 even though a larger component of that is part-2. So, in that sense from points perspective , four plus points in part-3 and around two in part-2. So , we still look for a 30% or 35% improvement on that trajectory, which is from our

consultations with KOL deemed clinically significant if that's the quest ion that you are asking.
Ishita Jain: Yes, absolutely. Thank you. My second question is slightly a broad -based question on PROSEEK. Inhib ikase Therapeutics for their drug have unblinded a part of their trial two weeks ago and there seems to be signs of efficacy. I think only for the highest dose 200 mg and they also got orphan drug designation for MSA which you know is also an alpha synucleop athy . So, I do understand that yo u perhaps cannot comment on a competitor, but strictly from a proof -of-concept point of view, can you draw some parallels with our c-Abl in hibitor, Vodobatinib ?
Anil Raghavan: I mean, they clearly claim a positive trend line for their study especially in the higher dose and that a ugurs well for our program. But we will take that with not just a pinch of salt, with a lot of apprehension because the sample sizes there are very, very low , I mean, this is 11 patient s data and if you look at the duration of treatment, it is a 12 -week duration and it is a known fact in the Parkinson's phase that the placebo response tapers only after six months, right . So, we expected to see some level of moderation of the trajectory in that earlier phase. So , we think tha t the overall treatment window and the patient size is inadequate to make the claims . And also, the l ower dose did not have an effect, but that is to be expected to a certain level because the blood brain penetration requires substantial peripheral concentration. So, in

that sense, that's reasonable . But if you look at the window for treatment and also the number of patients involved, I think it's too early to comment .
Ishita Jain: So perhaps a little far -fetched. Post our Phase -2 read out , what are we thinking in terms of Phase -3, how big would Phase -3 be? We have abou t 300 crores cash on books. Will there also be an agency meeting before we initiate protocol for Phase -3, and finally, will Phase -3 include L-Dopa or MAO

inhibitors patients as well, which is slightly more advanced stages of PD or are we sticking to early PD ?
Anil Raghavan : Our registration program for the initial indication will not have the baseline L-Dopa . We are allowing MAO -B in the protocol currently . We will have an end of Phase -2 discussion sometime in November 2024 timeframe based on our current projections . And in the end of Phase -2 meeting we will be going in with multiple possibilities, some of them are really positive and some of them in the worst case, we may require two more Phase -3s to get this into a registration depending on the data that we see and depending on the view the agency take s. But our final sign -off on our registrational study design can happen only after we have a consultation with the agencies, with the full data readout in August 2024 . But we will not be going into additional settings because if we introduce L-Dopa as a background therapy into this, that induces a significant level of variability of response. That is certainly an avenue for us to explore and pursue at an appropriate t ime because if we are bending the trajectory for the disease, we will definitely have an impact on later stages of disease, but that's a different program and our most important priority would be to replicate what we've done in PROSEEK and get this product to registration as soon as possible.
Ishita Jain: If you can just give an update on Phase -2 for Lewy body dementia ?
Anil Raghavan : Lewy body dementia program is a single center trial out of Georgetown. That's a very small trial , some of the issues that I spoke earlier include that the treatment period is small and also very few numbers of patients. And we are looking at that trial for biomarker guidance and we will have data around the

same time as PROSEEK , Georgetown had shut down their clinical trial activi

ty
for due to COVID. So , there were significant delays in that program . But from a proof -of-concept standpoint even for lewy body dementia, we think PROSEEK is a more relevant proof -of-concept because of both the mechanisms involved in both diseases and also a number of patients involved in PROSEEK trial. So, we will consider PROSEEK readout as a t rigger point or a stage gate for deciding on LBD.
Moderator: Next question is from the line of Chand pal Singh, individual investor. Please go ahead.
Chandpal Singh : I have just first one question regarding Vodobatinib for Parkinson's . Are we eligible for the disease modifying therapy drug ?
Anil Raghavan: Disease modifying therapy designation in the label is one thing . And actual disease modification from a clinical experience standpoint is different. Let me comment on both. So , disease modification or disease modifying therapy designation on the label may require a different design , in the sense that you may probably need to have a delayed start design which will further delay the program . So, that is something which we may not explore on day one and we may try to add that later and we will also have a consultation with the agency in terms of what we are seeing . If you look at the nature of dat a that we will have both in terms of the actual trajectory of deterioration and also the time to introduction of symptomatic therapy will give practical data and guidance on whether the drug is modifying with disease trajectory ? So, we hope to have a conversation with the agency in terms of disease modifying nature of the program and make the case for including disease modification designation in our label with this design. But if FDA has different views on that, then we will go ahead with the regist ration as a therapy in this disease before we pursue the disease modification.
Moderator: Next question is from the line of Jigar Valia from OHM Group. Please go ahead.

Jigar Valia : My question pertains to Vodobatinib. We have these catalyst events coming up in Q1 , Q3. Just a clarification that this certainly would not affect any optionalit ies on LBD or even further on CML ?
Anil Raghavan: I didn't fully follow the question. Can you repeat the question, please?
Jigar Valia: So, with regards to the catalyst events coming up for Vodobatinib for PD, that shouldn't affect the optionaliti es with regards to the LBD indications or even with regards to the CML which is again with regards to same mod ality?
Anil Raghavan: Well, it will , I mean, going back to my earlier response to Ishita , MSA and LBD

essentially will depend on a successful completion of PRO SEEK, in the sense that we are looking at PROSEEK as a definitive proof -of-concept for diseases driven by alpha synuclein. So , if there is a negative readout on PROSEEK, then it is a negative proof -of-concept for not only Parkinson's disease, but also for the mechanism more broadly , and that means across these diseases, like MSA and lewy body dementia. It is n ot going to affect CML because in CML we already have clinical proof -of-concept as S iu explained in his slide deck. And we see CML as a hedge even though a lower value hedge against the failure of the neurodegeneration hypothesis.
Jigar Valia: Also , similarly with regards to SCD -044, wo uld psoria sis or atopic dermatitis be either /or, scenarios as a hedge for dermatitis ?
Anil Raghavan: No, we will be driven by the data from their Phase -2 programs and as Sun Pharma is our commercialization partner Sun has the rights to take those decisions. And I'm pretty sure they will take appropriate decisions based on the data that they're seeing in these programs and the competitive landscape of these areas. So , I don't thin k we can give a generic response about whether it is going to be either /or at this point without actually seeing the outcome from the Phase -2 programs which wi

ll be coming soon.
Jigar Valia: Lastly , in terms of the incremental one run rate for the coming y ear, $60 million could be more ?

Anil Raghavan: So, you see, next year calendar year '24 or Financial Year ‘ 25 will be a true discontinuity from a trending standpoint for the burn rate because we have been having a consistent burn rate for the last few years we've been pursuing , in a certain number of clinical programs and there is a certain level of predictability to that spend . But depending on what we do with Vodobatinib and depending on how some of these early program’s scale and also depend ing on the partnering strategy in terms of how much development is retained within SPARC . There is a potential for bigger swing in development spend next year. So , unfortunately , I'm not in a position to give you a trend guidance on operating costs for nex t year , because we are going to have significant dependence and variability on some of the data readouts.
Jigar Valia: Any timelines for the site transfer for Elepsia , Xelpros ?
Anil Raghavan: In the case of Elepsia , we're looking for a site transfer in the next financial year . Xelpros hasn't gone out of supply at this point. So , we are still waiting for an actual plan from Sun in terms of transitioning that to a different plant. So, we don't have definitive guidance on Xelpros . But in Elepsia , we are looking at as in Financial Year ‘ 25 as a target for tech transfer.
Moderator: Next question is from the line o f Manish Jain from GormalOne . Please go ahead.
Manish Jain : Really delighted to see the kind of biologics capabilities that have been built up, and ADC already really gunning for IND. So, I would say hats off to the team and I had three questions. The first one was , can't we pursue Vodobatinib both for PD and CML if we license it to two separate people altogether ?
Anil Raghavan: This is a n interesting question in terms of pursuing PD and CML together . There are two or three complicating factors here. One is obviously the commercial landscape and the price implications. If you take late-stage cancer and t he price ranges usually runs to several hundred thousands of dollars per year per patient. But Parkinson's and the neurodegenerative diseases

spectrum being significantly large disease burden with significantly larger number of patients under management, per patient , per year pricing tends to be significantly lower . So, if you have a product even at a high dose, as in the Parkinson's dose is going to be higher than the CML dose with much lower price hence the substitution risk for someone who is commercializing CML is quite high. Two, there are safety profile implications , in the sense that patients who are going to come to CML with this drug are significantly sicker than patients who you see in early -stage Parkinson's. So , a lot of commercial partners will have trouble reconciling the fact that they have to d eal with the complex safety profile, which carries some of the encumbrances of late-stage disease in ca ncer i n trying to market a drug in early -stage Parkinson's disease and in competing with programs which are not really encumbered with that kind of issues in l eukemia. So, there are some practical issues in terms of doing it both in terms of the cannibalization risk and also safety profile .

Manish Jain: My second question was pertaining to SEZAB Y that is Phenobarbital . So, where are we going to manufacture it?

Anil Raghavan: So, we are in the process of inducting one more manufacturing plant. Currently , it's supplied out of one of the Sun’s facilities . And the second facility our partner is now inducting is a third -party outside of the Sun network. So , as we work towards enforcing market exclusivity, we will have two sources one from the Sun network and one from outside .

Manish Jain: Related to PDP -716, in terms of alternat

e API partner in how much time do we think before we can respond to the CRL?

Anil Raghavan: So, we have already identified an alternative manufacturing partner and we are in the process of initiating the exhibit batches with alternative partners ’ API. It would require significant stability data which is from a timeline standpoint, and we expect it to happen towards the end of the second half of next financial year.

Moderator: Next question is from the l ine of Ishita Jain from Ashika Stock Broking . Please go ahead.

Ishita Jain : So, moving away from PROSEEK , on PDP -716, the one with Visio x, I think that you already answered the timeline , so Visio x also in -licensed Omlonti, which is for the same indication as PDP -716. And since SPARC is a shareholder of Visio x, can you give some color on how has the ramp up or the scale up been from Omlonti ?

Anil Raghavan: No, we cannot comment on that program in this call , I mean, we would like to stay with the SPARC program , not qualified or equipped to comment on Visiox or other program s, sorry, Ishita.

Ishita Jain: On Vibolizimod which is with Sun… I have a feeling you won't be able to answer this question either , but can you give us some color to better understand the terms of contract , so we can understand what is the payout especially in calendar year 2024, which is we're expecting a clinical proof -of-concept ?

Anil Raghavan: As you anticipated , we are not in a position to say. We haven't disclosed those terms . What I can say at this point is, the read out is a milestone event for us , and we also have substantial royalties expected from this program once we cross data thresholds in Phase -3 into a registration.

Moderator: Next follow up question is from the line of Manish Jain from Gormal One. Please go ahead.

Manish Jain : On the ADCs, we have highlighted one or two indications . At what stage post IND can we look at running three, four or five , six multiple indications ?

Anil Raghavan: I think the early -stage dose escalations in a program like antibody drug conjugates would have patien ts from multiple indications. So , we are clearly looking ahead here, because these are matters which are under discussion and finalization at this point. But we will look for cer tain threshold of expression of MUC -1. We will have a cut off range and we think that we can

get patients from across three, four tumor types , you have possibly breast cancer, lung cancer and a few other . And there is a possibility to do this more as a basket trial, as against going up in one program or one entity.

Moderator: Next follow up question is from the line of Ishita Jain from Ashika Stock Broking. Please go ahead.

Ishita Jain : So just one last question on PROSEEK . I think I missed asking this. We have had six DSMB meetings . Were they all 100% green light to go ahead on both dosages ?

Anil Raghavan: Yes, we specifically asked if they would like us to change anything in terms of the doses used or the design or inclus ion, exclusion criteria . They didn't want to change anything about the design or the dosages used.

Ishita Jain: I also noticed that I think last month we set up a wholly owned subsidiary called SPARC Life in the US . Can you comment on the goal to create th is, I mean apart from monitoring clinical trials in the US, can this subsidiary someday become somewhat of a front end?

Anil Raghavan: I don't want to get ahead of myself here. I mean, we have an operation in the US, including Dr. Siu and others who spearh ead our clinical activity. And I think from a regulatory and governance standpoint, it's cleaner to have them in a subsidiary . And we also have a branch structure. At the moment , it is a part of a cleanup process, but that would become an operating entity for us in the US and if we scale into a late-stage development program for Vodobatinib that wo

uld become meaningful arm for us .

Moderator: Next question is from the line of Tushar Bo hra from MK Ventures. Please go ahead.

Tushar Bohra : Sir, this is regarding the PROSEEK outcomes. We mentioned that we saw potential collaborations or outlicensing opportunities or something on those lines . If you can highlight a bit more details wh o are the players we are

approaching , what kind of interest we are s eeing , anything around that will be useful ?

Anil Raghavan: I won't be able to dis close who are the players that we are talking to , because we are governed by fairly strict confidentiality commitments on these conversations. But , what I can say is that right thr ough this Phase -2 program, there have been consistent interest from larger pharmaceutical companies and we will initiate a process as we go towards data flow from this program to see what kind of partnership structures are possible , and we will also explore what is probably a smart way to split the value between us and a potential partner depending on where we actually land with the data. But I'm not able to give you a specific answer in terms of names that you're looking for.

Tushar B ohra: I was looking for number of players and the kind of players we are discussing with , what kind of companies , whether you -?

Anil Raghavan: So, we are looking for at large commercial players, I mean , bigger pharma companies with the commercial footprin t in the CN S space or the larger companies with strategic intent in CN S space and we have a fairly significant field which is interest ed.

Tushar Bohra: Second, if we were to proceed to a full -fledged Phase -3 clinical trial assuming that we need one more tr ial for this after PROSEEK , is there an estimate as to how much resources we would need , and what could be the timelines possibly for this?

Anil Raghavan: So, it clearly a function of what is the extent of pursuit that we can have, right , and what is the regulatory expectation . If we 're going to have one more clinical program , then we are talking about something in the range of $50, $60 million to do that one more clinical trial. And if we need to have two more, then we are talking in the range of something in excess of $100 million. But these are all very exploratory numbers. It's a larger question and probably a driver for collaboration is pursuing this program more fully, I mean we have

gone to some length to describe some of the possibi lities both in late phase disease and also through prodromal disease , all the possibilities in the next ring of opportunity which is MSA and things like lewy body dementia. So , if you treat PROSEEK as a proof -of-concept for synucle opathy , then how aggressively you pursue this both the inner core of opportunity and also the peripheral set of opportunities is going to determine what is the kind of partnership or resourcing that is required and that will be a major consideration going into .

Tushar Bohra: You mentioned that in the most conservative case , we could maybe need two more trials, but what about the most optimistic scenario given the safety profile of Vodobatinib which is clearly at least as per the data upto now , clearly very, very strong as well as the efficacy part, what is the most optimistic scenario that you guys are hoping for or are building in as one of the probabilistic scenarios?

Anil Raghavan: I think we will be getting to a speculative realm at that point. I don't want to get into that issue with this group , because at this point, we did not have any such conversation with the agency about the future of this program. So , anything that I say is going to be based on my read of what is possible and what is not possible and that could be misleading. I don't want to be misled by what I can say at this point.

Tushar Bohra: Maybe I'll rephrase the question. What are the possible scenarios a

fter
PROSEEK depending on the data , what are the pathways available from a
regulatory standpoint and whichever one may happen?
Anil Raghavan: See, PROSEEK is an unusually large Phase -2 program. You have 504 evaluable
patients. You have clinical endpoint which is what FD A has advocated for this
disease indication for a long time and we will also have some significant
biomarker data depending on the nature of the samples that like C SF or
biopsies or blood and we will have fairly large numbers in some cases and
smaller numbers in some cases. So , this is a substantial data set. How FDA
would a ctually look at this and how FDA would look at the disease is to be

seen , because we don't have too many pre cedents in Parkinson's disease for
neuroprotective therapi es coming to a registrational path . FDA ’s CNS division
has been fairly progressive in deal ing with indications like Alzheimer's and
ALS. If you want to look for the most optimistic scenario, my guidance would
be to take a look at what happened in ALS and in AD recently. There are three
approvals in AD and ALS. In the most optimistic scenario that can become a
guidance benchmark for us with the agency . But I'm not saying that because
there is no precedent and the con tours of this disease is somewhat different
to like ALS and AD . So, I think any speculation on what is possible is of limited
value because of these differences in my mind.
Tushar Bohra: If you can highlight some of the newer drugs that you are either looking to in
license or are at early stages of development . You said in the beginning of the
call that you're looking at multiple moda lities extending into new er
modalities. If you can just highlight a little bit more around that ?
Anil Raghavan: If I look at our trajectory, we have a historical focus in smaller molecules , and
we have added biologics as a competency in the last two -three years. And
there are you know different possibilities with that broader area, antibodies
drug conjugates which has traditional cytotoxin, which is essentially much of
the ADC field at this point. But we think that ADC will shift substantially with
other kinds of payloads like other targeted therapies and even other
modalities . I don't want to specu late there, but even things like RNA
therapeutics are all possibilities coming from the ADC kind of constructs. Plus ,
you will see bispecific or multispecific structures, both bispecific or
multispecific standalone antibodies plus bispecific or multispecif ic ADC
constructs , try to improve target expression by adding in another specificity.
So, there are different possibilities there . And clearly one other possibility
given our strengths in small molecule is small molecule directed on cytotoxin
ADC . And then you can use a small molecule of synthetic ligand to target
specific cancer cells, and that's something which we are definitely doing, and
we have a program in consultation with UCS F, I think one of the slides have

refer ence to that . We already have a prog ram in fairly late-stage preclinical
program, and we will have new s flow from that program going forward.
Moderator: As there are no further questions, I now hand the conference over to Mr.
Jaydeep Issrani for closing comments.
Jaydeep Issrani : Thank you, Neerav, and thank you everyone for being on the call today. In
case you have any additional questions, feel free to reach out to us. We'll be
available to answer your questions o ver e-mail or through discussion s. Thank
you again for being on call today.
Anil Raghavan: Thank you very much.
Moderator: On behalf of SPARC , that concludes this conference. Thank you for joining us.
You may now disconnect your lines.

Call: Jan 2024

SPARC/Sec/SE/202 3-24/080 January 15, 202 4

National Stock Exchange of India Ltd.,
Exchange Plaza, 5th Floor,
Plot No. C/1, G Block,
Bandra Kurla Complex,
Bandra (East), Mumbai – 400 051. BSE Limited,
Market Operations Dept.
P. J. Towers,
Dalal Street,
Mumbai - 400 001.

Scrip Code: NSE: SPARC; BSE: 532872

Dear Sir/Madam,

Sub: Transcript of the Investor’s call on PROSEEK Program held on January 6, 2024

Please find attached herewith a copy of the transcript of the Investor’s Call on PROSEEK Program held on January 6, 2024

The aforesaid transcript shall also be available on the website of the Company.

Yours faithfully,
For Sun Pharma Advanced Research Company Ltd.

Kajal Damania
Company Secretary & Compliance Officer

 “Sun Pharma Advanced Research Company Ltd. (SPARC)
Transcript of Investor Call held on January 06, 2024
Update on PROSEEK Program”
MANAGEMENT : MR. ANIL RAGHAVAN – CHIEF EXECUTIVE OFFICER
MR. JAYDEEP ISSRANI – HEAD BUSINESS DEVELOPMENT , CORPORATE COMMUNICATIONS & INVESTOR RELATIONS

Moderator: Ladies and gentlemen, good day and welcome to SPARC Conference Call. As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes . Should you need assistance during the conference call, please signal an operator by pressing star, then zero on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Jaydeep I ssrani. Thank you and over to you , sir.
Jaydeep Is srani: Thank you, Michelle. Good evening, ladies and gentlemen. My name is Jaydeep Is srani, I head the Business Development and Investor Relations at SPARC . On behalf of SPARC , I welcome you to today's call and appreciate you for taking the time out on a Saturday evening to attend the call.
I'm joined by our CEO, Mr. Anil Raghavan and the senior management team at SPARC .
Anil will walk you through the presentation, which we have shared earlier .
And after his presentation, we will open the call for questions.
Before we start, I would like to remind you that our discussion today includes forward -looking statements that are subject to risks and uncertainties associated with our business, hence the actual results may be different from those projected in the presen tation today. I will now hand it over to Mr. Anil Raghavan for his presentation. Over to you, Anil.
Anil Raghavan: Thank you, Ja ydeep . Good evening, everybody. Good morning or good afternoon if you are joining internationally. Thank you for taking this cal l on short notice.
I have four key objectives for this call . I wanted to provide an update on the Vodobatinib Program . I want to set up our immediate priorit ies going in 2025-2025 financial year for Vod obatinib particularly after the data events that

we're expecting. And also want to provide some perspective in terms of how to look at this program in balance d way. We've spoken about the opportunity

extensively in our investor call earlier this year. Also , want to give a sense of some of the risks in the program that are or customary to this kind of program s. Finally, and probably most importantly, this is an op portunity to answer any questions that you may have before we go into a quiet period on the interim analysis.

So, with that, let's move to Slide 3. We have a recap of the status of the program. We have touched upon some of this in our earlier call . Vodobatinib PoC program, it's called PROSEEK completed its enrollment target last year around October of 2023. Our targe t was 506 evaluable patients , we ended up with 513 patients globally. And the interim analysis as communicated earlier is planned with 85 %- 86% of patients. That's the enrollment cut off was May of 2023. So , we have 441 patients going into this interim analysis and that is planned for late March -early April of 2024. A s we communicated earlier, the broader organization will be bl inded , not just a broader organization, the external ecosystem or investigat ors and others who are participating in the trial will also be blinded because we' ll still have around 70 -odd patients in different stages of treatment and protecting the in tegrity of status in those patients is an important consideration for the trial and also for the regulators in different countries. In that sense, we are committed to maintaining the blind and protecting this information to ensure that we don't in duce unnecessary bias in conduct of the trial .

The design of the program we have covered that in many earlier conversations, but just to give a very brief overview, it has two doses of Vodobatinib 168 patient s in each arm 384 m g on the top dose and 192 m g on the lower dose against the placebo arm. As we have spoken about in the past , MDS-UPDRS Part-III is the primary endpoint . We have a host of secondary endpoints and biomarke

r s which are mostly exploratory . And th e study also has Part -II long -term extension study, where patients are on the drug for another forty weeks , that's roughly around 10 -months. So , the patients on Part-A and Part -B combined are for almost 80 -weeks and those patients go through this 80-week s period without any symptomatic therapy. So , that's the

design of the program and I have already talked about some of the milestones like interim analysis in April . The full top line data for PROSEEK is expected in August -September of 2024, that's when we will have full disclosure on the data.

Now , moving on to the next slide , has a listing of our nea r-term priorities post availability of data from PROSEEK . The primary objective in 2024 -25 financial year is to ensure that we move on to the next phase of development without a phase lag if we're fortunate to have positive data in line with expectations of the hypothesis . That requires several steps, most importantly , an agreement with the regulatory agencies globally in terms of the nature of registrational studies required. So , that would happen during the end of Phase -II consultations with US FDA and other important regulatory agencies globally. And we hope to do that in short order after the August -September data readout .

And in parallel, we will be continuing the long -term extension s tudy of PROSEEK . And as soon as we have clarity in terms of th e nature of the registrational studies required, we hope to initiate the pivotal Phase -III programs globally and our strategic intent would be to minimize that lag between end of Phase -II and initiation of Phase -III. And we will also be using this time right from the availability of inter im analysis data to explore and execute a partnering strategy so that as we move into late stage development post PROSEEK we can do that with a partner who we may be going with for commercialization of the asset.

And as I said , the regulatory agreement is an important element in triggering

this and it is not just about additional clinical studies required , we may also require additional pre -clinical studies, particularly toxicity studies , and also additional steps to be t aken for ensuring manufacturing our business, particularly given the size and potential of this drug.
So, let's go to the next slide , when we met you in the last quarter of 2023 for our annual investor ’s update, we have spoken extensively about the program,

the potential of the hypothesis and its implications on the standard -of-care in Parkinson's disease and also more broadly about the possibilities of diseases driven by alpha synuclein and in the outer r ung of possibility , diseases that are driven by other proteins impacted by oxidative stress and A bl-linked activation.
We have spoken about the opportunity clearly . We thought that it is important given some of the communication and coverage that we are se eing on this drug and recently it's important to also highlight some of the risk s which are inherent in programs like these, especially in trans lation programs in neuroscience. So , we want to highlight three -four major risks that you may want to keep in mi nd as you kind of think about these pro grams.
One is the translatability of animal models in diseases like this which we have seen . These animal models built with inten t to mimic the underlying mechanism and create the manifestation of disease which can be addressed with the drug . While they are fine from a science standpoint, their true validation will come , when programs which are developed with these models go on to clinic and get validating clinical data and go into market .
In diseases like Parkinson's , we haven't seen significant clinical translation in the long -term, especially in disease -modifying therapies which are designed to bend the ne urodegenerative arc. So,

we are one of the early companies
which is trying to translate the oxidative stress pathway and therefore these models carry a certain level of risk and it is important to keep that in mind as you evaluate this program.
A couple of other risk s that I want to highlight :
One is about target engagement and dose. If you've been following this program, we have used the top dos e in our animal model that is 45 mpk as a marker for deciding our do se for the Phase -II clinical program. The ma x brain exposure associated with the top dose in animal studies which is the most efficacious dose in animal stud y. And in early clinical studies , we matched that

exposure in Parkinson's patients C SF, which we believe is a good proxy for levels in the bra in. So even though the translation process is sound , this field clearly lacks a tar get engagement marker because we cannot open the brain and see what happens. So, there are challenges in terms of clearly getting appropriate doses in humans. So , there is a certain level of approximation that is done by extrapolating animal data that may create certain level of risk which needs to be factored in.
The third point that I want to highlight is the reproducibility of early clinical proof -of-concept studies as you t hink about repeating dose results in later stage clinical setting. And ty pically, if you look at companies which are doing early proof -of-concept studies and they're more smaller pilot studies and much of this risk comes from inadequate powering of early studies. And that we have tried to a ddress some of that in the design of t he PROSEEK trial.
PROSEEK is the Phase -II program. It is an extensive study with 500 -plus patients which is powered at around 80%. So , in that sense , even though we have tried to address some of the trans lation risk i.e. some of the risks associated with reproducibility of early -stage clinical results in late stage clinical programs, that is still a risk in the sense that it needs to be reproduced in the larger s tudy in Phase -III setting.
And the last point is which will require extensive additional work in terms of Phase -III programs and resourcing that and actually executing that in a timely manner .

We have also in the earlier presentations talk ed about the opportunities for Vodobatinib beyond Parkinsons disease and that would require additional preclinical and clinical work and we are in the process of doing some of that. These are mainly in two buckets , which are driven by alpha synuclein like lewy body dementia and MSA .
So, I was talking about additional indications fo r Vodobatinib which would require additional preclinical studies and clinical studies and some of those studies are currently under way. You may know that we have been working

with Georgeto wn in investigat or-initiated trial to explore this program in smalle r proof -of-concept study for lewy body dementia. We're also working in early stage preclinical work in Alzheimer's disease with this mechanism with academic investigators. So, there is additional work that needs to be done to fully explore the potential of this program in other indications and other diseases, but also in other settings in Parkinson's. If you look at the design of Vodobatinib PROSEEK trial , we're looking at earl y stage Parkinson 's patients who are pre -symptomatic that those are patients who are very early in their disease process and before they get into symptomatic therapies like L-Dopa. If you demonstrate disease modification or bending of the neurodegenerative arc in th e setting, clearly, there is justification for exploring this program in other settings in Parkinson 's disease like the combination of symptomatic therapy . Both these settings that would require additional studies.
Success on PROSEEK w ould be the beginning of an ex clusive journey, which would then valid

ate the program in the early setting, it should be our first registration al program, but also it will initiate a significant number of opportunities outside, but all of that would require an additio nal investment and additional exploration from a pre-clinical and clinical standpoint.
That takes me to my last slide. I just wanted to highlight a few market -related risks here in this slide . We have seen in a recent coverage on Vodobatinib which in our v iew captures some aspects of the program not all aspects of the program comprehensively. So , for investors who intend to price in Vodobatinib 's potential to their decisions and deliberate analysis of the potential of the program both from a sales standpoin t and also extension into other possibilit y stand point along with an understanding of cause and risk, and also time -to-market, it's important before you take those calls . In a setting like this early -stage biotechs go into data events like th ese, there is significant risk of price volatility. And that is somewhat magnified in the market like in the absence of informed analyst coverage, which looks at these programs carefully . So, when you look at reporting that is coming in, we urge

you to take a balance d view on the program which balances both the potential of the program as well as the risk of the program.
And finally, I want to reaffirm our commitment to this area. We're going to see significant data coming from this program i n the rest of this year with interim analysis and also in the final analysis . That's going to teach us a lot about c-Abl’s role in neurodegeneration and teach us about how the moderation of stress path way is going to have a role in treating neurodegenerative disease s. And there is a significant number of possibilities in th is data that you can have clear validation of this hypothesis. You can also have a clear debunking of this hypothesis, but there's also a lot of gray space in between. So, this data we're looking forward to contex tualiz e in what we're going to learn and then finding ways to move forward with continuing exploration of this program . Not just in this area as in this hypothesis , we also want to reaffirm our commitment in the neuro degenerative disease. In fact, if we go back several years that was the bet that we have taken in that neurode generation in spite of conventional wisdom going the other way, we believe that the ne urode generation is an area is maturing from a science standpoint and that has also been valida ted recently with several transactions

happening in this space. So , we will continue to be interested , continue to be excited about making a dent on these difficult di seases in the neurodegenerative spectrum . So, with that, I will conclude my comments and open up this call for the questions you may have. Thank you very much again for attending this call on short notice.
Moderator: We will now begin the question -and-answer session. We'll take the first question from the line of Ketan Gandhi from Gandhi Securities. Please go ahead.
Ketan Gandhi : In the November 2nd presentation , you indicated on Slide 19 that approximately 87% of the eligible patient s enrolled in part-II from part -I. However, in current presentation , there is no update. Do you have any up date on that , sir?

Anil Raghavan: That is true , 85% to 87% of eligible patients who are completing part -I of the study a re eligible for continuation into a phase and long -term extension study . We don't have additional data . That trend continues.
Ketan Gand hi: Do you interpret the rollover rate to part-2 as indicative of patients positively embracing the treatment or is it possibly as a result of patient not experiencing the desired improvement , what is your thought process on that?
Anil Raghavan: It's very difficult to conclusively say that because we have significan

t number
of patients on placebo . Even if they're not improving, the y may want to access the drug in the long -term extension settings. There are a lot of motivation that patients are fee ling better and they may want to continue . If patients feel that they're not feeling better, but they may be on placebo and they may want to transition to drug . So, there are significant number of different motivations that may be driving that conclusion. So , while it maybe encouraging, but it's difficult to draw any definitive conclusions based on that.
Ketan Gandhi : On Slide 5, under the section , “Expanding Evaluat ion of Vodobatinib ” it is mentioned that SPARC would explore initial registrat ion in an early treatment and n aive setting. Can you throw some light on regulatory pathway post the announcement of PROSEEK top line data in August '24, whether we would go for regulatory approval or we would go after Phase -III, can FDA help us in getting approval post EoP2?
Anil Raghavan: So first let me give you a little bit of context to this statement in this presentation, which would explore initial registration in early treatment naive setting . And that is the most logical way to plan a registration like for this program. The patients who are coming into PROSEEK are early -stage patients who are treatment naïve from L-Dopa and symptomatic standpoint. So , if you're getting a positive readout from that trial, the least risk option from a registration standpoint is to repeat those studies and reproduce those results, which would become the basis of registration. The intent of that statement is that early or the initial registration setting would be a repeat of what we have explored and studied in PROSEEK . Now , the second part of your question is

“in terms of what would be an actual registration program and regulatory ask,” that is subject to the discussions that we need to have with the regulatory agencies. We are going to have end of Phase -II conversation as soon as we have data . And there are very many possibilities. I mean, traditionally , the agencies would require two additional Phase -III studies and that's th e classical ask in these kinds of settings . But there are also other possibilities of like factoring into a registration al package in some form o r fashion, but that is all strategies that would require validation and agreement with the agencies around the world and we intend to do that as soon as we have data from the PROSEEK program. So, final shape on the registration al packa ge can only be clear after we have these discussions with the agency.
Ketan Gandhi : Is my understanding right that we will be starting partnership program between interim data analysis and the top line data between that period ?
Anil Raghavan: That is true. We expect to initiate conversations with the potential partners. We may not be able to conclude that before a final dat a disclosure in August

- September, but we intend to kind of use this time to engage and create interest and work towards a partnership as we kind of get to September.
Moderator: We'll take the next question from the line of Ishita Jain from Ashika Stock Broking . Please go ahead.
Ishita Jain : Hi, Anil . Thanks for the update. Appreciate this reiteration of the risks associated to drug development. My first question is so our enrolment concluded in October 2023 . Was the enrollment timeline as anticipated or did we face any challenges in recruiting?
Anil Raghavan: Your voice is a bit muffled. I couldn't hear you.
Ishita Jain: My question is that our enrollment concluded in October 2023 . Was the enrollment timeline as anticipated or did we face any challenges in recruiting?
Anil Raghavan: So, if you remember, we started this program even though technically in 2019 the actual dosing, started in early part of 2020. Exactly when health systems

around

the world started shutting down because of the pandemic . The initial couple of years have been rough for the program because most of the hospitals were not seeing patients in -person in hospital settings, and many of our endpoints require practitioners , assistant s at site . So, in that sense we clearly have seen slower than anticipated in the first part of th e study, but it has clearly picked up in the last 18 -months or so, which helped us to conclude this program , almost in line with the revised timeline. We did a revision of the time line during the COVID phase . So COVID induced certain delays. But other than that , in the last 18 -months to a couple of years , the program has been tracking to the plan.
Ishita Jain: What I'm trying to understand is that if we go into Phase -III, what would mean Phase -III in terms of phase and then enrollment concerns in Phase -III, is that significant , I mean , obviously not withstanding existing drug development concerns?
Anil Raghavan: I won't be able to comment on the actual size of the Phase -III program. There are a significant number of statistical considerations that will go into defining that size and it will also be informed by the data that we are seeing in PROSEEK in terms of the doses that are effective , what is a registration al dose that we want to carry and the effect size that we are seeing in the Phase -II setting . So, there are a lot of variables which can only be informed by data from the PROSEEK trial. I won't be able to clearly indicate the size of the Phase -III program. We expect to maintain the momentum that we had in the later part of PROSEEK in terms of recruitment going into the Phase -III program . Because we have relationships with the investigators across and the other service providers in this ecosystem, and we understand this space probably better than when we started off. So , I think we are confident that we can maintain the moment um that we had in the second half of this trial f rom an actual recruitment rate standpoint, but actual size of the program in terms of number of patients that would be required is a function of where we land with PROSEEK is difficult to extrapolate that now.

Ishita Jain: I think I'm not sure if you alrea dy mentioned it and did I miss it , but meeting with the agency would only be post final data, right , there would be no agency meeting with inter im data ?
Anil Raghavan: Yes, the agencies would require completion of the trial and data from the ITT population before we can have the end of Phase -II meeting.
Ishita Jain: You mentioned that we may require additional preclinical studies. What kind of studies would these be , I mean I know it would be around efficacy , concentration of drug in brain or I mean checking for a specific biomarker, what kind of additional preclinical studies would be required?
Anil Raghavan: So, there are several toxicity studies, which are standard expectations in the registration al package , to give you an example , carcinogenicity studies, is a standard expectation in the registration al package . And there may be additional clinical trials like drug -drug interaction studies or other toxicity studies i n preclinical settings. So , there are a part of customary expectations

in a registrational setting in both preclinical studies and certain additional clinical studies . Clinical studies are not major clinical studies, but they are the studies which explore safety endpoints.

Moderator: We will take the next question from the line of Bino Pat hiparampil from Elara Capital . Please go ahead.

Bino Pathiparampil: Could you please make some comments around the IP estate around Vodobatinib, do you have a drug substance patent , till how long does it run , etc.?

Anil Raghavan: I don't have the exact dates in front of me, but I can indicate that he comp ensation of matter and the regulator

y compensation for the development time, we will go into late second half of 30s from IP coverage and we may also have some additional patents which are covering this space in the method of treatment patent and also potentially formulation patent. So, w e are confident that we may go into late second half of 2030s for sure.

Moderator: We'll take the question from the line of Tu shar Bohra from MK Ventures. Please go ahead.

Tushar Bohra : Sir, just a couple of points. So , while the end of Phase -II discussions with the FDA would happen only after the final data, but t rying t o assume that with the interim data analysis, whatever the readouts that we would have , the same would be shared with FDA as well , and that may be useful in whenever we have the final discussions , this would be something like a pre -read or an advance discussion item that is shared with them ?

Anil Raghavan: We do n ot have current plans of sharing the interim analysis outlook, I mean , the data with the agency . Our next planned interaction with the agency on PROSEEK data would be pos t full data disclosure in September of '24.

Tushar Bohra: In the November interaction , you had mentioned with regards to my question only , about two or three drugs, especially in the case of Alzheimer's, where FDA has shown inclination for a faster registration pathway. Is it fair to assume that a similar pathway is potentially one of the o ptions for SPARC as well as in PROSEEK t rial is large enough and pivotal enough that FDA may in one of these situations have that as an option that this can itself be used as a registrational study ?

Anil Raghavan: If I answer that, it would be speculat ive, I mean because it require s the agreement with the agencies, but I want to highlight a couple of points. We have a significant number of endpoints in PROSEEK, b oth clinical endpoints and biomarker endpoints . The possibility of an accelerated approval would be based on how the data comes in where all the chips fall . Especially , I mean if you look at the pre cedents in Alzheimer's or ALS recently, they have use d conditional approval pathway which was based on validated biomarkers in those areas. Alzheimer's b ecause of the tracers and data because of the validation of neuronal death marker called neurofilament light had significantly more clarity in terms of the correlation of these biomarkers with clinical outcome . So, we may see similar trends and similar correlation in PROSEEK , but in the field at the moment we don't have validated markers like

Alzheimer 's had going into those events. So , in that sense there are some challenges inherent in the indication, which kind o f differentiates with AD or ALS setting. But , there may be possibilities and I think it is highly speculative from our part to indicate that as a possib ility at this point , because it is a function of what we see in the trial and also how the regulatory ag ency is reviewing that.

Tushar Bohra: In previous discussions at one point I think we mentioned that there is a backup compound to Vodobatinib that we are looking at potentially for Alzheimer's as well. And I think in a previous discussion in November it was highlighted that Vodobatinib can also work for Alzheimer's , the alpha synuclein pathway if I get that right. So , which one is it? It's maybe a bit early, but just to understand the thought process, whether it is Vodobatinib itself that we may look f or Alzheimer's also at some point or is there another

compound maybe a similar domain that we may look for Alzheimer's ?
Anil Raghavan: I think if we continue to see preclinical data coming in the expected lines, we think Vodobatinib can be explored in Alzh eimer's disease with activity seen in mechanistic studies in vitro in IPSC -derived cell , we have seen proteins like activated or phosphor -tau, A -β 40 and 42 moving in a positive

direction with
Vodobatinib. So, this mechanism of cAbl inhibition may have legs in diseases which are driven by other proteins other than al pha synuclein , and that is a key part of the data that we have already presented and disclosed. A nd that is what gives us the confidence to look at these other indications like a bet a-driven diseases or ALS. But from actual strategy standpoint, we have spoken about this in the past. We have a backup program for Vodobatinib. There are additional assets in that package , and we may develop an additional backup compound which will give us a longer IP life from a composition of matter standpoint of additional indications. So , we may choose to develop a backup compound in all the indications driven by other protein s.
Tushar Bohra: On the Parkinson's, beyond PROSEEK, assuming that we need to do a Phase - III trial, how prepared are we for launching the trial , I mean , typically phase

two to phase three, there is a decent lag in a number of programs, because of lack of preparedness on the trial design or regulatory clarity or even funding clarity for that matter, assuming we have a very strong positive data on PROSEEK and we are required for another trial, how well prepared are we , have we already started working in that direction, sir?
Anil Raghavan: There are multiple components to that question . If you look at various different variables of preparedness from a la te stage , one is what to do in terms of regulatory and scientific clarity about the nature of our registration trial, pivotal program that we want to have , and activities that we need to obtain that clarity , and then an ability to kind of design a trial according to those expectations and execute that . And there's a competency and scale - related aspect which is substantial in a Phase -II to Phase -III because kind of competencies that you re quire and the sc ale that you require in Phase -III setting maybe somewhat different from a Phase -II program. As you rightfully pointed out, resourcing expectation is significantly more in the Phase -III because you may need to conduct multiple global trials to complete the traditional Phase -III package. So, on all these accounts , we want to address in the next few months and we already started working on some of the aspects thinking about what could be th e nature of the Phase -III program from a science standpoint and also try to put together a sense of the capability gaps that we want to bridge and also the extent of resourcing that is required. So , we are actually doing some of those regulatory activities as well. But, we can only get full steam once we have clarity from PROSEEK both from the data standpoint and also induce certain ability to discuss this more openly and transparently with the other players that we need to discuss this with , for example , for resourcing . It's a nuanced answer. Yes, to the extent that is possible now, but we are extremely aware of the extent of work that needs to be done and it will all get in focusing those data in September .
Tushar Bohra: Sir, even in the eventuality that two outcomes, one where unfortunately let's say we don't get the relevant data outcome in PROSEEK, and two , where we have a very strong outcome and the trial goes through well and maybe we go

through to a registration al study , in both these situations also, in any case, you will do a trial for Vodobatinib for a symptomatic setting, right , in combination with L-Dopa and the existing regime , so that Part-In any case, you would be already working on some kind of a trial setting which is something any which ways do?
Anil Raghavan: No. So, let's look at the spectrum of possibilities that can come on the PROSEEK trial . On the positive side, you can have a full validation of the hypothesis data in line with the design expectations of t he trial. On the other

end, you can have a full rejection of the hypothesis , in the sense that going in assumption about this m echanism producing a certain level of differen ce on the trajectory in neuro degeneration was not correct. So , there can be a rejection of hypothes is and there can be a host of possibilities in between , that is, it is working in patients with a certain disease severity o r it is working with the patients who may have m orbidity in the data as a background there. So, there are a lot of other possibilities in terms of that gray area between a full rejection and a full validation. The question was pertaining to the full rejection of the hypothesis if I understand it rightly. In that case, we don't believe that there is a justification for exploring this program further in any setting because it rejects primary mechanistic hypothesis. But , in all the other possibilities, whether it is full validation or validation in parts in different settings, that 's difficult area . We think there are possibilities to move forward with an appropriate registration package.

Tushar Bohra: I was just wondering that if the trial is successful as well , then as you said that you would look at Vodobatinib for the extension of the entire Parkinson's program , so that Part-In any case you would already be working on , right?

Anil Raghavan: That if the program is successful. If Vodobatinib is proving , is validating the cAbl hypothesis in PROSEEK, then there is justification in extending this t o other settings in Parkinson's disease . If it is slowing down the trajectory of the disease , then it can be a companion for L -Dopa and slow down late stage PD,

or you can think about exploring this condition. But , if it is not slowing down , then there is no basis. That's the point I was trying to make .

Tushar Bohra: But if it is slowing down or if it is having some reaction , but maybe not sufficient enough to meet the trial design , in that case, is there still a scop e or a reason to study it in symptomatic setting?

Anil Raghavan: We have to take a close look at the data at that point . What we are indicating is that you're seeing a trend, but you may not be meeting statistical significance because the trend is not as strong as we initially thought. We will take a close look at that trend and take a call based on the data that we're seeing. But that is the possibility. I don't want to reject that po ssibility .

Moderator: The next question is from the line of J igar Valia from OHM Group. Please go ahead.

Jigar Va lia: Sir, if you can help understand the administrative interim approach versus the full interim ? And while you still maintain that there might be a requirement for proper Phase -III and would certainly go with a partner approach and t here would be challenges in terms of getting a leeway on the Phase -III. So, would want to understand with regards to the administrative and is there a slight possibility that it may be thought for a Phase -IV direct , so we're in a full phase this thing happens, but post approval ?

Anil Raghavan: The difference between the interim analysis o r administrative interim analysis , the full data set is the number of patients who are going into , we have 4 41 patients going into the interim analysis in end of March or early April , and 513 patients going into the full data set, which would happen in early September next year. So , essentially, we will have practically all of the clinical endpoints and many of the biomarker endpoints in interim analysis, but not all , some biomar ker endpoints would require additional time. So , we will have all endpoints, the primary endpoints, all the secondary endpoints and all the biomarker endpoints as part of the final in September, while we may have a substantial subset of that in the interim analysis . Substantively ,

those are the two differences

-- the number of patients and also the extent of data from a n endpoint standpoint . Your second question is tricky , in the sense, I don't want to get into speculative realm . There a re possibilities in terms of the nature of registra tional expectations from different agenc ies. But, me giving a guidance in terms of what is possible at this point is dangerous in the sense that it is subject to significant change based on data and how th e regulatory agency is going to see that data. So , I would refrain

from taking a position in terms of what could be a possible registration package till w e have those discussions with the agency.
Jigar Valia : Sir, other is, since the additional patient sub types now would require additional investment time , if you can help understand the partnership approach, would it be only for the Parkinson 's disease right now or would it also try to cover the additional sub types?
Anil Raghavan: We are getting into this partnership process with a certain level of openness in terms of the kind of partnerships that we want to have. We would like to have a relationship which maintain some level of participation in both development and also commerciali zation of this program , plus some flexibility in terms of additional indications retained within SPARC. But, at the same time these discussions can take a life of its own once you get into them. So, in that sense, we are open -minded in terms of approaching our partnership strategy. Our b aseline expectation is that we will continue to retain some level of role in development and commercialization, but in actual nature of partnership would be a function of what we are going to see in this process. But, w e wil l explore that fully p ost the interim analysis data in March.
Jigar Valia: To understand , since the partnership is going to be for the P D perspective, etc., would you be looking at funding for any other programs or I mean only this would be a priority unti l this is settled ? So, you also have the fundraise approval, etc., t hose in place apart from the partnership what we could get.
Anil Raghavan: I don't want to throw a hard line and say, partnership would only be for Parkinson's disease . One of the objectives for a partnership or one of the

drivers for seeking a partner is to expand the program aggressively . So, in that sense we would like to leve rage both the resources and also competency muscle of larger partners in terms of fully expl oring this program in Parkinson's and also in other indications . But, how we actually carve out those different spaces in terms of ownership and extent of d eletion is a function of the discussion . So, we don't have a set view that the partnership is only g oing to be for a limited PD setting and SPARC would continue to reso urce everything else , that's not the intent at least going into this process.
Jigar Valia: Just a hypothetical this thing is that, god forbid , in case of a negative outcome, will it remain open to pursue the CML indication given that this would be only -?
Anil Raghavan: Yes, we already have clinical proof -of-concept for C ML and it would require an additional study , we have some clarity in terms of what the study is and then we have to take a decision that SPARC will invest additional resources into conducting those trials or whether we need to see the partner today. That's the call that we can take once we have clarity on the data.
Moderator: The next question is from the line of Bino Pat hiparampil from Elara Capital. Please go ahead.
Bino Pathiparampil: If you're not going to share the administrative interim analysis with pretty much anybody like you said, what is the purpose of this administrative interim analysis?
Anil Raghavan: On a quest ion that we just had, we spoke about the partnership strategy and one of the key objectives of the in terim analysis is to engage w

ith the
potential partners early on so that we can try and minimize the time that we require post the full data in September. So, it is not correct that we are not sharing this with anybody , we will share this with the potential partners under CDA a t an appropriate level with a limited number of potential partners , that's the expectation at this point.

Moderator: Ladies and gentl emen, that was the last question of our question -and-answer session. I would now like to hand the conference over to the management for their closing remarks. Over to you , sir.
Jaydeep Is srani: Thank you, everyone for taking ou t time for today's discussion . We now end the call . In case you have any follow -on questions, you can reach out to us on the e -mail and the numbers provided on our web site. Thank you again . Have

a nice weekend.
Anil Raghavan: Thank you, everybody.
Moderator: Ladies and gentlemen, on behalf of SPARC , that concludes this conference.
We thank you for joining us and you may now disconnect your line s.

Call: Jan 2024

Page 1 of 20 “Sun Pharma Advanced Research Company Ltd. (SPARC)
Transcript of Investor Call held on January 06, 2024
Update on PROSEEK Program”
MANAGEMENT : MR. ANIL RAGHAVAN – CHIEF EXECUTIVE OFFICER
MR. JAYDEEP ISSRANI – HEAD BUSINESS DEVELOPMENT , CORPORATE COMMUNICATIONS & INVESTOR RELATIONS

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Moderator: Ladies and gentlemen, good day and welcome to SPARC Conference Call. As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes . Should you need assistance during the conference call, please signal an operator by pressing star, then zero on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Jaydeep I ssrani. Thank you and over to you , sir.
Jaydeep Is srani: Thank you, Michelle. Good evening, ladies and gentlemen. My name is Jaydeep Is srani, I head the Business Development and Investor Relations at SPARC . On behalf of SPARC , I welcome you to today's call and appreciate you for taking the time out on a Saturday evening to attend the call.
I'm joined by our CEO, Mr. Anil Raghavan and the senior management team at SPARC .
Anil will walk you through the presentation, which we have shared earlier . And after his presentation, we will open the call for questions.
Before we start, I would like to remind you that our discussion today includes forward -looking statements that are subject to risks and uncertainties associated with our business, hence the actual results may be different from those projected in the presen tation today. I will now hand it over to Mr. Anil Raghavan for his presentation. Over to you, Anil.
Anil Raghavan: Thank you, Ja ydeep . Good evening, everybody. Good morning or good afternoon if you are joining internationally. Thank you for taking this cal l on short notice.
I have four key objectives for this call . I wanted to provide an update on the Vodobatinib Program . I want to set up our immediate priorit ies going in 2025-2025 financial year for Vod obatinib particularly after the data events that we're expecting. And also want to provide some perspective in terms of how to look at this program in balance d way. We've spoken about the opportunity

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extensively in our investor call earlier this year. Also , want to give a sense of some of the risks in the program that are or customary to this kind of program s. Finally, and probably most importantly, this is an op portunity to answer any questions that you may have before we go into a quiet period on the interim analysis.
So, with that, let's move to Slide 3. We have a recap of the status of the program. We have touched upon some of this in our earlier call . Vodobatinib PoC program, it's called PROSEEK completed its enrollment target last year around October of 2023. Our targe t was 506 evaluable patients , we ended up with 513 patients globally. And the interim analysis as communicated earlier is planned with 85 %- 86% of patients. That's the enrollment cut off was May of 2023. So , we have 441 patients going into this interim analysis and that is planned for late March -early April of 2024. A s we communicated earlier, the broader organization will be bl inded , not just a broader organization, the external ecosystem or investigat ors and others who are participating in the trial will also be blinded because we' ll still have around 70 -odd patients in different stages of treatment and protecting the in tegrity of status in those patients is an important consideration for the trial and also for the regulators in different countries. In that sense, we are committed to maintaining the blind and protecting this information to ensure that we don't in duce unnecessary bias in conduct of the trial .
The design of the program we have covered that in many earlier conversations, but just to give a very brief overview, it has two doses of Vodobatinib 168 patient s in each arm 384 m g on the top dose and 192 m g on the lower dose against the placebo arm. As we have spoken about in the past ,

MDS-UPDRS Part-III is the primary endpoint . We have a host of secondary endpoints and biomarke

r s which are mostly exploratory . And th e study also has Part -II long -term extension study, where patients are on the drug for another forty weeks , that's roughly around 10 -months. So , the patients on Part-A and Part -B combined are for almost 80 -weeks and those patients go through this 80-week s period without any symptomatic therapy. So , that's the

design of the program and I have already talked about some of the milestones like interim analysis in April . The full top line data for PROSEEK is expected in August -September of 2024, that's when we will have full disclosure on the data.

Now , moving on to the next slide , has a listing of our nea r-term priorities post availability of data from PROSEEK . The primary objective in 2024 -25 financial year is to ensure that we move on to the next phase of development without a phase lag if we're fortunate to have positive data in line with expectations of the hypothesis . That requires several steps, most importantly , an agreement with the regulatory agencies globally in terms of the nature of registrational studies required. So , that would happen during the end of Phase -II consultations with US FDA and other important regulatory agencies globally. And we hope to do that in short order after the August -September data readout .

And in parallel, we will be continuing the long -term extension s tudy of PROSEEK . And as soon as we have clarity in terms of th e nature of the registrational studies required, we hope to initiate the pivotal Phase -III programs globally and our strategic intent would be to minimize that lag between end of Phase -II and initiation of Phase -III. And we will also be using this time right from the availability of inter im analysis data to explore and execute a partnering strategy so that as we move into late stage development post PROSEEK we can do that with a partner who we may be going with for commercialization of the asset.

And as I said , the regulatory agreement is an important element in triggering this and it is not just about additional clinical studies required , we may also require additional pre -clinical studies, particularly toxicity studies , and also additional steps to be t aken for ensuring manufacturing our business, particularly given the size and potential of this drug.

So, let's go to the next slide , when we met you in the last quarter of 2023 for our annual investor ’s update, we have spoken extensively about the program,

the potential of the hypothesis and its implications on the standard -of-care in Parkinson's disease and also more broadly about the possibilities of diseases driven by alpha synuclein and in the outer r ung of possibility , diseases that are driven by other proteins impacted by oxidative stress and A bl-linked activation.

We have spoken about the opportunity clearly . We thought that it is important given some of the communication and coverage that we are se eing on this drug and recently it's important to also highlight some of the risk s which are inherent in programs like these, especially in trans lation programs in neuroscience. So , we want to highlight three -four major risks that you may want to keep in mi nd as you kind of think about these pro grams.

One is the translatability of animal models in diseases like this which we have seen . These animal models built with inten t to mimic the underlying mechanism and create the manifestation of disease which can be addressed with the drug . While they are fine from a science standpoint, their true validation will come , when programs which are developed with these models go on to clinic and get validating clinical data and go into market .

In diseases like Parkinson's , we haven't seen significant clinical translation in the long -term, especially in disease -modifying therapies which are designed to bend the ne urodegenerative arc. So,

we are one of the early companies
which is trying to translate the oxidative stress pathway and therefore these models carry a certain level of risk and it is important to keep that in mind as you evaluate this program.

A couple of other risk s that I want to highlight :

One is about target engagement and dose. If you've been following this program, we have used the top dos e in our animal model that is 45 mpk as a marker for deciding our do se for the Phase -II clinical program. The ma x brain exposure associated with the top dose in animal studies which is the most efficacious dose in animal stud y. And in early clinical studies , we matched that

exposure in Parkinson's patients C SF, which we believe is a good proxy for levels in the bra in. So even though the translation process is sound , this field clearly lacks a tar get engagement marker because we cannot open the brain and see what happens. So, there are challenges in terms of clearly getting appropriate doses in humans. So , there is a certain level of approximation that is done by extrapolating animal data that may create certain level of risk which needs to be factored in.

The third point that I want to highlight is the reproducibility of early clinical proof -of-concept studies as you t hink about repeating dose results in later stage clinical setting. And ty pically, if you look at companies which are doing early proof -of-concept studies and they're more smaller pilot studies and much of this risk comes from inadequate powering of early studies. And that we have tried to a ddress some of that in the design of t he PROSEEK trial. PROSEEK is the Phase -II program. It is an extensive study with 500 -plus patients which is powered at around 80%. So , in that sense , even though we have tried to address some of the trans lation risk i.e. some of the risks associated with reproducibility of early -stage clinical results in late stage clinical programs, that is still a risk in the sense that it needs to be reproduced in the larger s tudy in Phase -III setting.

And the last point is which will require extensive additional work in terms of Phase -III programs and resourcing that and actually executing that in a timely manner .

We have also in the earlier presentations talk ed about the opportunities for Vodobatinib beyond Parkinsons disease and that would require additional preclinical and clinical work and we are in the process of doing some of that. These are mainly in two buckets , which are driven by alpha synuclein like lewy body dementia and MSA .

So, I was talking about additional indications fo r Vodobatinib which would require additional preclinical studies and clinical studies and some of those studies are currently under way. You may know that we have been working

with Georgeto wn in investigat or-initiated trial to explore this program in smalle r proof -of-concept study for lewy body dementia. We're also working in early stage preclinical work in Alzheimer's disease with this mechanism with academic investigators. So, there is additional work that needs to be done to fully explore the potential of this program in other indications and other diseases, but also in other settings in Parkinson's. If you look at the design of Vodobatinib PROSEEK trial , we're looking at earl y stage Parkinson 's patients who are pre -symptomatic that those are patients who are very early in their disease process and before they get into symptomatic therapies like L-Dopa. If you demonstrate disease modification or bending of the neurodegenerative arc in th e setting, clearly, there is justification for exploring this program in other settings in Parkinson 's disease like the combination of symptomatic therapy . Both these settings that would require additional studies.

Success on PROSEEK w ould be the beginning of an ex clusive journey, which would then valid

ate the program in the early setting, it should be our first registration al program, but also it will initiate a significant number of opportunities outside, but all of that would require an additio nal investment

and additional exploration from a pre-clinical and clinical standpoint.
That takes me to my last slide. I just wanted to highlight a few market -related risks here in this slide . We have seen in a recent coverage on Vodobatinib which in our v iew captures some aspects of the program not all aspects of the program comprehensively. So , for investors who intend to price in Vodobatinib 's potential to their decisions and deliberate analysis of the potential of the program both from a sales standpoin t and also extension into other possibilit y stand point along with an understanding of cause and risk, and also time -to-market, it's important before you take those calls . In a setting like this early -stage biotechs go into data events like th ese, there is significant risk of price volatility. And that is somewhat magnified in the market like in the absence of informed analyst coverage, which looks at these programs carefully . So, when you look at reporting that is coming in, we urge

you to take a balance d view on the program which balances both the potential of the program as well as the risk of the program.
And finally, I want to reaffirm our commitment to this area. We're going to see significant data coming from this program i n the rest of this year with interim analysis and also in the final analysis . That's going to teach us a lot about c-Abl's role in neurodegeneration and teach us about how the moderation of stress path way is going to have a role in treating neurodegenerative disease s. And there is a significant number of possibilities in th is data that you can have clear validation of this hypothesis. You can also have a clear debunking of this hypothesis, but there's also a lot of gray space in between. So, this data we're looking forward to contex tualiz e in what we're going to learn and then finding ways to move forward with continuing exploration of this program . Not just in this area as in this hypothesis , we also want to reaffirm our commitment in the neuro degenerative disease. In fact, if we go back several years that was the bet that we have taken in that neurode generation in spite of conventional wisdom going the other way, we believe that the ne urode generation is an area is maturing from a science standpoint and that has also been valida ted recently with several transactions happening in this space. So , we will continue to be interested , continue to be excited about making a dent on these difficult di seases in the neurodegenerative spectrum . So, with that, I will conclude my comments and open up this call for the questions you may have. Thank you very much again for attending this call on short notice.
Moderator: We will now begin the question -and-answer session. We'll take the first question from the line of Ketan Gandhi from Gandhi Securities. Please go ahead.
Ketan Gandhi : In the November 2nd presentation , you indicated on Slide 19 that approximately 87% of the eligible patient s enrolled in part-II from part -I. However, in current presentation , there is no update. Do you have any up date on that , sir?

Anil Raghavan: That is true , 85% to 87% of eligible patients who are completing part -I of the study a re eligible for continuation into a phase and long -term extension study . We don't have additional data . That trend continues.
Ketan Gand hi: Do you interpret the rollover rate to part-2 as indicative of patients positively embracing the treatment or is it possibly as a result of patient not experiencing the desired improvement , what is your thought process on that?
Anil Raghavan: It's very difficult to conclusively say that because we have significan

t number
of patients on placebo . Even if they're not improving, the y may want to access the drug in the long -term extension settings. There are a lot of motivation that patients are fee ling better and they may want to continue . If patients feel that they're not feeling better, but they may be on placebo and they may want to transition to drug . So, there are significant number of different motivations that may be driving that conclusion. So , while it maybe encouraging, but it's difficult to draw any definitive conclusions based on that.

Ketan Gandhi : On Slide 5, under the section , “Expanding Evaluat ion of Vodobatinib ” it is mentioned that SPARC would explore initial registrat ion in an early treatment and n aive setting. Can you throw some light on regulatory pathway post the announcement of PROSEEK top line data in August '24, whether we would go for regulatory approval or we would go after Phase -III, can FDA help us in getting approval post EoP2?
Anil Raghavan: So first let me give you a little bit of context to this statement in this presentation, which would explore initial registration in early treatment naive setting . And that is the most logical way to plan a registration like for this program. The patients who are coming into PROSEEK are early -stage patients who are treatment naïve from L-Dopa and symptomatic standpoint. So , if you're getting a positive readout from that trial, the least risk option from a registration standpoint is to repeat those studies and reproduce those results, which would become the basis of registration. The intent of that statement is that early or the initial registration setting would be a repeat of what we have explored and studied in PROSEEK . Now , the second part of your question is

“in terms of what would be an actual registration program and regulatory ask,” that is subject to the discussions that we need to have with the regulatory agencies. We are going to have end of Phase -II conversation as soon as we have data . And there are very many possibilities. I mean, traditionally , the agencies would require two additional Phase -III studies and that's th e classical ask in these kinds of settings . But there are also other possibilities of like factoring into a registration al package in some form o r fashion, but that is all strategies that would require validation and agreement with the agencies around the world and we intend to do that as soon as we have data from the PROSEEK program. So, final shape on the registration al packa ge can only be clear after we have these discussions with the agency.
Ketan Gandhi : Is my understanding right that we will be starting partnership program between interim data analysis and the top line data between that period ?
Anil Raghavan: That is true. We expect to initiate conversations with the potential partners. We may not be able to conclude that before a final dat a disclosure in August - September, but we intend to kind of use this time to engage and create interest and work towards a partnership as we kind of get to September.
Moderator: We'll take the next question from the line of Ishita Jain from Ashika Stock Broking . Please go ahead.
Ishita Jain : Hi, Anil . Thanks for the update. Appreciate this reiteration of the risks associated to drug development. My first question is so our enrolment concluded in October 2023 . Was the enrollment timeline as anticipated or did we face any challenges in recruiting?
Anil Raghavan: Your voice is a bit muffled. I couldn't hear you.
Ishita Jain: My question is that our enrollment concluded in October 2023 . Was the enrollment timeline as anticipated or did we face any challenges in recruiting?
Anil Raghavan: So, if you remember, we started this program even though technically in 2019 the actual dosing, started in early part of 2020. Exactly when health systems

around

the world started shutting down because of the pandemic . The initial couple of years have been rough for the program because most of the hospitals were not seeing patients in -person in hospital settings, and many of our endpoints require practitioners , assistant s at site . So, in that sense we clearly have seen slower than anticipated in the first part of th e study, but it has clearly picked up in the last 18 -months or so, which helped us to conclude this program , almost in line with the revised timeline. We did a revision of the time line during the COVID phase . So COVID induced certain delays. But other than that , in the last 18 -months to a couple of years , the program has been tracking to the plan.
Ishita Jain: What I'm trying to understand is that if we go into Phase -III, what would mean Phase -III in terms of phase and then enrollment concerns in Phase -III, is that significant , I mean , obviously not withstanding existing drug

development concerns?
Anil Raghavan: I won't be able to comment on the actual size of the Phase -III program. There are a significant number of statistical considerations that will go into defining that size and it will also be informed by the data that we are seeing in PROSEEK in terms of the doses that are effective , what is a registration al dose that we want to carry and the effect size that we are seeing in the Phase -II setting . So, there are a lot of variables which can only be informed by data from the PROSEEK trial. I won't be able to clearly indicate the size of the Phase -III program. We expect to maintain the momentum that we had in the later part of PROSEEK in terms of recruitment going into the Phase -III program . Because we have relationships with the investigators across and the other service providers in this ecosystem, and we understand this space probably better than when we started off. So , I think we are confident that we can maintain the moment um that we had in the second half of this trial f rom an actual recruitment rate standpoint, but actual size of the program in terms of number of patients that would be required is a function of where we land with PROSEEK is difficult to extrapolate that now.

Ishita Jain: I think I'm not sure if you alrea dy mentioned it and did I miss it , but meeting with the agency would only be post final data, right , there would be no agency meeting with inter im data ?
Anil Raghavan: Yes, the agencies would require completion of the trial and data from the ITT population before we can have the end of Phase -II meeting.
Ishita Jain: You mentioned that we may require additional preclinical studies. What kind of studies would these be , I mean I know it would be around efficacy , concentration of drug in brain or I mean checking for a specific biomarker, what kind of additional preclinical studies would be required?
Anil Raghavan: So, there are several toxicity studies, which are standard expectations in the registration al package , to give you an example , carcinogenicity studies, is a standard expectation in the registration al package . And there may be additional clinical trials like drug -drug interaction studies or other toxicity studies i n preclinical settings. So , there are a part of customary expectations in a registrational setting in both preclinical studies and certain additional clinical studies . Clinical studies are not major clinical studies, but they are the studies which explore safety endpoints.
Moderator: We will take the next question from the line of Bino Pat hiparampil from Elara Capital . Please go ahead.
Bino Pathiparampil: Could you please make some comments around the IP estate around Vodobatinib, do you have a drug substance patent , till how long does it run , etc.?
Anil Raghavan: I don't have the exact dates in front of me, but I can indicate that he comp ensation of matter and the regulator

y compensation for the development time, we will go into late second half of 30s from IP coverage and we may also have some additional patents which are covering this space in the method of treatment patent and also potentially formulation patent. So, w e are confident that we may go into late second half of 2030s for sure.

Moderator: We'll take the question from the line of Tu shar Bohra from MK Ventures. Please go ahead.
Tushar Bohra : Sir, just a couple of points. So , while the end of Phase -II discussions with the FDA would happen only after the final data, but t rying t o assume that with the interim data analysis, whatever the readouts that we would have , the same would be shared with FDA as well , and that may be useful in whenever we have the final discussions , this would be something like a pre -read or an advance discussion item that is shared with them ?
Anil Raghavan: We do n ot have current plans of sharing the interim analysis outlook, I mean , the data with the agency . Our next planned interaction with the agency on PROSEEK data would be pos t full data disclosure in September of '24.
Tushar Bohra: In the November interaction , you had mentioned with regards to my question

only , about two or three drugs, especially in the case of Alzheimer's, where FDA has shown inclination for a faster registration pathway. Is it fair to assume that a similar pathway is potentially one of the o ptions for SPARC as well as in PROSEEK t rial is large enough and pivotal enough that FDA may in one of these situations have that as an option that this can itself be used as a registrational study ?

Anil Raghavan: If I answer that, it would be speculat ive, I mean because it require s the agreement with the agencies, but I want to highlight a couple of points. We have a significant number of endpoints in PROSEEK, b oth clinical endpoints and biomarker endpoints . The possibility of an accelerated approval would be based on how the data comes in where all the chips fall . Especially , I mean if you look at the pre cedents in Alzheimer's or ALS recently, they have use d conditional approval pathway which was based on validated biomarkers in those areas. Alzheimer's b ecause of the tracers and data because of the validation of neuronal death marker called neurofilament light had significantly more clarity in terms of the correlation of these biomarkers with clinical outcome . So, we may see similar trends and similar correlation in PROSEEK , but in the field at the moment we don't have validated markers like

Alzheimer 's had going into those events. So , in that sense there are some challenges inherent in the indication, which kind o f differentiates with AD or ALS setting. But , there may be possibilities and I think it is highly speculative from our part to indicate that as a possib ility at this point , because it is a function of what we see in the trial and also how the regulatory ag ency is reviewing that.

Tushar Bohra: In previous discussions at one point I think we mentioned that there is a backup compound to Vodobatinib that we are looking at potentially for Alzheimer's as well. And I think in a previous discussion in November it was highlighted that Vodobatinib can also work for Alzheimer's , the alpha synuclein pathway if I get that right. So , which one is it? It's maybe a bit early, but just to understand the thought process, whether it is Vodobatinib itself that we may look f or Alzheimer's also at some point or is there another compound maybe a similar domain that we may look for Alzheimer's ?

Anil Raghavan: I think if we continue to see preclinical data coming in the expected lines, we think Vodobatinib can be explored in Alzh eimer's disease with activity seen in mechanistic studies in vitro in IPSC -derived cell , we have seen proteins like activated or phosphor -tau, A -β 40 and 42 moving in a positive

direction with

Vodobatinib. So, this mechanism of cAbl inhibition may have legs in diseases which are driven by other proteins other than al pha synuclein , and that is a key part of the data that we have already presented and disclosed. A nd that is what gives us the confidence to look at these other indications like a bet a-driven diseases or ALS. But from actual strategy standpoint, we have spoken about this in the past. We have a backup program for Vodobatinib. There are additional assets in that package , and we may develop an additional backup compound which will give us a longer IP life from a composition of matter standpoint of additional indications. So , we may choose to develop a backup compound in all the indications driven by other protein s.

Tushar Bohra: On the Parkinson's, beyond PROSEEK, assuming that we need to do a Phase -III trial, how prepared are we for launching the trial , I mean , typically phase

two to phase three, there is a decent lag in a number of programs, because of lack of preparedness on the trial design or regulatory clarity or even funding clarity for that matter, assuming we have a very strong positive data on PROSEEK and we are required for another trial, how well prepared are we , have we already started working in that direction, sir?

Anil Raghavan: There are multiple components to that question . If you look at various different variables of preparedness from a la te stage , one is what to do in terms of regulatory and scientific clarity about the nature of our registration trial, pivotal program that we want to have , and activities that we need to

obtain that clarity , and then an ability to kind of design a trial according to those expectations and execute that . And there's a competency and scale - related aspect which is substantial in a Phase -II to Phase -III because kind of competencies that you re quire and the sc ale that you require in Phase -III setting maybe somewhat different from a Phase -II program. As you rightfully pointed out, resourcing expectation is significantly more in the Phase -III because you may need to conduct multiple global trials to complete the traditional Phase -III package. So, on all these accounts , we want to address in the next few months and we already started working on some of the aspects thinking about what could be th e nature of the Phase -III program from a science standpoint and also try to put together a sense of the capability gaps that we want to bridge and also the extent of resourcing that is required. So , we are actually doing some of those regulatory activities as well. But, we can only get full steam once we have clarity from PROSEEK both from the data standpoint and also induce certain ability to discuss this more openly and transparently with the other players that we need to discuss this with , for example , for resourcing . It's a nuanced answer. Yes, to the extent that is possible now, but we are extremely aware of the extent of work that needs to be done and it will all get in focusing those data in September .
Tushar Bohra: Sir, even in the eventuality that two outcomes, one where unfortunately let's say we don't get the relevant data outcome in PROSEEK, and two , where we have a very strong outcome and the trial goes through well and maybe we go

through to a registration al study , in both these situations also, in any case, you will do a trial for Vodobatinib for a symptomatic setting, right , in combination with L-Dopa and the existing regime , so that Part-In any case, you would be already working on some kind of a trial setting which is something any which ways do?
Anil Raghavan: No. So, let's look at the spectrum of possibilities that can come on the PROSEEK trial . On the positive side, you can have a full validation of the hypothesis data in line with the design expectations of t he trial. On the other

end, you can have a full rejection of the hypothesis , in the sense that going in assumption about this m echanism producing a certain level of differen ce on the trajectory in neuro degeneration was not correct. So , there can be a rejection of hypothes is and there can be a host of possibilities in between , that is, it is working in patients with a certain disease severity o r it is working with the patients who may have m orbidity in the data as a background there. So, there are a lot of other possibilities in terms of that gray area between a full rejection and a full validation. The question was pertaining to the full rejection of the hypothesis if I understand it rightly. In that case, we don't believe that there is a justification for exploring this program further in any setting because it rejects primary mechanistic hypothesis. But , in all the other possibilities, whether it is full validation or validation in parts in different settings, that 's difficult area . We think there are possibilities to move forward with an appropriate registration package.
Tushar Bohra: I was just wondering that if the trial is successful as well , then as you said that you would look at Vodobatinib for the extension of the entire Parkinson's program , so that Part-In any case you would already be working on , right?
Anil Raghavan: That if the program is successful. If Vodobatinib is proving , is validating the cAbl hypothesis in PROSEEK, then there is justification in extending this t o other settings in Parkinson's disease . If it is slowing down the trajectory of the disease , then it can be a companion for L -Dopa and slow down late stage PD,

or you can think about exploring this condition. But , if it is not slowing down , then there is no basis. That's the point I was trying to make .
Tushar Bohra: But if it is slowing down or if it is having some reaction , but maybe not sufficient enough to meet the trial design , in that case, is there still a scop e or a reason to study it in symptomatic setting?
Anil Raghavan: We have to take a close look at the data at that point . What we are indicating

is that you're seeing a trend, but you may not be meeting statistical significance because the trend is not as strong as we initially thought. We will take a close look at that trend and take a call based on the data that we're seeing. But that is the possibility. I don't want to reject that po ssibility .
Moderator: The next question is from the line of J igar Valia from OHM Group. Please go ahead.
Jigar Va lia: Sir, if you can help understand the administrative interim approach versus the full interim ? And while you still maintain that there might be a requirement for proper Phase -III and would certainly go with a partner approach and t here would be challenges in terms of getting a leeway on the Phase -III. So, would want to understand with regards to the administrative and is there a slight possibility that it may be thought for a Phase -IV direct , so we're in a full phase this thing happens, but post approval ?
Anil Raghavan: The difference between the interim analysis o r administrative interim analysis , the full data set is the number of patients who are going into , we have 4 41 patients going into the interim analysis in end of March or early April , and 513 patients going into the full data set, which would happen in early September next year. So , essentially, we will have practically all of the clinical endpoints and many of the biomarker endpoints in interim analysis, but not all , some biomar ker endpoints would require additional time. So , we will have all endpoints, the primary endpoints, all the secondary endpoints and all the biomarker endpoints as part of the final in September, while we may have a substantial subset of that in the interim analysis . Substantively ,

those are the two differences

-- the number of patients and also the extent of data from a n endpoint standpoint . Your second question is tricky , in the sense, I don't want to get into speculative realm . There a re possibilities in terms of the nature of registra tional expectations from different agenc ies. But, me giving a guidance in terms of what is possible at this point is dangerous in the sense that it is subject to significant change based on data and how th e regulatory agency is going to see that data. So , I would refrain from taking a position in terms of what could be a possible registration package till w e have those discussions with the agency.
Jigar Valia : Sir, other is, since the additional patient sub types now would require additional investment time , if you can help understand the partnership approach, would it be only for the Parkinson 's disease right now or would it also try to cover the additional sub types?
Anil Raghavan: We are getting into this partnership process with a certain level of openness in terms of the kind of partnerships that we want to have. We would like to have a relationship which maintain some level of participation in both development and also commerciali zation of this program , plus some flexibility in terms of additional indications retained within SPARC. But, at the same time these discussions can take a life of its own once you get into them. So, in that sense, we are open -minded in terms of approaching our partnership strategy. Our b aseline expectation is that we will continue to retain some level of role in development and commercialization, but in actual nature of partnership would be a function of what we are going to see in this process. But, w e wil l explore that fully p ost the interim analysis data in March.
Jigar Valia: To understand , since the partnership is going to be for the P D perspective, etc., would you be looking at funding for any other programs or I mean only this would be a priority unti l this is settled ? So, you also have the fundraise approval, etc., t hose in place apart from the partnership what we could get.
Anil Raghavan: I don't want to throw a hard line and say, partnership would only be for Parkinson's disease . One of the objectives for a partnership or one of the

drivers for seeking a partner is to expand the program aggressively . So, in that sense we would like to leve rage both the resources and also competency muscle of larger partners in terms of fully expl oring this program in Parkinson's and also in other indications . But, how we actually carve out those

different spaces in terms of ownership and extent of d eletion is a function of
the discussion . So, we don't have a set view that the partnership is only g oing
to be for a limited PD setting and SPARC would continue to reso urce
everything else , that's not the intent at least going into this process.
Jigar Valia: Just a hypothetical this thing is that, god forbid , in case of a negative outcome,
will it remain open to pursue the CML indication given that this would be only
-?
Anil Raghavan: Yes, we already have clinical proof -of-concept for C ML and it would require
an additional study , we have some clarity in terms of what the study is and
then we have to take a decision that SPARC will invest additional resources
into conducting those trials or whether we need to see the partner today.
That's the call that we can take once we have clarity on the data.
Moderator: The next question is from the line of Bino Pat hiparampil from Elara Capital.
Please go ahead.
Bino Pathiparampil: If you're not going to share the administrative interim analysis with pretty
much anybody like you said, what is the purpose of this administrative interim
analysis?
Anil Raghavan: On a quest ion that we just had, we spoke about the partnership strategy and
one of the key objectives of the in terim analysis is to engage w

ith the
potential partners early on so that we can try and minimize the time that we
require post the full data in September. So, it is not correct that we are not
sharing this with anybody , we will share this with the potential partners under
CDA a t an appropriate level with a limited number of potential partners , that's
the expectation at this point.

Moderator: Ladies and gentl emen, that was the last question of our question -and-answer
session. I would now like to hand the conference over to the management for
their closing remarks. Over to you , sir.
Jaydeep Is srani: Thank you, everyone for taking ou t time for today's discussion . We now end
the call . In case you have any follow -on questions, you can reach out to us on
the e -mail and the numbers provided on our web site. Thank you again . Have
a nice weekend.
Anil Raghavan: Thank you, everybody.
Moderator: Ladies and gentlemen, on behalf of SPARC , that concludes this conference.
We thank you for joining us and you may now disconnect your line s.

Call: Apr 2024

SPARC/Sec/SE/202 4-25/008 April 18, 202 4

National Stock Exchange of India Ltd.,
Exchange Plaza, 5th Floor,
Plot No. C/1, G Block,
Bandra Kurla Complex,
Bandra (East), Mumbai – 400 051.

Scrip Symbol: SPARC BSE Limited,
Market Operations Dept.
P. J. Towers,
Dalal Street,
Mumbai - 400 001.
Scrip Code: 532872
Dear Sir/Madam,

Sub: Transcript of the Investor's call on Vodobatinib Parkinson's Disease program

This is with reference to our Investor Call which was scheduled on April 15, 2024. Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, we hereby attached the Transcript of the said Investor Call for Vodobatinib Parkinson 's Disease and the same is also available on the website of the Company on the below weblink: https://sparc.life/presentations/

This is for your information and dissemination purpose .

Yours faithfully,

For Sun Pharma Advanced Research Company Ltd.

Kajal Damania
Company Secretary and Compliance Officer

Page 1 of 13 "Sun Pharma Advanced Research Company (SPARC)
Transcript of Investor Call on PROSEEK's Interim
Analysis Results held on April 15, 2024"

MANAGEMENT : MR. ANIL RAGHAVAN -- CEO , SPARC
MR. JAYDEEP ISSRANI – HEAD, BUSINESS DEVELOPMENT &
INVESTOR RELATIONS , SPARC

Page 2 of 13
Moderator: Ladies and gentlemen, good day and welcome to the SPARC 's Conference Call for Update on PROSEEK 's Interim Results.
As a reminder, all p articipant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing "*" then "0" on your touchtone phone. Please note that this conference is being recorded.
I now hand the conference over to Mr. Jaydeep Issrani. Thank you and over to you, sir.
Jaydeep Issrani: Thank you Sagar. Good evening, ladies and gentlemen. My name is Jaydeep. I head the Business Development and Investor Relations at SPARC.
On behalf of SPARC, I welcome you for the update on the interim analysis of PROSEEK study.
I have with me, Mr. Anil Raghavan, CEO of SPARC. Anil will provide a brief update on the outcome of the "PROSEEK Study ," following which we will open the call for question -and-answer.
Before I hand it over to him, I would like to remind you that our discussion may include forward - looking statements that are subject to risks and uncertai nties associated with our business.
I will now hand it over to Mr. Anil Raghavan for his presentation. Over to you, Anil.
Anil Raghavan: Hello everybody, thank you for joining the call today. Thanks, Jaydeep for the intro setting up the call. As he mention ed we plan to provide an update on the Interim Analysis of our PROSEEK study and briefly talk about the next steps and the way forward for SPARC. The primary objective though, is to try and address your questions and concerns as much as possible. So, I will keep my opening comments brief.

But before I begin, I would like to apologise for the delay in convening this call. We received data from the PROSEEK interim analysis last week and made a disclosure of the results through a press release within a day. Our intent was to provide an update and talk to you as soon as possible. However, the immediate focus thereafter was to initiate the study -close out activities and think through the near -term priorities which t ook a few days to complete. We believe it was important to map out key next steps for V odobatinib before we can have a productive conversation with you. But again, we understand the sensitivity, and sincerely apologise for not following up immediately afte r the release.
You may recall that our initial plan was not to disclose the interim results before completing the review of all the 513 patients randomized on the study. However, in the interest of patients, our investigators and the larger investor community we decided to unblind the study and share the results immediately given the trend lines seen.

As we disclosed, we completed the review of 442 patients who f inished the part I of the study by March 2024. The data suggested that the V odobatinib arms. that is the low dose and high dose arms didn 't meet the primary endpoint of change in score from baseline in MDS -UPDRS part III at week 40. Key secondary endpoints have also shown a similar trend. Overall incidence of serious adverse events was low in this trial, indicating an acceptable safety profile.
That was indeed a disappointing outcome, and certainly not what we hoped for our patients, especially after having generated significant body of evidence in preclinical model systems in collaboration with some of the most important thought leaders in this space demonstrating the neuroprotective role of V odobatinib in mul tiple pre -clinical studies.
Based on our statistical projections that suggested that the study is unlikely to show clinical benefit

in the overall study population, we decided to close the trial. Having said that, we will review the full data set which is not expected to change the p rimary endpoint of the study however it can provide meaningful insights on the mechanism, the drug and the trial. We will keep you informed as we complete the planned analysis.
Once we have reviewed the full data, we will also evaluate the expectations for and the value of the Lewy Body Dementia Investigator Initiated Trial and pre -clinical programs for the back -up series of V odobatinib as they target the same mechanism of action as V odobatinib did in PROSEEK . But as of now, we have parked all activities in these studies.
Obviously these results represent a setback for SPARC which will pose questions on certain parts of our portfolio and challenge our ability to pursue an aggressive R&D agenda. As we complete the data analysis of PROSEEK, we will also work towards resetting our portfolio, optimizing our operating model and cost structure. This is really important considering the additional cards we have got, to tu rn. As we have indicated previously, SPARC 's pipeline has significant optionality including multiple clinical assets and a late -stage preclinical program which is expected to enter clinical testing in the last quarter of this financial year. Please allow me to take a minute to highlight these assets with short to medium term milestone visibility.
The most advanced program is SCO -088, which is V odobatinib 's oncology leg, for treatment of CML. We have completed the Phase 2 study and the data provides validation of its hypothesis in Leukemia with excellent response even in patients who have exhausted all available therapeutic options. As next steps, we are discussing the registration study design and expectations with the USFDA, in fact later today, to decide future development strategy for SCO -088. Since SCO -088 has established human PoC, we may explore licensing the asset to a partner to raise non -dilutive capital for developing other late -stage assets i.e. SCD -153 and SBO -154.
SCD -153 is under Phase 1 evaluation in India and we hope to complete the Phase 1 study by FY25 and subsequently an early patients ' study in Alopecia Areata in India by FY26 at which point we w ould have established human PoC for SCD -153. Similarly, for SBO -154, our Antibody Drug Conjugate targeting a novel epitope of Muc -1, we have initiated IND enabling

studies and we expect to file the IND by the end of current financial year. Both these oppor tunities represent first in class positioning for SPARC across multiple indications.
Additionally, Vibozilimod is under Phase 2 assessment and the top line results of the SOLARES AD study is expected in Q4 20 24. This, as you know, is licensed to Sun Pharma and the development costs are bourn by the partner.
The continued development of these programs would require additional resourcing and our current cash balanc e and approved credit may be sufficient to cover the expenses for 12 -15 months if we factor in expected cost savings from V odobatinib and related deprioritizations. We are also eligible for additional milestones from Vibozilimod and Phenobarbital depending on certain outcomes that can increase the runway for SPARC. We may require additional resources

to get to the next set of milestones in certain scenarios. We will tightly manage the portfolio and spend to get to the next set of data events.
As I indicated earlier, we will review our current clinical and pre -clinical portfolio to identify and prioritize high value prog

rams and optimize the cost base to extend our runway.
While the PROSEEK interim results set us back definitely, we have additional assets specially the clinical assets that carry significant value which we will pursue using appropriate resourcing models which may involve partnering early if that make s sense. We will keep you posted as we make progress with the full analysis of the PROSEEK data and complete our strategy review.
Thanks again for joining us today.
And at this point, we would like to open th e call for any questions, concerns, suggestions that you may have. Thanks again for your time.
Moderator: We will now begin the question -and-answer session. The first question is from the line of Ketan Gandhi from Gandhi Securities. Please go ahead.
Ketan Gandhi: In the November call, you said, we have a clearer option for V odobatinib for CML. So, in terms of timeline, when can we expect the licensing deal to happen and what are the opportunity size for this molecule?
Anil Raghavan: Thanks, Ketan. I think as we 've explained in the last investor call, we had parked the CML development till we have the results of V odobatinib study as in the PROSEEK study. We have planned conversation with the FDA to have a clear visibility to the regulatory pathway and we are now in the process of reinitiating potential business development activity on the CML, part of the program. In terms of giving you a specific timeline for closing out of that licensing transaction, I think probably we are a little too early in the process . We just started this process now and we may need additional time to kind of scan and go through the process and conclude a transaction for V odobatinib. In terms of the opportunity size, I think we haven 't disclosed specifically sales potential, but it is significantly lower than the Parkinson 's disease opportunity

and here we are talking about potentially several hundred million dollars as against several billion dollars in the case of Parkinson 's disease.
Ketan Gandhi: Sir, I want the timeline in terms of whether it will happen in this financial year or next financial year or a year after the next financial year, can you give some sort of ballpark understanding so that we can model that into our -?
Anil Raghavan: We're definitely targeting in this financ ial year and that 's our objective.
Ketan Gandhi: So, in terms of cash burn, we have enough cash position to go ahead for other programs or we need to raise some equity in near -term?
Anil Raghavan: We are not contemplating any near -term equity raise. If I l ook at our current cash balance which should be in the range of around $20 million and we also have access to approved credit, and if I actually look at both approved credit and the current available cash, we should be able to basically go beyond the curre nt financial year. We 're looking at the broad window of around 12 -to-15 months. And this also factors in some of the cost savings from the reprioritization, and if you're not spending on the V odobatinib clinical program and also the backup program that we have, that is a significant part of our budget for this year. So, in that sense, if I take out that from the provision, this cash balance and access to credit is sufficient to take us to 12 -to-15 months runway, but we 're taking a deeper look at our portfol io priorities. Clearly, the assets that I spoke about in the call earlier are important and we have clinical validations coming from mostly in India -based trials in these cases except MUC -1. So, beyond that we may need to have additional cash infusions but that's depending on how some of these outcomes pan out , we have milestone visibility on Vibozilimod which is reading out later this year and Sezaby can also provide additional milestones. So, there are additional mi

lestone opportunities which can augment this
beyond the 12 -to-15 months window. We will take a call as we kind of move towards that, but we will take a hard look at the portfolio and our cost structure in the interim.
Ketan Gandhi: Sir, can you throw some light on the Phenobarbital, what happene d there because I believe we have filed some case in District Court of Columbia in 2nd of April, so, what is that about and where are we in terms of launching it?
Anil Raghavan: The case that we have filed against the FDA has nothing to do with the launch of the product. The product is launched, and the takeoff is expected when we get the exclusivity established. And there is a whole work stream going on and our expectation right from the beginning is that by end of the second year or beginning of third yea r of launch, we will be able to establish the exclusivity or enforce the exclusivity and take out the unapproved products in the market. But the case is about a different matter. We believe we are eligible to get a pediatric rare diseases voucher. And that pediatric rare diseases voucher was denied because of certain interpretations of statutes involved and we disagree with that. I cannot dwell into this matter more than that

since this is sub judice at the moment. So, I don 't want to talk a lot about our position and the specific nature of our disagreement. But we believe that we have a defensible case and that 's the reason why we initiated this process.

Moderator: Our next question is from the line of Ashut osh from Zydus Investments. Please go ahead.
Ashutosh: For PROSEEK study, sir, when can we expect full analysis results?
Anil Raghavan: I couldn 't follow the person. Did you ask the availability of the full analysis results of PROSEEK?
Ashutosh: Yes, sir.
Anil Raghavan: We are looking to complete the full analysis of PROSEEK by the second quarter of this financial year.
Moderator: The next question comes from the line of Vishal Bohra from MK Ventures. Please go ahead.
Vishal Bohra: Sir, just want to underst and, are there any qualitative data points that you can share further on the PROSEEK trial or while we did not meet the primary endpoint, was there actually no efficacy outcome at all or like no response at all or was the response there, but not sufficient to cross the hurdles, what was the case here?
Anil Raghavan: In the PROSEEK interim analysis, we looked at the primary endpoint, which is Part -III of MDS - UPDRS and few secondary endpoints, which is Part -II plus Part -III and the time to treatment as in the time to systematic therapy. So, this was a subset of the secondary endpoints we have in the trial plus we also looked at three biomarkers, which is neurofilament light, we 've looked at alpha -synuclein assay, which essentially diagnose the disease and then in CSF fluid look at the trends of alpha -synuclein. So, in the primary endpoint and as we have indicated we did not meet the significance between placebo and treatment arms and we have a similar trending for these set of secondary endpoints that we 've looked at . In both these cases, what I can say at this point is that we have an extraordinarily high placebo response. In fact, the placebo curve stayed flat for almost the study period, but we are not disclosing the specific data at this point and we will take a look at all the secondary endpoints and also all the biomarker endpoints. In biomarkers, we have conflicting data points coming from that biomarker study. So, we will take a look at both the clinical outcomes across all secondary endpoints. We have looked at the first nine month period data in this analysis. We also have an additional nine month data for a subset of this patient pool. And we will also take a look at how this trajectory kind of pan out with that. That 's obviously an uncontrol led data as in it doesn 't have a placebo control in the second part

of this
trial. So, we will take a look at both the second nine -month period, all biomarkers and make a final, view on the target and trial and the drug.
Vishal Bohra: So, a couple of related quest ions. So, just to understand, we are saying that the trial did not fail because of poor outcome or a poor data on V odobatinib but more because your placebo response was extraordinarily good. Could it be a case of maybe a trial design or an externality? Do you think that there is a possibility that you may want to look at a retrial when maybe revise the trial design or revise the endpoint and look at it again, is there a possibility for that, sir?

Anil Raghavan: I wouldn 't frame the way you framed it. I thin k when the study was designed for showing a 35% difference between placebo and drug, 35% improvement over placebo. Obviously, the analysis that we have seen so far did not show that difference. And we have to wait till the full analysis of the data for any additional steps that we may have to take. In terms of actually factoring in V odobatinib into the financial model, our advice at this point is discount V odobatinib fully based on the trend lines that we have seen, and if there is a room for any kind of re framing this hypothesis or reframing based on any additional trials, we will definitely come back and advise you on the basis of additional review that we are going to take.
Vishal Bohra: I may just want to frame the question differently. In terms of V odob atinib PROSEEK trial out next steps is a retrial. One of the possibilities, sir, even if it is, let 's say, 1% possibility, is that one of the option that is still available to us or that is completely closed out?
Anil Raghavan: I cannot close out anything at this point because we haven 't seen the full data set. Unless we see the full data set and we understand what happened in the trial fully. All of these options are possible, but there are very remote options based on what we 've seen on the primary endpoi nt and secondary endpoint. That 's why we are advising you to discount the program in the model.
Vishal Bohra: What is the backup option we said, I think was it referring to the Alzheimer 's disease or are you referring to backup as V odobatinib for CML and LBD?
Anil Raghavan: So, the hedge for V odobatinib as in the hedge for PROSEEK was essentially CML. Even though we have other preclinical programs for V odobatinib and this backup series in neurodegenerative diseases. We always considered that as one package , one battery which will come alive if we have a positive response in PROSEEK. In the absence of that, the real alternative for V odobatinib

is leukemia program, which is what we just discussed.
Moderator: The next question is from the line of Manish Jain f rom GormelOne LLP. Please go ahead.
Manish Jain: I just wanted to understand that in CML given that the way Asciminib has been ramping up to $450 million sales already at an annualized run rate, does it make sense for us to take it all the way to the final approval?
Anil Raghavan: If the question is about SPARC spending the resources to take this to the regulatory approval in the US, the answer may not be yes. It may not be an appropriate use of SPARC 's capital to take that all into regulatory approval in t he US, given other options or opportunities that we have in the portfolio. But there may be other midsize, some critical companies for sure who may be interested in a program like this which can give significant enough opportunity from a commercial standpo int for a smaller sales force. And with opportunities to go up against a product which is ramping up fast and there are not very many options and especially the safety profile that we have seen for the product and the activity that we have seen for late li ne patients makes it a developable option. And we are seeing validation for that. Even after we kind of deprio

ritize d the program we get several requests for off -trial access for that drug because of its activity in patients who exhausted all lines of ther apy. So, that's an interesting proposition for

someone who is looking to build a $200, $300 $400 million product, but that may not be a right opportunity to kind of prioritize our capital over the ADC or over the potentially first -in-class dermatology prod uct.
Manish Jain: My second question was that in phenobarbital, given that we have licensed it to an external party, should we win the PRV case, the entire PRV value belongs to us or we have to share some proceeds with the partner as well?
Anil Raghavan: The PRV is with SPARC.
Manish Jain: So, 100% value of PRV is with SPARC, right?
Anil Raghavan: Yes.
Moderator: The next question is from the line of Chandpal Singh, who 's an individual investor. Please go ahead.
Chandpal Singh: Is this the end of the c -Abl hypothesis for other drugs that are in trial for Parkinson 's?
Anil Raghavan: Well, at the moment I don 't think PROSEEK interim analysis is certainly not the last word for the mechanism. I mean it 's a serious setback for the mechanism. But we need to basically look at the full data set. We are practically sitting at a Treasure Cove of information in terms of how the mechanism actually behave. So, in that sense, whether it is a tractable hypothesis i s something that needs to be determined based on the full analysis of the data set.
Moderator: The next question comes from the line of Rohan Parekh from Home Stock Brokers. Please go ahead.
Jigar Valia: Hi, this is Jigar Valia. So, my question is while V o do failed on the primary and secondary, it did cross the blood brain barrier through, so it 's a question, did it actually go through with regards to the 442 patients and is that anything of value or nothing happening?
Anil Raghavan: Can you just repeat the last line of your question?
Jigar Valia: So, my question was that did the results show a successful crossing of the blood brain barrier while it did not meet the primary, secondary endpoint? If indeed, then is that of value for either us or as far as the therapy or a study is concerned generally?
Anil Raghavan: That's an important question, Jigar, because the interim analysis did not include PK analysis. Even though we have access to both blood cross from our patients and also CSF data for an appropriate n umber of patients, we haven 't analyzed that to determine the exposure that we have both peripherally and in the brain. So, that is an important part of the analysis that we will be doing as part of the complete review.

Jigar Valia: Second question is with regards to the CML, the failure that happened for the PD thing, those primary, secondary endpoints would be entirely listing as far as the CML is concerned, and this is completely linked to PD and that would be linked to CML, is it fair that there is no co rrelation between?
Anil Raghavan: Primary and secondary endpoints of the Parkinson 's trial is very specific to Parkinson 's and the progression of that neurodegenerative condition, and it has got nothing to do with leukemia and it's proof -of-concept for leu kemia. It 's already available from our earlier Phase -II study. And if anything, the safety data from this trial is supportive of its dose in leukemia, which is actually lower than the doses that we have used in the Parkinson 's trial. We don 't see any negative fallout of PROSEEK in the leukemia program. If anything, it is positive because of the safety profile.
Jigar Valia: And for the leukemia program we continue after Phase -II as Imatinib plus one more as a Line -III product, right?

Anil Raghavan: No, we are essentially in the process of having conversations with the FDA and actual treatment line for the trial will be decided based on the feedback that we receive from FDA. But the study t

hat we have done so far was in three lines failed patients. That i s the original proof -of-concept, and that is probably the most difficult setting to go after because they 've exhausted all available lines of therapy. If we have an opportunity to move up and test a different line would be a function of what we hear from t he agency.
Jigar Valia: As far as the funding is concerned, it would have nothing to do with the timelines for the final study which will be in Q2 and it would also be a function of you having the discussion with FDA or -?
Anil Raghavan: No, no, the fundra ise is not a function of our conversations with the FDA. Because as I explained earlier to another question, we are not looking to allocate our capital for developing this program in leukemia. We are initiating a process to actively look for licensing part ners for leukemia. So, in that sense from a cash flow funding standpoint, these conversations and our actual plans for leukemia may not be a major factor.
Moderator: The next follow up question is from the line of Ashutosh from Zydus Investments. Please go ahead.
Ashutosh: I wanted to ask, what is the trial cost?
Anil Raghavan: The overall budget for PROSEEK was in the range of $45 million. But we may not end up spending all of $45 million because that assumed completion of the trial all the way to visit le vel, that's 40 weeks for the Part -I and completion of the Part -II study. So, now because of the early termination, we may be able to have some savings on that $45 million original allocation.
Ashutosh: Sir, can I ask you how much you have spent?

Anil Ragha van: Currently, it may be in excess of $35 million and original budget was $45 million.
Ashutosh: Currently, how much?
Anil Raghavan: I don 't have an exact number right at the moment, but it is certainly between 35 and 40, but closer to 35 than 40.
Moderat or: We have the next question from the line of Mr. Vishal Bohra from MK Ventures. Please go ahead.
Vishal Bohra: Sir, if you can highlight the next steps on your specialty pipeline, if you can just highlight the liquidity events, monetization events therei n and also what kind of milestones and we look at what next steps across SEZABY and ELEPSIA and the other one?
Anil Raghavan: I think the major milestone payment from programs which we already licensed, SEZABY on clearance of the market, we have additional milestone payments which will happen as we move forward and clear the other products from the market and get our exclusivity established. PDP - 716, which is our ophthalmology program, we 're going in for refiling in September timeline and we have milestones on approval of that product and Vibozilimod which is the Phase -II program in psoriasis and atopic dermatitis and successful completion of those programs would have additional milestone events. So, those are the three identified milestones for us. Other th an that you have further clinical and developmental and regulatory milestones for Vibozilimod. We also have these two other programs which I talked to you about, which is SCD -153 and SBO -154. SCD -153 is a collaboration with Hopkins in alopecia areata and t hat we just started our Phase -I clinical trial in India. We are on a dose escalation phase . And our intent is to establish a clinical proof -of-concept for that program in alopecia areata before we start looking at the partnering options, SBO -154 is antibod y drug conjugate targeting new epitope of MUC -1 and we are in the process of scaling up our manufacturing for the clinical program and its non-clinical program started. Our hope is that we will be able to do the IND for this program before the turn of this financial year. So, those are the two major programs which are closer to encash other than the three I spoke about license program.
Vishal Bohra: And there 's not much mention in the last couple of presentations about Vibozilimo

d for psoriasis.
So, in the call today I think you mentioned atopic dermatitis. So, can you highlight for what 's the status for psoriasis as well?
Anil Raghavan: Our commercialization partner, Sun hasn 't given a timeline for the completion of the psoriasis program. The psorias is program is behind atopic dermatitis program. So, it will follow the results of atopic dermatitis program and we don 't think that we will have data availability from that program this year. That 's why we 're not talking about that. But that program also is progressing and probably Sun can give an update on specific timelines for the psoriasis program.
Vishal Bohra: What would be your estimate as to SEZABY we when do we start seeing the commercial going in meaningfully once you have the exclusivity establi shed, would you have a timeline sense to it?

Anil Raghavan: We had filed a citizen 's petition with FDA and FDA came back to us asking more time. They said that this is a complex matter, and it will require additional time and they did not set a specific timeline in terms of when they will act on the citizens petition. In this process, we are also adding additional leverage points in terms of working with patient advocacy groups and also directly engaging with some of the unauthorized manufacturers. If I loo k at what happened with other programs which came into a market similar to SEZABY usually takes two to three years before the product gets full exclusivity established. So, we just completed the first year and now into the second year and we have a full -fledged process going and our hope is that we will be able to maintain a similar timeline.
Vishal Bohra: For ELEPSIA and XELPROS, any further updates?
Anil Raghavan: We are in the process of transitioning ELEPSIA to a different partner, but we don 't have finalization of that process yet. So, we 're still in the process of decision there.
Vishal Bohra: Just one more thing on V odobatinib for CML. In terms of head -to-head, what are the options if we have to go for a frontrunner trial, do you have any specific opt ions earmarked or highlighted that you have decided that this would be most relevant comparison?
Anil Raghavan: So, if you were to do a comparative study which looks like that 's what we should do, and most relevant comparative product would be Bosutinib but we will take that final call based on the FDA feedback.
Vishal Bohra: Is it possible for us to do maybe more than one arm of study given that couple of the other products are more prevalent where it 's Dasatinib or Nilotinib.
Anil Raghavan: That's again a function of the design, which we agree with FDA. I 'm assuming your question is about multiple comparative. Is that what you 're saying?
Vishal Bohra: Yes, because Dasatinib being the largest and it 's a $2 billion molecule in that ballpark in any case. So, would it be not fair for us to target that trying to do head -to-head comparison against something like Dasatinib and if you 're able to do equally well, or maybe better, that opens up a faster, bigger commercialization possibility you have, then the $200 to $400 million, so is that ballpark of a few hundred million that you highlighted?
Anil Raghavan: I think it 's not just a regulatory question. It 's also a clinical science question in terms of how you actually maximize your probability of success with the p rogram. And if you look at the second or third generation trials that happen in leukemia and if you look at what has been the comparator for those programs, that will give you a clear sense of what is the preferred comparator program for even programs like Dasatinib or any other like second or third generation program that has been pursued. It 's a complex question which is driven by both commercial calculus and also probability of success and the ability to kind of reduce the risk in the program. So, we wil l take that call ba

sed on what we hear from the agency.

Moderator: The next question is from the line of Manish Jain from GormalOne LLP. Please go ahead.
Manish Jain: I just wanted to understand on SCD -153. Till what level will we develop it on our own and when will we explore partnering 153?
Anil Raghavan: So, the development program for SCD -153 has three identified steps so far. One is completing its healthy human volunteer as in single ascending dose trial which is where we are at the moment. That establ ishes early safety for the program. And typically in a program like this, this would have been followed by a 14 -day multiple ascending dose study. So, instead of doing that 14-day multiple ascending day study, we are proposing to do a patient study in mult iple ascending setting, in a sense, we will test this trial in ascending doses in alopecia areata patients. So, that will give us two things. One, it will give us a better sense of safety profile in actual patient setting and even though we are not powered to detect that it may provide an early signal on efficacy and then we will follow that up with Phase -II study to determine the dose and also get proof -of-concept for the mechanism. And we are planning that in two steps. The first part is an India -based st udy where we will go up to 250 patients in India and do an interim analysis for those India -based patients. And if we get a proof -of-concept for 250 patients, that protocol will have a provision to expand the study to the rest of the world. But that India -based proof -of-concept for 250 patients is what we believe as an ideal licensing opportunity given where we are.
Manish Jain: The second clarification I wanted is for ADC which manufacturing ramp up that we 're talking about. Is it in -house manufacturing ca pability that we have developed?
Anil Raghavan: No, we are working with the San Diego -based CDMO for scaling up this program.
Moderator: The next question is from the line of Rohin Kumra, who 's an individual investor. Please go ahead.
Rohin Kumra: Just to be sure, when we say that V odobatinib will be out-licensed in the next two or three years, that means we will also outlicensing it for our front runner program also?
Anil Raghavan: Sorry, your line was very blurred and I couldn 't hear you properly.

Rohin Kumra: When we see that V odobatinib will be outlicensing in next one or two years?
Anil Raghavan: You're talking about the CML arm of V odobatinib, right?
Rohin Kumra: Yes, sir, CML arm. Is it a fair assumption to assume that we are out -licensing it for our front runner program also?
Anil Raghavan: So, I understand your question as whether the CML part of the V odobatinib would be out - licensed in this year or next year. The answer is that 's our objective, right? We are initiating this

process and as I said ea rlier, our objective is to finalize that process this year. I didn 't quite follow the second part of your question.
Rohin Kumra: I mean that we are also trying V odobatinib in the front line settings.
Anil Raghavan: Yes. If it is out -licensed, it will be fo r all settings of V odobatinib in leukemia not just the last line.
Rohin Kumra: Then that means that milestone payment that we are going to see while keeping in mind the potential of process and settings.
Anil Raghavan: That you need to model, but yes, it will cover both the front line setting and the earlier line as well as the last line.
Moderator: The next question is from the line of Ashutosh from Zydus Investments. Please go ahead.
Ashutosh: Sir, for V odobatinib is there any corporate presentation d eck available on the recent reports?
Anil Raghavan: There is a presentation on the website. You 're asking about V odobatinib, right?
Ashutosh: Yes.
Anil Raghavan: Yes, yes, it 's on the website.
Ashutosh: Where it is?
Anil Raghavan: It's on the SPARC website.
Modera

tor: As there are no further questions, I would like to hand the conference over to Mr. Jaydeep Issrani for closing comments.
Jaydeep Issrani: Thank you, Sagar, and thank you, everyone for joining the call today. In case you have any additional quest ions, feel free to reach out to us on the e -mail IDs and numbers that are shared on our website. Thank you again for being with us on the call.
Moderator: On behalf of SPARC, that concludes this conference. Thank you for joining us. You may now disconnect your lines.

Call: Dec 2024

SPARC/Sec/SE/202 4-25/059 December 24, 202 4

National Stock Exchange of India Ltd.,
Exchange Plaza, 5th Floor,
Plot No. C/1, G Block,
Bandra Kurla Complex,
Bandra (East), Mumbai – 400 051.

Scrip Symbol: SPARC BSE Limited,
Market Operations Dept.
P. J. Towers,
Dalal Street,
Mumbai - 400 001.
Scrip Code: 532872
Dear Sir/Madam,

Sub: Transcript of the Investor’s call for an Update on Clinical Programs and R&D Pipeline

This is with reference to our Investor Call which was scheduled on December 19 , 2024. Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, w e hereby attached the Transcript of the said Investor Call for an update on Clinical Programs and R&D Pipeline and the same is also available on the website of the Company on the weblink - https://sparc.life/presentations/

This is for your information and dissemination purpose .

Yours faithfully,

For Sun Pharma Advanced Research Company Ltd.

Kajal Damania
Company Secretary and Compliance Officer

“Sun Pharma Advanced Research Company Ltd. (SPARC)
Update on Clinical Programs and R&D Pipeline Conference Call”
December 19 , 2024

MANAGEMENT : MR. ANIL RAGHAVAN – CHIEF EXECUTIVE OFFICER
DR. NITIN DHARMADHIKARI – CHIEF OPERATING OFFICER
DR. NITIN DAMLE – CHIEF INNOVATION OFFICER , HEAD BIOLOGICS
DR. VENKATA PALLE - GLOBAL HEAD, DRUG DISCOVERY & PRECLINICAL DEVELOPMENT
DR. VIKRAM RAMANATHAN – HEAD PRECLINICAL DEVELOPMENT
DR. YASHO RAJ ZALA – HEAD DRUG DELIVERY SYSTEMS
DR. MUDGAL KOTHEKAR – VICE PRESIDENT , CLINICAL DEVELOPMENT - IMMUNOLOGY
DR. SANDEEP INAMDAR – VICE PRESIDENT , CLINICAL DEVELOPMENT - ONCOLOGY
MR. ANUP RATHI – CHIEF FINANCIAL OFFICER

Moderator: Ladies and gentlemen, good day and welcome to SPARC'S Update on Clinical Programs and R&D Pipeline Conference Call .
As a reminder, all participant lines will be in the listen -only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star, then zero on y our touchtone phone.
Please note that this conference is being recorded.
I now hand the conference over to Mr. Jaydeep Is srani from SPARC. Thank you and over to you.
Jaydeep Is srani: Thank you, Yashashree. Good evening, ladies and gentlemen. My name is Jaydeep I ssrani , I head the Business Development and Investor Relations at SPARC.
On behalf of SPARC, I welcome you to SPARC's yearly Update on Strategy and Program Updates. I am joined by our CEO, Mr. Anil Raghavan, and members of SPARC's senior management team for the call today.
The format for today's discussion will be similar to our previous presentations. That is, SPARC presenters will walk you through the slides, after which the call will be open for questions and discussions.
The presentation for t oday's discussion was shared earlier.
We hope you have received it.
Before we start today's discussion, I would like to remind you that our discussion today includes forward -looking statements that are subject to risks and uncertainties associated with our business. Hence, actual results may be different from those projected in today's presentation.
I will now hand it over to Mr. Anil Raghavan for his presentation. Over to you, Anil.
Anil Raghavan: Thank you Jaydeep for the introduction.

Good evening everyone. It gives me great pleasure to welcome you to SPARC's 14th annual portfolio update. Thanks so much for your continued engagement and trust. We couldn't have reached where we have without your steadfast support every step of the way. Thank s again, and thank you for taking the time to be with us today.
As Jaydeep said, w e will follow our regular discussion flow, starting with a commentary on our strategy and operational priorities followed by a report on the progress on key programs. My intent is to capture the most important elements of our post PROSEEK plan and set expectations for the short to medium -term. I will also touch upon several programs which we do not plan to cover in the program updates. On that part of the presentation, we will focus o n reviewing the progress made on two specific programs which are super important for SPARC going forward.
I am delighted to introduce Dr. Mudgal Kothekar and Dr. Sandeep Inamdar who recently moved to therapeutic leadership roles for Immunology and Oncolog y, respectively. Mudgal and Sandeep have been with SPARC for many years and will play important leadership roles in shaping our portfolio in their respective therapeutic domains.
Mudgal will discuss our SCD -153 program for Alopecia Areata and potentially other dermatology autoimmune conditions. Sandeep will go over the M UC-1 ADC program, SBO -154 with us.
So, with that, let's start where we have left off in our last call. Slide 4 please. This is an overview of the PROSEEK full results. As we mentioned duri ng interim analysis call, we have closed the study at 491 patients immediately after interim analysis results were out. We have also discontinued the long - term extension study with roughly 100 patients completing that leg of the program. We have now comple ted the full analysis, including the planned biomarkers and several post hoc questions. Unfortunately, the results from the full analysis closely matches the interim analysis trends that we shared with you.

The MDS -UPDRS part III trends are shown here in the graphs below. Mean of the change from baseline was the primary endpoint, which also showed a similar trajector

y without adjusting for drop -outs. But in model -based analysis, adjusted for drop outs, drug arms marginally deteriorated and underperformed t he placebo. Placebo performance in this study has been a clear outlier, with a modest improvement in MDS UPDRS part -3 vs both, natural history and trends from other trials in early -stage Parkinson's disease . And on biomarkers, while most markers mirrored t he clinical trend, a key mechanistic biomarker, alpha -Syn in CSF bucked that trend and demonstrated a reduction in the high dose arm, a result that actually supported the c -Abl hypothesis. P harmacokinetics , PK from both plasma and CSF, were consistent with prior studies and expectations. In fact, in the CSF, even at the low dose, we had multi -fold coverage for the c -Abl IC 50. We have also analysed the LTE trends which was not a controlled study. All placebo patients transitioned to high -dose post week 40 in line with the LTE study design. Patients who remained on the drug continued to deteriorate slightly when we adjust for drop -outs using a statistical model. We used MMRM, a widely used statistical technique for making such corrections.
While it is difficu lt to draw firm conclusions without a placebo arm, it appears that these patients deteriorated slower than expectations based on natural history studies even in the long -term extension arm. We plan to review the full results with our scientific advisory bo ard later this month and looking forward to publish ing the study formally as soon as possible . That's all I wanted to say at this point. We can revisit PROSEEK during our Q&A if you have additional questions which I am sure some of you may have . Slide 5 please .
Understandably, PROSEEK was a significant turn for us. Now as we pivot from that chapter, we have to address the important, top of the mind question that ourselves and our investors have…… regarding the value of the residual portfolio, and how do we resource them? We spent a lot of time after PROSEEK data readout, analysing our other active programs and potential options and settled on three important priorities for SPARC going forward –

First pillar is having an optimized portfolio with a narrower therapeutic area focus with SC D-153 and SBO -154 as the anchor assets, that's Itaconate and MUC-1 ADC.
Second ly, we have to adopt a more flexible business model for resourcing and encashing our assets and capabilities, this means for example, a willingness to partner key assets much ea rlier than we have been traditionally exploring or we were comfortable exploring, an d third, finally a sharper focus on execution of short - term, cash generating opportunities. We have several milestones with varying degrees of likely impact, varying proba bilities of success and time to event horizons. Now let me talk to all three of these pillars in a bit more detail in the rest of my presentation.
Let's start with slide 7 on portfolio optimization .
Let's first look at what is not here. Neurodegenerative diseases. While our challenges with Vodobatinib were truly multi -factorial, couple of key ones stood out on reflection. Reliability of animal models and viability of appropriate clinical trial designs in terms of duration of the study , number of patients, availability of reliable biomarkers, etc. These were two of the most important. We had several programs in the neuro -degenerative diseases area, going into the P ROSEEK data event, pursuing similar or complimentary hypotheses. Since the outcome of PROSEEK which was a real downer for these programs, we have parked all but a couple of early platforms building efforts in the neurodegeneration space, pending our review with our SAB and determination of ways to mitigate the key risks.
In the interim, Oncology a nd Immunology will remain mainstay of our portfolio and they are somewhat natural choices for us from both an oppor

tunity attractiveness standpoint and capability maturity perspective.
Oncology represents significant unaddressed disease burden and pricing

support, in spite of intense competition. We will focus on two key themes which enjoyed a lot of success off late. Smart delivery of cancer drugs, mediated either through an antibody or small molecule ligands. We will look to deliver a variety of pay loads including chemo therapeutics, immune activators, and other targeted therapies. Another theme we are excited about

is new synthetic lethality targets which can work in either PARP resistance or in new synthetic lethality pairs. We have a couple of interest ing ideas that we are currently pursuing. One key element of this approach, especially the smart therapeutics part, is its modular nature. Many of its components, be it targeting moieties, linkers or payloads can work in multiple combinations and permutati ons, which gives us significant efficiencies in terms of discovery and early preclinical development .

Immunology is a bit more nuanced picture; the autoimmune field has seen significant level of success moving the standard of care substantially in recent past. Mostly on the back of resounding of success of antibodies blocking key cytokines such as IL -23, 17 in Psoriasis and 4 and 13 in A topic Dermatitis . But the field really has a sub -optimally addressed need for a safe oral or topical alternative for biolo gics depending on disease severity. J AK inhibitors which provide an effective alternative have their own liabilities on safety. That's where SPARC is trying to position in Immunology. Novel mechanisms which can lead to safe topical alternatives to Biologic s and JAKs. We believe it will also meet ano ther key unaddressed need in this space, that's for combinations which can improve efficacy and therapeutic windows. We will have more to say on this later on. N ow let's move to s lide 8 please .

Bit more on 154 an d 153 programs . We have introduced these programs in our earlier calls. Let me give you a quick recap of these programs as a refresher. SBO -154 is an Antibody Drug Conjugate which targets a novel combinatorial epitope of a tumor associated antigen called M ucine -1. As you know , we have in -licensed these unique antibodies from a university of Tel - aviv start -up called, Biomodifying . 154 is the first product on that platform and it leverages a robustly validated linker and payload, that is MMAE using a PABC linker. We have already achieved preclinical validation for several key aspects of the hypothesis which we will go over in the n ext set of slides.

SCD-153 is a pro -drug of an analogue of an endogenous immunosuppressive metabolite called Itaconate, which was originally developed by a team at Johns Hopkins. We have built the 153 program on multiple topical formulations of this itaco nate analogue which we believe, can be effective interventions in Auto -immune disorders like Alopecia Areata and Vitiligo. The

program has completed its 'first in human' single ascending dose study recently and a multiple ascending dose study in Alopecia A reata patients is expected to start in early part of 2025. Mudgal will walk us through the program in a bit more detail. As I mentioned earlier, we are really excited about the platform nature of both these assets. Upon clinical validation, both assets can deliver multiple indications and products in the monotherapy setting as well as in combinations. I want to talk a bit more about the promise of these two opportunities in the next couple of slides before I move on from the portfolio optimization priority , starting s lide 9 .

The ADC field has been on fire recently on the back of the success of products like Enhertu and Trodelvy. And in fact, a lot more is in the works , making ADCs one of the hottest, busiest areas for innovation in Oncology, and hopefully even beyond, particu

larly in Immunology in the future. What is making this surge possible ? Evolving maturity of linker payload technologies is a big factor in addition to opening up of several key technology elements such as linker systems, giving broad free dom to operate. The field is also learning from growing clinical experience about optimal DARs, i.e., drug antibody ratios, effective ways of management of typical adverse events, viable dosing regimens and so on and so forth. SBO -154 looks to leverage som e of these key learning from the field's recent success with a potentially novel targeting moiety, the SEA domain of Mucine 1. Why is that important ? Let's get to the

slide 10 .
A lot of intense activity that we are currently witnessing in the ADC space is driven by classic herd mentality. Take a look at the antigens targeted. A lion's share of these programs target s less than 10 cancer specific antigens. Even within that distribution, two antigens HER -2 and TROP -2 account for a vast majority of current acti ve programs. In our view, HER-2 and TROP -2 boats have already sailed for classic ADCs with chemotherapeutic payloads, which is what almost all of these programs are trying to do. So , there is a real dearth of potentially high impact targeting agents and th at's what we believe what gives M UC-1 SEA a distinctive edge. MUC -1 alpha has been and continues to be a target of active interest for ADCs and even other approaches like cancer vaccines. All these programs need to swim against substantial blood levels of

cleaved alpha region which makes tumor targeting difficult. 154 overcomes that issue by focusing on the SEA combinatorial epitope which improves its tumor specificity significantly. We now have validation for two key elements of this idea – the relatively low levels of floating SEA compared to floating alpha in patient plasma and the appreciable surface expression of the epitope of our interest on highly prevalent tumor types. Sandeep later in the presentation will discuss the data from these validation stu dies in greater detail. As I said earlier, if we successfully reproduce these outcomes in the clinical setting, that gives us a new viable targeting moiety which can be used extensively with other types of modalities such as other cytotoxins, immune activa tors and T-cell engagers etc. That's certainly exciting.
And now a brief update on SCD -153, slide 11 please .
This slide builds on a couple of points I mentioned earlier on. In spite of its enormous commercial success, the clinical impact of biologics is l imited to more advanced patients fighting severe manifestations of certain diseases, such as A topic Dermatitis , Pso riasis , IBD, R heumatoid Arthritis , etc. The number of lives impacted is far fewer than patients who are managed with small molecule orals or topicals. The real issue here is the stagnation in non - biologics standards of care both in terms of safety and efficacy. So , moving the therapeutic needle would require new approaches which expands choice for physicians and patients fighting these difficul t diseases either as standalone agents or potential combination partners. That's the real promise of SCD -153. We have learned so much about the pathway and the new chemical entity in our productive collaboration with JHU and are really looking forward to s ubsequent phases of clinical development. Mudgal will review some of the science and early clinical outcomes with us later in the call.
So, let me leave this segment with two or three key messages.
SPARC will pursue select themes in Oncology and Immunolo gy for its portfolio build going forward. That's leveraging tumor specific delivery options and synthetic lethality in Oncology and novel pathways which can become topical options in certain dermatology auto immune conditions .

SCD-153 and SBO -154 offer pot entially high value options and ideal vehicles t

o test this approach with great upside if we are successful .
And we will direct our resources preferentially to developing these programs to its clinical inflection points and these will rem ain the primary fo cus of post PROSEEK SPARC. Now , let me quickly go over the next two tenets of our strategy going forward. Starting with a key shift in our partnering approach and I will use the SCO -155 program to illustrate the change we are trying to highlight . Slide num ber 13 please .
Certainly, one of the challenges we have, coming out of a costly clinical data setback, is the resource constraint that we have to navigate. Committing to continue developing the prioritized assets would consume a significant share of resou rces that we have access to. That leaves several important programs in our portfolio with very difficult choices .
Historically our intent has been to stay on to our programs at least till we obtain clinical proof of concept and enter late -stage clinical de velopment. We

need to re -examine that construct and that is what we are intending to do. We will look for partnerships at an earlier stage of development. In addition, we will also look for alternative structures, like asset specific NewCo creation. We hav e a specific example for this shift in approach, coming out of our work with UCSF. I want to go over the program and the construct in a bit more detail in the coming slide. But before I do that, let me also make a brief comment on another business model op portunity which we have shied away from in the past , that's leveraging our discovery and translational capabilities in a services model to de -risk SPARC. Even though there are really strong tail winds in terms of market forces supporting India based servic es businesses, and a real need to de -risk SPARC for downside protection, we continued to stay on the side -lines for several reasons and we have spoken about them in the past. While that remain the case, we will continue to review our options and risks care fully and take a final position on this opportunity in the coming year.
Now to get back on our collaboration with UCSF, let 's look at slide 14 .

This slide captures the broad timelines of our relationship with UCSF. UCSF was one of our earlier strategic c ollaboration with a master collaboration agreement taking shape in 2017. The program that led to SCO -155, was conceived in 2020 and we could identify a lead candidate with preclinical validation in two years flat. We are proud of the quality of collaborati on with the UCSF team and the accelerated nature of our early development.
We have explored setting up a NewCo for advancing this asset and that thought led to the formation of Tiller therapeutics. A company founded by a team of UCSF investigators, SPARC and UCSF itself. Earlier this week we announced the successful closure of the letter of intent between SPARC and UCSF to go ahead with this construct. SPARC and UCSF will license rights to its joint IP to Tiller Therapeutics and Tiller will raise external dollars to fast -track SCO-155 to clinic. We believe this is an exciting business model option which we can explore with many other programs in our early -stage pipeline which in the broader scheme of things allows more shots on the goal and certain level of risk mitigation at the portfolio level . Next slide, slide 15 has more color on the program per s ay.
PSMA has been targeted for tumor specific delivery of chemotherapeutics for a long time . The field had its first major breakthrough with the radio ligand therapy pluvicto which uses a small molecule ligand of PSMA to deliver radiotherapy. In spite of impressive data early on, the RLT field faces many challenges, leaving alternatives to come in and improve the outcomes. There is significant opportunity to red uce variability in therapeutic benefits plus improve overall safety profile, particularly reduci

ng the bone marrow toxicity.
Alternatives can also help overcome limitations in terms of life term hard caps in addition to all the logistical challenges in put ting together and distributing a radio therapy. SCO -155 provides a differentiated approach using a synthetic PSMA ligand for targeting, but delivering alternative payloads. We believe this approach helps to overcome some of the limitations of the radio lig and therapy as well as PSMA targeted ADCs which are tested so far. In an indication where unfortunately, there is progression with tragic quality of life and survival implications, SCO -155 can offer a very useful treatment option .

Please move to slide 16 which discusses some illustrative data from this program .
The graph on the left highlights efficacy in an in vitro system. The chart plots PC3 prostate cancer cells for both PSMA overexpressing PC3 -PIP cells and PSMA null PC3 -FLU cells. SCO -155 efficiently inhibits the PIP cells at less than a picomolar IC 50 and has a 1000 -fold advantage over the null FLU cells .
And on the chart over on the right side, we have an in vivo proof of concept from a PC3 xenograft model. Our small molecule drug conjugate at 60 microg ram/kg dose is tested in both PIP and FLU Xenografts against the vehicle. While SCO -155 in the FLU xenograft and vehicles in both xenografts did not make any impact . Look at the drug curve on the overexpressing PIP

model , it leads to a complete regression of the tumor.
These and other critical pieces of data validating this asset have paved its way to a set of IND enabling studies setting up its clinical entry in the short to medium -term . Slide 17 please .
As mentioned earlier, Tiller Therapeutics was formed between SPARC, UCSF and its scientific founders. SPARC will receive 55% of Tiller's initial shares in two tranches. This equity will be issued in full within six months of signing the definitive agreemen t.
Tiller plans to build its pipeline focusing on the S mall Molecule Drug Conjugate modality with additional targeting ligands and payloads in a wide variety of solid tumor milieu. We are super excited about many things here. Firstly, SCO - 155 as an asset and its potential to emerge as a true alternative to other PSMA targeted approaches. That is foremost. We are working with a very high profile and high impact team as scientific founders of Tiller. We believe in their ability to follow through and develop a pipeline using this approach. And finally, this experiment offers a viable alternative model to advancing interesting programs, particularly on the face of resource constraints. That's very promising as we continue to build differentiated preclinical pro grams, all fighting to find a way to clinic sooner than later.

That takes me to the final set of slides on my part of the presentation which aims to cover certain short -term cash catalysts which we are tracking closely. Let's move to slide 19 please .
In the next few slides I want to focus on two things. On the left bucket here, we have a bunch of short -term milestones with potential cash events. They come with a mix of probabilities which will be difficult to estimate accurately at th is moment, but they al l offer a definitive path to adding additional resources to support prioritized programs and beyond. And therefore, very important.
Equally important is optimizing our cost structure to the demands of our current portfolio and intent. Not just structural adjustments though, but finding ways to do more with less constantly. We have done quite a bit of this since PROSEEK IA and may have potential additional adjustments to make in the coming year depending on where the chips my fall in the first bucket . Let's take a look, slide 20 please .
We have four programs which can potentially lead to cash generating milestones in the short -term. Let's go one by one starting with Se

zaby, which
is our benzyl alcohol free Phenobarbital formulation which got approved in Nove mber , 2022. There are two potential opportunities which we are aggressively pursuing, that's finding a way to convince the agency to reconsider the denial of a pediatric rare disease voucher which we believe we were eligible for. And secondly, convincing t he agency to enforce the orphan drug exclusivity that Sezaby was granted. I will cover both these opportunities in bit more detail in the next slide .
Vodobatinib's CML opportunity is the next one in this set. It was always meant as a hedge for the PD progr am and I will update you on where we stand on our efforts to find a development and commercialization partner .
PDP-716 is another asset with potential access to cash. As you may remember, we received a complete response letter which primarily cited the unacceptable regulatory status of our API partner. Since then we have replaced the API source and made certain changes to the process. Primarily

moving to a higher volume capacity for the finished product manufacturing.
On the partner end, Visiox went throug h a couple of ownership transitions involving a SPAC deal which did not conclude and a merger with another pharmaceutical company. We are working very closely with the current management of Ocuvex, that's their new name, to complete the CRL response by 2nd quarter of the next financial year and ensuring a successful launch once we get the approval which we hope to get before the turn of FY26. PDP - 716 launch has a significant milestone attached to it.
We have also made substantial progress with Vibozilimod, which has a couple

of phase two studies targeting AD and Pso.
Let's start with Sezaby. Over to slide 21 .
Let me start with a caveat on the potential pediatric rare diseases voucher. This matter is the subject of an active litigation and there are signifi cant restrictions in terms of how much we can discuss. So, I will limit my comments to providing necessary background and the potential impact of a positive outcome.
PRD voucher program was established with an intent to incentivise development of better a nd safer medications for pediatric rare diseases. The PRDV statute lays down very specific qualifications for a product to meet in order to be eligible for a pediatric rare diseases voucher. During the approval of Sezaby in 2022, FDA denied us the voucher and shared with us the agency justification supporting the denial which was built on a certain interpretation of the PRDV statute. SPARC believes the agency interpretation led to an unfair denial of the voucher and we are committed to exploring all availab le options to correct this. We have been on this path since the approval including directly presenting our case for reconsideration to the agency. After exhausting all available reasonable options, we have approached the court for direction in February 202 4 and we expect the courts initial opinion on this matter in the last quarter of this financial year. We hope for a positive turn of events and if we get a favorable outcome, as you can see from the rest of this slide, PRV is a highly valued tradable devis e which has recently been sold for values in

excess of 150 mn USD. Unfortunately, we cannot go into any more detail on this here, given the active status of this matter. We will keep you posted .
Now on the exclusivity piece, please go to the next slide please. Slide 22 .
When the agency approved Sezaby, they granted SPARC a 7 -year orphan drug exclusivity, which if enforced will force marketers of unapproved formulations of Phenobarbital IV out of the market. FDA follows a long - standing policy of risk -base d enforcement of exclusivity. We have been working with the agency through this period to remove unapproved products from the market .
We have been in touch with the agency through direct representations and

using formal devices like citizen petition. Given the complexity of the case, FDA has indicated they need more time to formally respond to our citizen petition. In the meantime, we have been communicating with the marketers of unapproved formulations, making them aware that they cannot continue to be in the market as Sezaby is formally approved in the US market. This was done through formal cease and desist letters.
We have also been working to make our supply chain more robust by adding additional external capacity which is under agency review as we spe ak. We remain hopeful that the exclusivity will be enforced as it is not just a matter of getting unapproved products out. In addition to being the only approved IV Phenobarbital in the US, ours is the only product which doesn't have the potentially harmfu l excipients such benzyl alcohol. FDA has a stated intent to remove products containing benzyl alcohol, especially when it can pose significant potential risks to vulnerable populations such as neonates. So, going in to 2025, all hands on the deck and stay ing hopeful.
Now let's turn to slide 23 for an update on Vodobatinib .
Post PROSEEK IA results, we engaged with USFDA for CML to have clarity on registration expectations and agree on key elements of a phase 3 design. The schematic here on the top half of the slide captures the expected phase 3 program. There are a couple of major elements to highlight here in terms of FDA expectations. First the population – patients who have failed at least one

2nd generation TKI. We need to include a smaller dose -finding leg with at least two more doses other than proposed 174mg dose. So, in this design we have 20 patients at 200 and 130mg each. We have kept this an integrated protocol which can move to the phase 3 part of the program post analysis of the randomized dose -finding part. We have to get an agreement on the Phase 3 dose with the agency at this stage before we initiate a formal comparative

study against 500mg bosutinib.
Our intent is to keep the program at a state of readiness to launch a registrational program , while we finalize a potential development and commercialization partner. We have initiated that process in the second half of this year and have completed our initial outreach. As you can expect, given the niche orphan indication, we are working with a l imited field here. We are working towards identifying a partner by the end of this financial year.
Final program in this list is Vibozilimod, or SCD -44 which is licensed to Sun Pharma. Slide 24 please .
As you know, we have two active phase two trials for Vibozilimod ongoing in Atopic Dermatitis and Psoriasis. The most important update here is, we have achieved enrolment competition for both these programs. Atopic dermatitis 16 week topline is expected in Q4 of FY25, while Psoriasis is expected to reach topline readout in Q1, FY26.
At this point, we keenly await data and looking forward to working with our commercial partner to advance the program to phase 3 . Slide 25 please .
Coming into this year on the back of the PROSEEK outcome, one of key objectives wa s to find ways to preserve capital without sacrificing the most important portfolio priorities. So, as you can see, we have prioritized M UC-1 ADC and Itaconate AA program as our top execution priorities. Plus wanted to continue to shape these two programs and its extensions preclinically without losing momentum, in addition to exploring some of our more promising discovery programs. What are the implications of this?
We minimized all additional spend on the ongoing CML program and decided to focus on trans itioning the asset to a potential partner with a fully conceived

program. We found an alternative model for developing the SCO -155 which as you can see, we are really excited about. And even on our high priority programs, we have significantly increas

ed th e India clinical component to
keep the overall costs to clinical PoC's under check.
On the organizational side, coming into this year, we were gearing up for a significant Vodobatinib PD phase 3 program. Or even multiple programs. Given the early -stage na ture of assets we are assigning higher priority now, there was a significant mismatch between the scale of our clinical development team and our current scope. Or even the scope we are expecting in the short -term. As you can see here on these charts, we ar e currently at 3 24 against a planned FY 2025 headcount of 400+. This reduction is driven by significant down -sizing in the clinical development capability, here in India and also substantially in the US. Our US headcount dropped from 37 at the beginning of this year to 7 currently.
Our resourcing is certainly an evolving situation. We started the year with 15.2 mn opening cash. But with PROSEEK turning out the way it turned out, we had to rely on our smaller operating cash flow and approved debt limits which are primarily coming from our promoter group companies or from commercial banks with promoter group guarantees. This may take us to mid of Q1 next year. As I mentioned, we have several potentially cash generating catalysts in the short -term. We will rev iew where are at the end of this year before finalizing a medium -term resourcing plan for the company. We will keep you posted . Slide 26 please .
Here is a snapshot of our short to medium -term execution priorities. I have touched on objectives for Sezaby, Vodobatinib, PDP-716 and Vibozilimod in detail in the previous slides which I don't plan to repeat here. We have additional objectives set for SCD -153 and SBO -154 in this slide. But in the subsequent sections, Mudgal and Sandeep will give you additional co lor on the status and plans for these programs. So, let me not steal their thunder.
So, in closing, let me say this. We have tried to turn the page and focus on two very promising assets to have additional shots for SPARC. We have several other interestin g ideas preclinically which have important data milestones

coming up in short order. What we did with SCO -155 may give us a good template to progress these programs without committing additional resources from our end. In the short -term, we will stay super focused on achieving potentially cash generating milestones. Once we are through with that, we will take a hard look at the end of the year to see where we are and our options going forward and go from there. So, thanks for your time today. I will transit ion the call to Dr . Mudgal Kothekar at this point for an update on SCD-153 and looking forward to seeing you for the Q&A later. Thank you.

Mudgal Kothekar: Thank you, Anil. Good afternoon. So, I will provide an update on the SCD -153 program for the treatm ent of Alopecia Areata . As shown here on Slide 28, Alopecia Areata is an autoimmune disorder that results in patchy hair loss as shown here. It affects 2% of the global population and the prevalence is increasing. Some patients, especially with a mild dise ase recover spontaneously but most need medical intervention for hair growth. Corticosteroids are used off -label with limited efficacy and risk of side effects on long -term use. Recently approved JAK inhibitory carry black box warning for some serious side effects such as cardiovascular events, infections and malignancies. This hair loss in Alopecia Areata is because of infiltration of immune cells such as the CD8+ T cells around the base of hair follicles that attack and damage these cells.

Next slide, please. So, SPARC has developed a topical agent called SCD -153 for the treatment of Alopecia Areata. This is a first in class compound that targets the basic pathogenesis of the d isease. SCD -153 was evaluated in an animal model of Alopecia Areata in mice. The pictures on the left side, show the

animals in this model; SCD -153 at various doses and dosing regimens resulted in growth of hair in this model while the animals treated with vehicle showed no hair growth. The figure on the right side shows hair growth in terms of hair growth index. You can see that SCD -153 at most dosing regimes resulted in hair growth over time while there was no hair growth in the vehicle arm. The dose 3, r egimen 1 showed the most remarkable hair growth . Next slide.

Now i n the same study, we evaluated the effect of SCD -153 on the CD8+ T cell infiltration around the hair follicles. The figure on the left side shows a reduction in the CD8+ T cells at the base of the hair follicle compared to the

vehicle arm. As shown on the right side, there was a significant and dose dependent reduction in the CD8+ and NKG2D+CD8+ T cells in the skin that was treated with SCD -153. It may be noted that the NKG2D+CD8+ T cells are a type of CD8+ T cells that are specifically implicated in the pathogenesis of Alopecia Areata . On Slide 31 is an update on the Phase 1 program.

We have recently completed a Phase I study of SCD -153 in healthy volunteers. In this study, single increasing doses of SCD -153 were evaluated in five sequential cohorts of heathy volunteers with 8 subjects in each cohort. SCD -153 was well tolerated up to the highest dose strength evaluated. The maximum safe dose was not reached as the highest dose was also found s afe. No subject experienced dose limiting toxicities. Next Slide. This slide shows the drug related adverse events that were reported from the study.

One subject at the dose level 4 experienced mild erythema and burning sensation. Two subjects at the dose level 5 experienced mild erythema and one subject experienced burning sensation. These events were mild in severity and resolved without treatment.

On the next slide, slide 33, is an update on the planned study. In this study, skin biopsies were taken 24 hours after the drug application to estimate the concentrations of SCD -153 in epidermis and dermis. As shown here, detectable concentrations of SCD -153 were observed dose level 2 onwards with an increase in the concentration with increase in the dose. Next slide.

We have planned a phase IB study in patients with Alopecia areata in India. The protocol has been submitted to the DCGI. This is a randomized double - blind study to evaluate the safety, efficacy and pharmacokinetics of SCD -153 compared to vehicle in the AA patients.

The flowchart describes overall design of the study.

After screening assessments, eligible subjects will be randomly assigned in a 4:1 ratio to SCD -153 or vehicle arm in sequential cohorts of four dose strengths of SCD -153.

Initially, patients will be enrolled in the first cohort of dose level 1.

Subjects will receive a single application of the assigned treatment , SCD-153 or vehicle on day 1 followed by safety and tolerability assessments up to day 8.
Subjects who have tolerated th e single application of the drug will receive once daily application of the assigned treatment , SCD-153 or vehicle for 2 weeks followed by safety assessments on day 22.
Subjects will continue to receive the assigned treatment for next 10 weeks i.e. up to w eek 13.
Subjects initially randomized to SCD -153 will continue to receive SCD -153 for another 12 weeks. Subjects initially randomized to the vehicle will be switched to SCD -153 at the same dose or strength as the active arm of the cohort for the next 12 we eks. Patients will be enrolled in the subsequent dose level after the evaluation of safety data up to day 22 by a Committee. Next slide please.
To summarize, SCD -153 employs a new mechanism of action to address the complex immune pathogenesis that could b e implicated in a diverse range of clinical manifestations. In addition to providing an alternative option for treatment as a single age

nt, SCD -153 also has a potential to be used in combination with the established treatments to improve their efficacy in terms of durability of response or increase in the response rates. SCD -153 has a potential to be explored in multiple dermato -immunological disorders and multiple topical formulations of SCD -153 could be developed. Thus , SCD-153 is a topical first -in-class alternative that has a potential to address the limitations of existing therapeutic options.
On the next slide, that is Slide 35, are the projected milestones for this program. The Phase IB study in Alopecia Areata patients will be initiated in the Q1 of FY26. We will get interim readout from this study in Q1 of FY27 and we target to initiate a global Phase IIB study in Q4 of FY27.
So here I conclude the update on SCD -153 and I hand over to Dr. Sandeep Inamdar for an update on SBO -154.

Sandeep Inamdar: Thanks Mudgal. So, we will now shift our focus to SBO -154, an anti -MUC -1 antibody drug conjugate with a monomethyl auristatin or MMAE payload being developed for the treatment of multiple advanced solid tumors. So, MUC -1 is a highly glycosylate tra nsmembrane protein consisting of an extracellular alpha subunit, a membrane proximal beta subunit and a short cytoplasmic tail. It is widely expressed in normal globular epithelial cells such as the lining of the gastrointestinal and respiratory tracts. It is normally expressed exclusively on the apical or glandular surface of the epithelial cells. However, during malignant transformation there is an increase in the cell surface expression of MUC1 along with a change in the normal pattern of the expression resulting in MUC1 expression across the entire cell surface in cancer cells as opposed to the apical expression seen in normal cells. This change in expression pattern may allow anti -MUC1 ADCs to selectively target the tumor because the apical surface in normal tissues is not usually accessible to administered drugs. Next slide.
SBO-154 is a first -in-class humanized IgG1 antibody targeting what is known as the SEA domain of MUC1. The SEA domain is located in close proximity to the cell surface, at the jun ction of the extra -cellular alpha sub -unit and the partially embedded beta sub -unit. Historically, it has been easier to develop antibodies against the alpha subunit, particularly the VNTR region and therefore most of the previous clinical efforts have be en directed against this region of the protein. However, the VNTR region is subject to significant proteolytic cleavage resulting in a large amount of circulating MUC1 in the peripheral circulation that may have originated from the tumor cells. In fact, the well -known cancer antigens CA15 -3 and CA 27 -9 that are over -expressed in breast and some ovarian cancers are circulating MUC1 entities cleaved off from the tumor cells.
Large amounts of circulating alpha sub -unit MUC1 in the periphery may sequester or bind to exogenously administered therapeutic ADCs resulting in a sink effect thereby limiting the access of the antibody to the actual site of

tumor. Since the SEA domain is not subject to the same level of proteolytic cleavage as the VNTR region it is u nlikely to be subject to this sink effect in the plasma. Next slide.

SPARC has evaluated the cell surface levels of SEA domain MUC -1 in various patient -derived tumor tissues using a proprietary immunohistochemistry assay. We have seen high levels of SEA do main -specific MUC1 expression across a variety of common tumors such as adenocarcinoma of the lung, ER+ positive breast cancer and ovarian cancer. The median H -score which a measure of cell surface expression of the protein exceeded 200 out of a maximum p ossible score of 300 in most of these tumors that were predominantly Stage IV. For context an H -score greater than 100 would be considered moderate and scores exceedi

ng 150 are generally considered
high. It also appears that the level of MUC1 expression in creases with increasing stage of the disease.
Amongst these, breast and lung cancer samples had broad circumferential expression. The expression pattern in ovarian and pancreatic cancers was predominantly apical, however given that tumors generally lose the typical glandular architecture that is present in normal tissue, the extent to which this apical expression will restrict access to SBO -154 is unclear. Given the very high expression levels in ovarian cancer we plan to enroll a cohort of these patien ts in our phase 1 study which I shall discuss in the next couple of slides. Next slide.
In order to further test the sink -effect hypothesis, we have also evaluated the levels of circulating SEA domain MUC1 and compared that to the VNTR domain in plasma sa mples of patients with advanced cancers. Across all tumor types tested we find that the levels of circulating SEA domain are significantly lower than the VNTR domain from the alpha subunit indicating that SBO -154 may not suffer from the same sink effect t hat impacted the earlier generations of MUC1 targeted therapies. Next slide.
We have evaluated the activity of SBO -154 in in vitro cell lines and in vivo animal models with different levels of MUC1 SEA expression. The in vitro cytotoxicity data are depict ed in the table at the top of this slide where it’s evident that SBO -154 shows higher potency as seen by lower IC50 values in the higher expressing COLO357 and MCF7 cell lines compared to the lower expressing HT29 cell line. Similarly, when the COLO357 cel ls were xenografted in nude mice the tumor volume reduction was significantly greater upon

treatment with SBO -154 compared to vehicle control. In contrast SBO154 resulted in relatively modest reduction in tumor volume in the HT29 xenograft study indicatin g that the preclinical efficacy of SBO -154 correlates well with target antigen expression. Slide 43 please.
Preliminary non -GLP toxicology studies have been completed in cynomolgus monkeys, which is the pharmacologically relevant species for this antibody . In an exploratory 7 -week dose range finding study in 2 animals, SBO -154 was administered at doses of 1, 3 and 6 mg/Kg for 3 doses at every 3 -week dosing schedule. SBO -154 was generally well tolerated up to the highest dose of 6 mg/Kg. There was no mort ality or adverse clinical signs with no effect on body weight and food consumption. There were laboratory abnormalities of bone marrow suppression such as reduction in blood cell counts which is consistent with the known adverse event profile of MMAE. Hist opathology was also consistent with the observed lab value changes. There was a dose proportional increase in exposure of SBO -154 and the highest non -severely toxic dose or the HNSTD was established as 6 mg/Kg. A repeat dose GLP tox study is currently ong oing as part of the pre -IND requirement. This will help confirm the preliminary tox results and estimate the starting dose in our phase 1 study. Next slide, please.
Now I would like to provide a program update in terms of next steps for this molecule. We had submitted a pre -IND meeting request to the US FDA for which we received a detailed written response in late November. Their response indicates broad agreement with SPARC’s proposed IND data

package and we do not anticipate any barriers to the IND fil ing early next year.
We proposed a multi -country phase 1 dose escalation and expansion study in patients with advanced epithelial solid tumors and standard eligibility criteria for a study like this. The dose escalation portion is expected to enrol approx imately 30 unselected solid tumor patients who have failed available therapy. Once a maximum tolerated dose has been established we plan to open 3 tumor specific expansion cohorts of approximately 30 patients each in

tumors that are known to highly express MUC1 SEA. This includes ER+ breast cancer, adenocarcinoma of lung and ovarian cancer. This will be an adaptive

design with the goal of establishing early clinical proof of concept for the program. Next slide.
Finally, a quick update on the upcoming mile stones for this program. We anticipate an IND filing by the end of Q4 FY2025, followed by initiation of the Phase I study in the subsequent quarter. Next slide.
So, while that concludes my specific discussion of SBO -154, I'd like to highlight that the MMA E payload -based approach is only one of the multiple ways we can leverage MUC1 targeting. This uniquely -targeted antibody has the potential to serve as a platform for other payloads, including other chemotherapeutic agents such as cytoskeletal disruptors o r DNA -damaging agents, immuno -oncology based approaches using both immune agonists as well as checkpoint inhibitors and agents targeting angiogenesis. We have very early programs in development for some of these approaches. I will now hand it back over to Jaydeep to direct the rest of this session.
Jaydeep I ssrani: Thank you, Sandeep. This is the last slide for discussion today and it summarizes SPARC's pipeline of disclosed assets and their stage of development. We have multiple other programs under develo pment that are not disclosed and we will share details of those programs at appropriate times in future. With that, we would now open the call for question and answer session.
Moderator: Thank you very much. We will now begin the question -and-answer sessio n. We have first question from the line of Vishal M from Systematix. Please go ahead.
Vishal M: Thanks for the opportunity. So , my question is assuming we have some milestones coming up wherein we can potentially generate cash. One is a licensing deal for your CML candidate. The other is a priority review voucher that you might get issued, but in a worst -case scenario, assuming there is a delay in monetization of these opportunities , what is the cash level we have currently and how long we can continue to f und our operations?
Anil Raghavan: Vishal, thank you for the question. I am sure it is top of the mind for a lot of investors and as I said we have a significant number of opportunities here,

not just the two that you noted , that is the priority voucher an d potential licensing. We have other options like the PDP -716 launch, the exclusivity enforcement on Sezaby . So there are four or five potential areas where we can have access to short -term cash.
But if you come back to where we are in terms of operating cash flows and access to debt that we have, it will probably take us to the early part of next year. And then depending on where we reach with the short -term cash -generating opportunities, which we will have visibility by the end of this year.
We will have to take a position in terms of how we plan to resource the continuing development of these programs, which we are committed to do.
But we haven’t had a final decision on how we will go forward from there, even though we have some options, which we will di sclose once we reach that point in the first quarter of next year.
Vishal: So, you also expect phase 2 data on Vibozilimod in Psoriasis and Atopic Dermatitis . Assuming the data is positive, do you expect milestone income on a positive data base ?
Anil Ragha van: It is. Ye s, once it reaches the next level of development, it should move to a phase 3 program. It is a milestone event for us. It is listed as one of the short -term opportunities in our presentation.

Vishal: Right, so just any sense that you would have gathered on the efficacy of
Vibozilimod in Psoriasis from your early data that you would have both in
Psoriasis and Atopic Dermatitis versus the other oral options in the same
therapy? While you are targeting th ese and safety is

one of the topmost
priority, but just getting a sense on the efficacy that you would expect from
these drugs compared to the other oral options on the market?
Anil Raghavan: Vishal, we are in a blinded study at the moment. We have no visi bility in terms
of early signals of efficacy and the studies that we have done earlier were
phase 1 trials. The only human trials that we had were multiple phase 1 trials.
And I don't know whether you've been part of the previous discussion , the
translatio nal case for Vibozilimod was built on two things. One, we have seen
significant ALC reduction, that is lymphocyte reduction in circulation, and that

is the mechanistic marker and the dose that we are studying in phase 2
setting.
And if you look at literatu re, there are other products in this class which were
tested in Atopic Dermatitis and Psoriasis . And there is a certain threshold of
ALC reduction that was required for competitive activity. And we have
reached that level of ALC reduction in the phase 1 se tting. So that was the
translational case for initiating these phase 2 programs. But we do not have
any inkling of what is in store given the blinded nature of the study.
Vishal: Right, and just one final one on Sezaby. Any technical hurdles there in terms
of getting exclusivity? So , just want to understand whether it is a process or
there is also uncertainty around the process?
Anil Raghavan: It is a process in the sense, the process that FDA follows is a risk -based
assessment of how and when to enforce th e exclusivity. So , FDA usually gives
time when a new product comes to establish a robust supply chain and be in
the market before they start enforcing the exclusivity. So , it is a process and
we are in the process of engaging with the agency.
And we are ho peful as we have indicated, by the third quarter of this coming
financial year, we hope to have exclusivity.
Vishal: Right. Just one more on the in-licensed asset S CD-153. Do you, would you kind
of and would you need to pay some milestone income there when ever you
get positive data or these are completely your own assets now?
Anil Raghavan: No, we have, these are licensed very early. That was a multi -year option
agreement that we had on an early -stage preclinical asset. We have
milestones and royalties whic h are typical to those kinds of deals , which is
usually in low single digit percentages. So we have a structure which has both
milestones and royalties. But given the early stage nature, it is not like
commercial licensing.
Vishal: Right. Got it. Thank you very much.
Moderator: We'll take our next question from the line of Bino Pathiparampil from Elara
Capital. Please go ahead.

Bino Pathiparampil: Hi. Good evening. A couple of questions from my side. Do you have any
estimate of the unapproved Phenobarbital market size in the pediatric
market?
Anil Raghavan: We haven't specifically disclosed a market size. But if you look at the current
usage in the unapproved market, it's in several scores of millions of dollars.
But I don't want to give you a top -of-the-mind number. But it is publicly
available.
Jaydeep I ssrani: If I could just add , the number of units being sold in the U.S. , it is in excess of
two million injectable units, which we believe is primarily used for neonatal
population.
Bino Pathiparampil: Two m illion units. Okay, got it. Second, on the Sezaby PRV, when you say in
4Q of FY 25, there would be a n opinion of the court. Is there a trial that has
already taken place? And it will be a final verdict by the court? What exactly
would we expect?
Anil Raghav an: So, the process is that we have to have written submissions from SPARC, FDA
and HHS and back and forth on those written submissions. And now later this
month or early January, they will decide whether an oral argument is

required. And that would be the first indication. An

d then if that's required,
we expect that to complete in the first quarter, I mean, the last quarter of this
financial year.
Bino Pathiparampil: Expected to complete in the last quarter of…
Anil Raghavan: Financial year . Yep.
Bino Pathiparampil: Okay, which means it's coming in the next three months
Anil Raghavan: Right, that's our expectation and hope.
Bino Pathiparampil: Okay, understood. And last on these two ophthalmic products, PDP -716 and
SDN -037. I think I'm a little bit less updated. I was going through an earlier
presentation from early this year. These two assets were not there. So , could
you give a bit of background on how these came in? What is the market
potential, etc.?

Anil Raghavan: Yep. PDP -716 is a reformulation of brimonidine, which is a widely used
second -line drug in glaucoma. And we give a significant dosing benefit for this
product. We have disclosed clinical results in previous presentations. We met
the regulatory standard from a clinical data expectation stand point and when
we filed this with an external API source.
And that external manufacturer of the API had regulatory issues. And that led
to a complete response l etter. And before we got there, we had licensed this
product to a commercialization partner, a c ompany called Visiox , a specialty
ophthalmology company in the U.S. Visiox has gone through a transition. It's
now sold to a different company and we are working with their management
to respond to the comp lete response l etter. We replaced the API source w ith
a new source. And we also made changes to the manufacturing process
because we expect higher volumes for this.
So, we are moving to a higher volume source facility for this. So , our
expectation is that by second quarter of next financial year, we will be in a
position to file from the new facility and then it has a six -month review time.
So, we are working very closely with Ocuvex.
And the second product in this group was a steroidal reformulation. And there
also, we have very good clinical results. But with the commercialization
partner, what we agreed was we will schedule these submissions
sequentially.
We will first complete PDP -716 and then we will go with the steroidal product.
So, we will wait for the closure of PDP -716 to take a position on the re gulatory
process for that product.
Bino Pathiparampil: Understood. Thank you very much.
Moderator: As there are no further questions, I would now like to hand the conference
over to Mr. Jaydeep Is srani for closing comments. Over to you.
Jaydeep Is srani: Thank you everyone for being on call today. In case you have any additional
questions, feel free to reach out to us on the number that we have provided

on the website and we will be happy to answer your questions. Thank you
once again for being on call today .
Moderator: Thank you.
Anil Raghavan : Thank you.
Moderator: On behalf of Sun Pharma Advanced Research Company, that concludes this
conference. Thank you for joining us and you may now disconnect your lines.

Call: Jan 2026

SPARC/Sec/SE/202 5-26/50 January 13, 2026

National Stock Exchange of India Ltd.,
Exchange Plaza, 5th Floor,
Plot No. C/1, G Block,
Bandra Kurla Complex,
Bandra (East), Mumbai – 400 051.

Scrip Symbol: SPARC BSE Limited,
Market Operations Dept.
P. J. Towers,
Dalal Street,
Mumbai - 400 001.

Scrip Code: 532872

Dear Sir/Madam,

Sub: Transcript of the Investor's call for SPARC R&D Day - Updates on Prioritized Programs

This is with reference to our Investor Call which was scheduled on January 08, 2026. Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, we hereby attached the Transcript of the said Investor Call for SPARC R&D Day – Updates on Prioritized Programs and the same is also available on the website of the Company on the weblink - https://sparc.life/presentations/

This is for your information and dissemination.

Yours faithfully,

For Sun Pharma Advanced Research Company Ltd.

Kajal Damania
Company Secretary and Compliance Officer

"Sun Pharma Advanced Research Company Limited (SPARC)
R&D Day Conference Call "

January 08, 202 6

MANAGEMENT : MR. ANIL RAGHAVAN – CHIEF EXECUTIVE OFFICER
DR. MUDGAL KOTHEKAR – VICE PRESIDENT , CLINICAL
DEVELOPMENT , IMMUNOLOGY
DR. SANDEEP INAMDAR – VICE PRESIDENT , CLINICAL DEVELOPMENT ,
ONCOLOGY
MR. JAYDEEP ISSRANI – HEAD, BUSINESS DEVELOPMENT ,
CORPORATE COMMUNICATION AND INVESTOR RELATIONS

Moderator: Ladies and gentlemen, good day and welcome to the R&D Day Conference
Call hosted by Sun Pharma Advanced Research Company Limited, SPARC.
As a reminder, all participant lines will be in the listen -only mode and there

will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pre ssing star then zero on your touch -tone phone.
I now hand the conference over to Mr. Jaydeep Issrani from SPARC. Thank you and over to you Mr. Jaydeep.
Jaydeep Issrani: Thank you, Ikra. Hello everyone, I am Jaydeep Issrani and I lead Business Development and Investor Relations here at SPARC. Thank you for taking the time to join us for the R&D Day presentation. We are excited to walk you through our strategy and the progress that we made across our pipeline programs. I am joined today by our CEO, Mr. Anil Raghavan and several members of our leadership team. If you have been on our previous calls, the format today will feel familiar.
We will walk you through the presentation first and then we will open the call for questions and comments. The deck was share d earlier today, hopefully all of you have had a chance to look at it. Before we get started, just a quick reminder, some of the things we will talk about today are forward -looking and as always come with their own risks and uncertainties. The actual resul ts could differ from what we discussed on the call today.
With that, let me hand it over to Anil to start his presentation.
Anil Raghavan: Thank you, Jaydeep. Good evening, everybody. A very warm welcome to all of you to SPARC's R&D Day. Thank you for your time today. It means a lot to us. We will begin with a brief review of our portfolio strategy and operational priorities. In the initial part of our presentation today, we will revisit some of the expectations set in our last call. I will also touch u pon several early stage and preclinical programs, which we don't intend to cover in a lot of detail.

In the second half, our therapeutic leads, Mu dgal and Sandeep, will join us to share updates on two prioritized clinical programs that are now leading our portfolio. So with that, let's start with slide 4, please. If you remember, we discussed this chart last time we spoke.
Over the last 24 months or so, we had two significant data readouts which did not go the way we hoped. That’s PROSEEK Phase 2 study in PD and Vibozilimod’s Phase 2 trials. This slide summarizes the most important elements of our reset since then, built on three key pillars.
The most important one was to narrow our therapeutic focus, buckle down on oncology and immunology, and execute we ll on two promising programs that were nearing entry into clinic at that time. The first product was our M UC1 ADC in solid tumors and the second one was a topical intervention in Alopecia Areata. Since then, we have aggressively reshaped our pipeline aroun d similar themes.
The second pillar here is centered on reducing the burden of clinical risk within SPARC and explore alternative structures l ike New Cos to develop certain parts of our work. We have made progress with our collaboration with Ti ller Therapeutics, a company set up by a very accomplished UCSF team to focus on developing small molecule drug conjugates or SMDCs. And the last piece here focused on converting some of the short -term cash options we have along with a really sharp focus on optimizing our cost structure , both our fixed cost base and the operational cost supporting the execution. That's obviously super important given where we are and important to continue resourcing the aggressive build of the portfolio. We are entering the New Year with significant movement on the most important parts of this agenda, as you will see in the rest of the presentation.
Even though

we have our fair share of challenges, we're genuinely excited about the potential of our clinical and preclinical portfolio as we go into the New Year . Especially on the reshaping of the early pipeline around a few big themes which we believe have the potential to reset those sp aces in the future. But before we go there, I would like to give a little bit more color on some elements of this plan to help you understand the short -term better.

So next slide please. Mud gal and Sandeep will cover these programs in a lot more detail, b ut the key message that I want to leave you with is that we achieved all the operational milestones on both these programs ahead of schedule. For those of you who are new to SPARC, SCD -153 is a topical formulation of an it aconate analog ue developed through a collaborative development partnership with Johns Hopkins.

Itaconate, as you may know, is an endogenous immunosuppressive metabolite, which is getting a whole lot of traction now across multiple autoimmune conditions. We completed the healthy volunteer component of the Phase 1 trials and started on the Phase 1 B program in alopecia areata patients in India recently. And this is an early signal seeking study and we expect to have safety and preliminary efficacy data by the fourth quarter of this calendar y ear and transition the program to a, hopefully to a larger Phase 2 trial thereafter.

SBO-154 is our MUC1 target A DC delivering a toxin called MMAE. We filed INDs in the US, Australia, and India and got approvals from all three geographies and the Phase 1 dose escalation study in solid tumors is currently underway and recently completed the first two dose levels and no w entering cohort three with doses which are beginning to approach the p harmacologically relevant dose.

We've prioritized these two programs for resource allocation through the last year and a half and are really happy to be ending the year with visibilit y on safety and early efficacy signals soon, maybe later this calendar year for one program and early next year for the next one. This not only begins unlocking the value of these programs, but also sets up some important adjacent programs across th e pipel ine.

Let's move to Slide 6, please. I'm sure most of you are familiar with the background of this program, which has two important elements in terms of value. First is our claim for a pediatric rare diseases voucher or PRV, which is a tradeable device that can help accelerate the NDA review of a non -priority review program to a six -month review cycle. And second is the market exclusivity, which we are eligible for because of the orphan drug status.

We got some good news on the PRV matter from the district court of District of Columbia, which ruled that the agency's decision to deny the voucher earlier

was contrary to law. This judgment comes with a two -month appeal window that is now, you know, getting to that expiry of that appeal window by end of Jan. Sub ject to a possible appeal, SPARC expects to receive a voucher, which can lead to a meaningful encashment.

The PRV market has been off late on an uptake, especially after the scheme got sunset an year back. The demand remained robust while the supply situa tion is beginning to shrink, resulting in transaction values going north of $100 million. We remain hopeful of a positive outcome, which would be quite significant for SPARC.

On the other end, we continue to make the case for enforcing the exclusivity. Our Citizen Petition is still under review, and the agency indicated that they need more time for resolving the CP, given the complexity of the matter. We will continue to press for the removal of the DESI phenobarbital formulations, especially given the pre sence of these toxic alcohol -based excipients, which are potentially impacting vulnerable populations. This is indicated for neonatal seizures. We

remain hopeful.

Let's go to Slide 7, please. This is on PDP -716, which is our once -a-day formulation of Brimonidine. This product was licensed to an ophthalmic specialty pharma company called Visiox Pharma for commercialization. And Visiox was recently bought out by Ocuvex Therapeutics, and Ocuvex now holds the rights to this product. Our original NDA received a complete response letter, driven by the OAI status of the third -party API vendor.

Ocuvex recently completed the resubmission, as the API vendor's regulatory status changed. While the review process is still underway, we may not be completely out of the wo ods here, as the finished product manufacturing si te has compliance issues which require further remediation. We are working with our partners, Ocuvex Therapeutics, and are in the process of qualifying third -

party sites for the finished product manufacturi ng. We hope these steps will see us through to the market and the regulatory gate.
Now, let me give you an update on the NewCo we are setting up for progressing SCO -155. What is SCO -155? Slide 8 here, please. SCO -155 is a small -molecule drug conjugate, as I mentioned earlier, which leverages a synthetic ligand of PSMA, which is a key cell surface antigen significantly

over-expressed in prostate cancer. We have reviewed preclinical data of this asset in our last call, and that data set and subsequent experi ments that we've done indicate MMA E gets significant tumor accumulation through PSMA - mediated internalization, and causes dose -dependent tumor kill.
Earlier this year, SPARC and UCSF signed a binding Letter of Intent with Tiller Therapeutics, a company formed by our scientific collaborators at UCSF. As part of this process, Tiller obtains exclusive global rights, and SPARC, in return, is eligible for s ignificant equity of Tiller. Our teams made meaningful progress on the development plans last year, nearly completing the IND - enabling tox studies and making substantial strides towards manufacturing clinical -grade material.
Tiller has also completed the p re-IND consultation with the FDA Oncology Division, getting some clarity on regulatory expectations. They've raised pre - seed capital in last year, 2025, and now in the process of raising priced external seed capital for funding the early -stage clinical pro gram. We're also very encouraged by how this whole NewCo structure has panned out for advancing a late preclinical program into clinic.
It can also be an interesting preposition for some late -stage programs, maybe even for vodobatinib in CML. So let's go t o the next slide to talk a little bit more about vodobatinib.
Here, you know, certainly our diversification into Parkinson's disease for this compound took a toll. Not just the spend, which was quite significant, but also the opportunity cost in terms of lost time and pipeline position.
We tried to get back to CML as soon as PROSEEK's full results came in. But as we discussed earlier, we've negotiated a protocol for a registration program in second -line CML with FDA, but the pathway remains challenging. T he CML landscape, as you can see in this chart, has been changing quite a bit with increased generalization of TKIs and the progression of second -generation and now third -generation set of products into earlier lines of treatment.
The standard of care and pipeline has evolved with asciminib and other allosteric inhibitors of BCR -ABL coming in. Asciminib is on the way to becoming a significant option for CML patients across the spectrum. While the development pipeline has expanded with new investigational p roducts, all

having exciting early clinical data, like less than 100 patients, new products, including ours, need validation through larger pivotal trials to establish their true position in the pipeline and the therapeutic continuum.
At the same time, the licensing interest fr

om large pharmaceutical companies
has been limited, largely on account of perceived market -size limitations, even with Asciminib 's commercial success and the rising industry interest in the next-generation TKI com panies. So that is proving to be a moderating force on the outlook going forward.
This complex dynamic puts us, as in SPARC, in a bind as progress now requires resourcing a costlier late -stage study with an active comparator, like several others in the fie ld are planning to do. We've been carefully gauging the implications of this emerging situation, especially with the best use of capital considerations in mind, given that we have several other pipeline opportunities.
We may prefer finding a way to work w ith potential partners and investors using alternative structures to continue pursuing marketing approval for this product, which we believe for sure can help CML patients significantly. We hope to take a final decision on the future of this program fairly soon. So let me leave you with the following thoughts from this segment.

SPARC has made significant effort to reposition its program around a pair of exciting assets, a differentiated antibody drug conjugate and novel immunology pathway in the derma auto immune segment. As you will see later today, these projects have important catalysts coming up in the short term.
The whole SCO -155 experiment is also maturing and gives us confidence that such partnerships can be an alternative path for some of our asset s. And a positive outcome on the PRV matter can be transformational and can give us room to explore our pipeline more aggressively, especially after the cost structure optimization that we achieved through the last year or so.
I'll touch upon some of the specifics of our cost reduction and funding situation in a bit. Before I do that, let's take a look at the preclinical side of our portfolio.
Over the last couple of years, we've been trying to move away from a broader, somewhat disparate set of projects, all chosen for individual attractiveness as a scientific hypothesis. Synergy was not a consideration that drove those

choices. But two things changed in the recent past. We exited neurodegeneration almost completely. That leaves us with oncology primarily and certain pockets of immunology. Secondly, we deliberately tried to bring in more thematic cohesion in our approach to portfolio building.
We are now focused on three tightly defined themes. We're interested in targeted delivery to tumors, either media ted by antibodies, internalized through cancer -specific antigens, or using synthetic small molecule ligands similarly.
We're interested in different modalities of payload delivery using this approach. And a substantial chunk of our effort now, the preclini cal effort now, is focused on that.
The second thread here is this broad area of synthetic lethality. Exploiting vulnerabilities like BRCA, loss of function mutations, or induced DNA damage, like in the case of radiation and chemotherapy. It's emerging as an important area of cancer research, and especially after the success of agents like PARP inhibitors. We are interested in multiple targets in this area.
And the last piece here, we see a clear opportunity in certain dermatology autoimmune disorders, wh ere the current standards of care have significant liabilities , either long -term safety concerns, or in some cases, meddling efficacy. We are focused on exploring novel pathways and potentially rational combinations as topical alternatives to the current o ral and I would say somewhat challenging set of options that these patients have.
We plan to maintain this focus in the medium term so that we can benefit from the mechanistic and operational synergies within a tightly defined field. I want to add a littl e bit more color to these three broad objectives to help you understand our specific interest in each

of these areas.
Next slide, please. Our overarching strategy is to develop modular platforms with plug -and-play options. The targeted oncology space offe rs best examples of this approach. We have at least six platform concepts, which can all expand into multiproduct baskets.
The MUC1 SEA targeting classical ADCs with payloads like MMAE or exatecan is probably the most advanced, the lowest hanging fruit, f ollowed by a range of options covering multiple payloads and structures like immune - stimulating agents such as STING agonists, T -cell engagers, bispecific ADCs,

SMDCs like SCO -155, which we just spoke about, and antibody -coated nanoparticles, which carry s iRNA that can knock down target proteins. We have programs at various levels of maturity in each of these six areas.
The next thread, as I mentioned earlier, is synthetic lethality, where we are going after, one, a key driver of PARP resistance, and two, an important player in the repair of DNA damage, which is a significant liability for most cancers.
And we spoke about the last block, which is our focus on dermatology autoimmune disorders.
It's primarily driven by CD8 positive T -cells, where the current standard of care is oral or topical JAK inhibitors. And we are looking to explore new

biological insights to create safer topical alternatives for better efficacy, either for better efficacy through mechanistically independent combina tions or just improving the safety profile.

Let me try to give you a few examples on each of these elements, starting with slide 13, with targeted oncology. Smart oncology therapeutics has had significant successes of late. Products like Enhertu and Trodelvy help ed rediscover the ADC field, which is turning out to be one of the richest pipeline segments in oncology.

Novartis' PLUVICTO added RLTs, as in radioligand therapies, into the mix, leveraging synthetic small molecule ligands, like what we are doing in SCO -155. This momentum is built on the back of superior clinical data and driven by the bigger therapeutic window, enabling that superior clinical data. We believe that this wave is just the beginning, with significant activity expected from novel antigens, bispecific targeting, and novel payload classes and their combinations in a bifunctional manner.

Payload diversity is going to be the key, and we are actively testing several novel pr opositions. On the multispecifics, T -cell engage rs are having a big impact on Haem -Onc. Their progress in solid tumors is somewhat constrained, largely because of cytokine release issues. Next generation in this class will look to include features that will enable tumor -specific T -cell activation and reduce the risk of CRS more.

We believe this will bring a segment of solid tumors into play, especially the ones showing activity against classical immuno -oncology products. We are

seeing evidence of this already in the development pipelines, where the ADCs and bispecif ic antibodies have now become the mainstay of many big pharma portfolios.

We are not very different in that regard. Let me walk you through a few examples.

As I mentioned earlier, ADCs targeting the MUC1 -SEA domain containing the alpha -beta combinatorial epitopes are the most proximal platform opportunity for SPARC.

Once the targeting hypothesis is validated in the ongoing trial, we have the flexibility to plug in several payload options using the same MUC1 series antibodies. On the chart here, we have t wo such examples. On the left is our MUC1 MMAE ADC with, as you can see, significant tumor activity in colo -357 pancreatic cancer model.

And on the right, another ADC delivering a TOPO 1 payload in the same model demonstrates dose -dependent tumor regressio n. It is important to point out the potential of this approach even in moderately expressing populations. With an H-Score of 130, this is the line w

ith moderate expression of MUC1.

Later in the presentation, Sandeep will discuss the actual expression leve l in patients' tissues, and that's higher. We are looking forward to early signs from current studies on the MMAE ADC, as I indicated earlier, and we will proceed with the TOPO1 variant only after the MUC1 hypothesis is validated. We believe the diversity of expression profiles will allow us to position multiple payload classes with distinctiveness.

In the next slide, we have an example from another payload, an immune -stimulating agent. Slide 15. This is a whole new class of ADCs coming into the clinical p ipeline now, where an intercellular immune -stimulating protein is activated by the payload to develop an immune response.

In parallel, the Fc component of these ADCs binds with Fc receptors of players in the innate immune system, such as macrophages, acti vating them. This multipronged approach provides an opportunity to convert cold tumors , hot and enabling combinations with classic IO agents like checkpoint blockers.

With several active players experimenting with this approach, including ADC class leaders like Daiichi Sankyo, providing early validation for this approach. Next slide, has more specifics on our plan. We are using a proprietary STING agonist series as the immune -stimulating payload in our program. As many of you know, STING has been an active area of interest as a cancer target for some time, but the field couldn't make much progress because of the safety issues.

STING agonism, as you can imagine, can unleash broad immune response, which can be problematic. That's where ADCs come in handy thr ough the specificity of antigen -mediated internalization and its ability to bridge macrophages into tumor microenvironment. We have excellent preclinical proof of concept for this approach, as you can see in the chart here.

In this experiment, we have adm inistered three infusions of three different doses, 0.3, 1 and 3 mg/kg IV with the COL O-357 tumor bearing mice. These are T -cell deficient nude mice with active innate immune system, and we see dose-dependent regression, including several complete responses, even in the middle dose. We also tested mouse variants of these ADCs in fully immune -competent animals , again eliciting exciting tumor response without any significant safety issue.

We are at the moment finishing the preclinical efficacy battery, and we are exploring ways to bring them into clinic. Before I move on from this theme, I want to briefly talk about T -cell engage rs and our plans in that area in Slide 17. Let me give a bit more background.

T-cell engage rs as a class is probably more mature and validated than immune -stimulating ADCs, especially in hematological malignancies like lymphomas and mye lomas. The chart on the right side gives a simplistic illustration of the way T -cell engage rs are engineered and structures of three generations of T -cell engage rs. Essentially, a TCE is a bispecific, combining a tumor -specific antibody with a CD3 binder o r something similar.

First-generation products using this concept, like Amgen's BiTE platform, had two ScFvs joined together by a flexible linker. While they proved efficacious, they had circulating half -life issues, which led to the second -generation TCE , which is a fuller bispecific antibody structure with a functional Fc. That did

improve the PK properties, but aggravated the off -target cytokine release because of the Fc receptor binding to T -cells and bridging outside of the tumor. The field tried to address this issue through mutated or partially incompetent Fcs, which partially took care of these issues. But we feel there is significant opportunity in solid tumors, provided the field comes up with constructs with sufficient safety margin to dose meaningfully higher. That's what SPARC is attempting to do.

I

n our first shot at this concept, we substituted the Fc component with human serum albumin, which lent similar half -life extension without the risk of Fc receptor binding and T -cell complexation . We have a promising proof of concept for this construct, with an illustrative asset built with MUC1 antibody as the targeting mAb and a CD3 binder and a full -length human serum albumin. We call this MUC1 -SEA Albufusion TCE .

Let's go to the next slide to take a look at the data. See the waterfall plot on the right side. In this experiment, using our workhorse COLO -357 pancreatic line, we have tested the first construct using admixed PBMCs , which has a mix of immune cells and a nonspecific control TCE bind ing to, in this case, HepB . The data cannot be clearer. Almost all animals responded, except one. 60% of animals had complete response. Encouraged by this data set, we are working on a second -generation construct, which will further improve the specificit y and mitigate the CRS risk. We are making it trispecific now by adding a CD8 binder for preferential activation of the cytotoxic CD8 T -cells and reducing the CD4 helpers. CD4s are primarily the drivers of CRS, and we are confident we can proceed to IND -enabling studies next financial year with a differentiated asset using this design.

Our intent in this part of the presentation was to give you a glimpse into our focus on smart oncotherapeutics. I didn't talk about everything we have in the basket. A coupl e of them maybe deserve mention before we move on. We consider bispecific ADCs as a natural expansion of the ADC space. We are working on a couple of novel combinations using Exatecan as the payload. Further down the line, antibody mediated delivery of nanoparticles containing siRNA as the payload is another area of active pursuit for us. So, SPARC, in the last couple of years, demonstrated the engineering capability to modularly

put together these kin ds of differentiated structures. This effort is going to be a major driver for our portfolio expansion going into the future.

Now, let's move to our second pillar, which is synthetic lethality in Slide Number 20. Because of consistent DNA damage and genom ic instability, cancer cells are, dependent on DNA damage repair pathways. This is especially true in tumors which are homologous recombination deficient, or HR deficient. HR is a crucial DNA process which is extremely critical to repair, and its deficienc y creates an exploitable vulnerability which was successfully exploited by agents like PARP1 inhibitors in the BRCA context.

We are working on two opportunities in this space. One is a sensitization agent which shuts down a key player in DNA repair. Such an agent can be an excellent sensitizer to radiation and also to certain chemotherapeutic agents. The second idea originated from the field of PARP inhibitor resistance. Our target of interest has emerged as a key compensatory mechanism in PARP1 resistanc e, and we are looking to leverage this as a PARP i alternative or as a PARP i sensitizer. In the next couple of slides, I want to very briefly comment on the early proof of concept data coming out of these programs.

21. As I said earlier, in the first progr am in this theme , we got what we consider as a master regulator of cellular damage repair, which helps cancer cells maintain genomic stability. This has been a target of interest for some time with multiple large pharmaceutical companies in the race.

The first generation programs failed to take off because of PK limitations. Adequate brain penetration was also an issue with GBM as a key opportunity for this approach. SPARC has developed now a series of brain penetrant high - potency inhibitors with good PK p rofile and has developed data along with multiple chemotherapeutic agents as shown in this chart.

We also have data with radiation. In all these experiments, SPARC c

ompounds

outperformed the leading second generation compound, which is going through early stage clinical trials now. We are in the process of finishing the preclinical efficacy trials and looking forward to transitioning the lead compound to clinic before the turn of the next financial year.

Now, on the second concept, let's go to Slide 22. T his program targets a chromatin remodel ler, which is a key player recruited into DNA damage repair process and significantly over -expressed in multiple tumors as cancer cells fight for stability. We have developed extensive genetic and pharmacological validation for this target internally within SPARC as this is a potentially first - in-class opportunity.

This protein has also emerged as an important compensatory mechanism as cancer cells become resistant to PARP inhibitors. As you can see in the chart below, our tool compounds exhibit excellent anti -tumor activity standalone and in combination with another PARP inhibitor, Olaparib, in this in -vivo model in a dose -dependent manner.

This early data is really releva nt as it validates our chemistry and opens potential path to a nomination by mid -next year. We are looking to accelerate the IND process as soon as we have an optimized lead compound given the first-in-class window on this area.

That takes me to the last set of this three -leg stool, derma autoimmune disorders. Let's go to Slide 24 to briefly discuss our priorities in this area. Diseases like Alopecia Areata and Vitiligo has been managed with a fairly dormant standard of care for a long time, primarily cort icosteroids and calcineurin inhibitors, which left significant room for growth, both in terms of efficacy and safety, in spite of the recent progress with JAK s.

As I mentioned earlier, this is an area where SPARC sees an opportunity in the short term. We made our first move with SCD-153 already. As we move forward, we are building other ideas which can be alternative standalone

options as well as synergistic combination partners for SCD -153.
This can take two paths, either establishing SCD-153 as a safe c hronic maintenance option after a combination induction or developing lower -dose combination regimen, which can push up the efficacy without some of the concerning safety risks of the current standard of care. At this point, we are yet to disclose these al ternative mechanisms we are pursuing, but hope to do that in the near future.
With that, let me conclude with a summary of our portfolio and short -term catalyst. I also want to make a few comments on our cash burn before I

transition. Let's go to Slide Number 25 for a view of the development pipeline. I've already spoken about most of these, so let me not recite this again.
We have not included a few things which are pre -POC, but I've alluded to some of them also. In terms of things to watch out for in terms of value creation, two clinical programs are really important. We expect to add another vitiligo trial for 153 and bring the radio / chemosensitizer to the clinic.
We also hope to see Tiller making progress with SCO -155 to an IND, hopefully in the short term. We will also be busy on the biologics side of the portfolio with additional assets expected to graduate to clinic in the short to medium term.
We will stay in these three broad thematic lanes I spoke to you about fo r further building on the story. Slide 26 has a snapshot of what to expect in the calendar year. Just a summary of expectations during the course of my comments today. Our overriding priority will be executing as well as we can to ensure that these milesto nes are delivered.
Let's go to Slide 27. We've been trying to optimize our cost structure very aggressively after PROSEEK without compromising our ability to deliver our critical priorities or diluting potentially differentiating competencies built over the years.

We certainly made progress on this front as you can see from this chart. The primary driver of our fixed cost is obviously manpower cost. During FY '2 4, we had 400 plus people across three different geographies. Next year, our headcount is expected to be around 250 -ish. And the location distribut ion is also important.
We will reduce US footprint significantly, which is primarily built to run late - stage clinical trials. We're also in the process of reducing the number of lab centers from four to two. To enable this transition, we've tried to align our projected workload with the scale and competencies required and have taken a decision to outsource certain competencies which require maintenance of costly infrastructure and capabilities inside.
These changes as you can see has resulted in an annual fixed cost savings of about $10 million, which is super significant relative to a 50 million -ish annual

spend. Let's go to the slide -- our next slide. We also have reined in our operational spend considerably. From 31 million last year, we are projected to go down to 29 million this year, even though we plan to scale our clinical program significantly.
Our overall ops cost is expected to only increase marginally. This is the outcome of several interventions that we focused on in the last couple of years now. But let me highlight just one thing. We strategically started increasing the India component of our clinical development effort.
The local clinical research environment has improved quite a bit in the recent past and we believe India provides an exce llent opportunity to conduct early signal -seeking studies at a very low cost of failure. And that can be an important differentiating driver of value for companies like SPARC.
But before I transition, I want to ensure you see this on balance. While there is significant promise, ours is an early -stage portfolio with key assets yet to have even early clinical proof of concept. So investors who want to review this portfolio need to approach this with probability -adjusted view to valuation.

Plus we've been fu nding our early development using promoter -backed debt, which is cumulatively 45 million -plus now as of Q2 FY '2 6. We need now a significant additional resources to deliver the outcomes discussed in this presentation and we are in the process of finalizing a funding plan to extend the cash window to FY '2 8 and hope to complete that process in the first half of this calendar year. Thanks again for listening in. I'll see you at the other end of the call for Q&A. I'm now transitioning the call to my colleague, Dr. Mudgal for the next leg of this presentation, which is an overview of SCD -153.

Mudgal Kothekar: Thank you, Anil. I'll present an update on the program SCD -153, which is a first-in-class agent currently being evaluated for alopecia areata. Next slide, please. SCD -153 is a novel topical agent that has the potential to be an alternative to JAK inhibitors in the treatment of several autoimmune dermatological disorders.

SPARC licensed the IP rights for this compound from JHU and IOCB. SCD -153 employs a novel approach of using an itaconate derivative to address the immun e pathogenesis. The topical delivery of this compound provides a dual advantage of targeted delivery at the site of action and minimizing the risk of

systemic adverse events that are the major limitations of the currently approved therapies.

We have completed several preclinical studies in which SCD -153 demonstrated hair growth in the animal model of alopecia areata. The preclinical PK studies have also been completed and the tox icology and safety package is also completed. Our Phase 1 first -in-human study showed that SCD -153 was well -tolerated up to the highest dose that was evaluated in the study. The Phase 1B clinical trial in alopecia areata has been initiated and an

interim
readout from this study is expected in the Q4 of the calendar year 2026.

Next slide, please. This slide shows the overall design of the Phase 1B clinical trial that is currently ongoing in alopecia areata. This study is enrolling male and female alopecia a reata patients with a SALT score between 25 and 90, which corresponds to the loss of hair from 25% to 90% of the scalp. This study will enroll a total of 70 patients, 15 at each dose level.

So, these four are the four increasing dose levels. So, 15 patien ts will be enrolled at each dose level and 10 additional patients at the top dose level. In each cohort at each dose level, patients will be randomly assigned in a 4:1 ratio to active or vehicle in sequential cohorts of four dose strengths of SCD -153. Initially, in each cohort, patients will receive a single application of the assigned treatment on day 1, followed by safety and tolerability assessments up to day 8. Once the single dose is tolerated, patients will continue to receive once daily treatment of the drug. Patients initially randomized to SCD -153 will continue to receive the same drug for a total of 24 weeks and patients who are initially randomized to vehicle will be switched to the active drug for the next 12 weeks.

Patients will be enrolled in subsequent higher dose levels after the evaluation of safety data up to day 22 from the previous dose code by an independent DSMB . SCD -153 was applied as a topical solution in the first cohort and we have made a change and optimized the formulation to foam formulation, which is being evaluated cohort 2 onwards.

The study will assess the safety as well as efficacy of increasing doses of the drug and we will also evaluate Pharmacokinetics, both the systemic PK as well

as the local concentrations at the site of action and we also will be evaluating biomarkers in the study by means of transcriptomics and proteomics.

Next slide, please. This is an update on the current status of this study. So, while the cohort 1 was ongoing, we amended the protocol to change the formulation from solution to fo am and also to revise th e cutoff for the extent of hair loss to be eligible to participate in the study.

Additionally, in the initial approval, the CDSCO had put up a condition to submit the cohort 1 safety data to the CDSCO and initiate enrollment in cohort

2 only after a revie w and approval from the DCGI to initiate the cohort 1. This would have significantly impacted the overall study timeline.
Therefore, along with the protocol amendment, we requested a waiver for this condition and proposed to allow opening up of this subseq uent cohort based on the review by an independent DSMB that was already a part of the study.
The protocol amendment as well as the request for the waiver was approved by the DCGI, saving us a lot of time in the study. The study has completed enrollment of all required 15 patients in the cohort 1. Nine of these 15 patients have already completed 12 weeks of treatment with no safety concerns.
The DSMB has reviewed the safety data from the cohort 1 and recommended opening up of the second cohort as no safety issues were found in the first cohort. Patients are currently being enrolled in the second cohort.
Moving on to the next slide. In terms of the key milestones, the enrollment in all cohorts of the study is targeted to be completed in the quarter 3 of the year 2026. Top line results are expected in Q4, and we plan to initiate a global Phase 2 clinical study in the quarter 2 of the calendar year 2027.
Next slide, please. We are also exploring this compound in an additional indication and another dermatologic al condition that shares the immune pathogenesis with alopecia areata. The image on the left illustrates the underlying mechanism involved in alopecia areata, and the image in the middle shows the mechanism involved in vitiligo.
As shown in th

ese two carto ons, CD8 + T cells are the implicated cell type in both these conditions. As shown in the image on the left, CD8 T cells infiltrate around the base of the hair follicle in the dermis in alopecia areata. And as

shown in the middle figure, the CD8 + T cells lo calize near the melanocytes in the epidermis in vitiligo.
In both the diseases, the CD8+ T cells release the cytokines such as interferon gamma and their cytolytic granules. In alopecia areata, they damage the hair follicles, causing loss of hair. In vitiligo, they damage the melanocytes that produce melanin, resulting in depigmentation of the skin.
As shown in the image on the right side, both hair follicle stem cells and melanocyte stem cells are preserved in the bulge of the hair follicle in these conditions, creating a potential for clinical benefit in both these diseases.
Next slide, please. We have conducted in -vitro studies to explore the effect of SCD -153 on the chemokines and cytokines that are involved in the pathogenesis of vitiligo. Studies were conducted in human melanocytes, human keratinocytes, and peripheral blood mononuclear cells, that is, PBMCs, which are the cell type basically involved in the pathogenesis of vitiligo.
In the top panel, human melanocytes were stimulated using interfer on gamma, and the expression of CXCL9, CXCL10, and CXCL11 genes were assessed.
As shown in the figure, the blue bar shows normal cells, that is, those that were not stimulated with interferon gamma. You will see that this bar is very low, and there is no i ncrease in the expression of these genes.
The red bar shows the melanocytes that were stimulated and incubated with vehicle. So, these melanocytes showed significant overexpression of these chemokines. The bars after the red bar show the melanocytes that w ere stimulated with interferon gamma in the presence of increasing concentrations of SCD -153. So, it is evident from this figure that the drug inhibited the overexpression of these genes for CXCL9, 10, and 11 in a dose -dependent manner.
Now, as shown in th e bottom panel, similar results were seen in the human keratinocytes that were stimulated with interferon gamma. The figure on the lower right side shows inhibition of interferon gamma production from anti - CD3, CD28 -stimulated human PBMCs, that are the key circulating immune cells.

Next slide, please. After promising results in the in -vitro studies, we have initiated evaluation of SCD -153 in the animal model of vitiligo. This model of

vitiligo was developed based on finding that a noticeable proportion of melanoma patients developed vitiligo after receiving checkpoint inhibitor therapy.
This is believed to be a result of immune response mounted by endogenous autoreactive CD8 T -cells against the melanocyte antigens, along with a decrease in the regulatory T -cells, resulting in an unrestricted immune attack on the melanocytes. The melanoma cells and the melanocytes carry the similar antigen, that is gp100.
A similar milieu is created in this animal model that we are using for evaluation in vitili go. In this model, melanoma cells are inoculated intradermally in the skin of the mice. After this, anti -CD4 antibody is injected to deplete the regulatory T -cells to allow for an unrestricted immune attack by the CD8 T -cells on the melanocyte antigens.
The tumor is then resected to ensure survival of the animal. After around a month of injection of the melanoma cells, the endogenous autoreactive CD8 T-cells infiltrate the epidermis and start mounting an attack on the melanocytes causing their destruction. The visible effect in the form of depigmentation of the tail skin of the mice is evident around Day 60.
The drug is expected to inhibit the depigmentation caused by the CD8 cells by its anti -inflammatory effect, as it is expected to inhibit the migration o f CD8 T-cells to the site of

immune response. This evaluation of SCD -153 in the animal model of vitiligo is ongoing. We intend to plan and initiate a clinical study in vitiligo once we have the results from this animal model. So, this concludes the update on the SCD -153 program.
Now, I would like to hand over to Dr. Sandeep Inamdar for the next update.
Sandeep Inamdar: Sure. Thanks, Mudgal. So, we will now move on to our oncology clinical asset, that's SBO -154, which is an anti -MUC1 antibody drug conjugate with the payload of MMAE.
Moving to the next slide. So, SBO -- so MUC1 is a glycoprotein that is widely expressed on cell surface of epithelial cells, including the epithelial tumors.

Expression of MUC1 in epithelial tumors differs from that in normal epithelium, making it a suitable antigen for targeting via an ADC.
While in normal tissues, the protein is expressed on only on the apical surface of the glandular epithelium , expression of MUC1 on tumor cells is more widespread across the entire cell surfac e, what we call as circumferential expression pattern.
The MUC1 protein consists of an extracellular glycosylated alpha chain, along with a cell membrane bound beta chain that you can see on the image on the right. Previous efforts at targeting MUC1 focuse d on the VNTR region of the alpha chain.
However, it is well known that the VNTR region undergoes proteolytic cleavage, resulting in large quantities of circulating VNTR subunits in the peripheral circulation. This may serve as a peripheral sink for ADCs targeting the alpha subunit, thereby limiting access of the therapeutic to the tumor site.
SPARC's approach is unique in that our binding epitope of SBO -154 targets the alpha beta junctions, which is less likely to be impacted by proteolytic cleavage, ther eby reducing the potential sink effect, allowing better tumor targeting. We have evaluated plasma samples of cancer patients with different solid tumors and have been able to demonstrate significantly lower levels of the alpha beta subunit in the periphera l circulation compared to the VNTR region, which further strengthens our hypothesis.
Go to the next slide. So in order to understand the expression pattern of our specific MUC1 epitope on various tumors, we evaluated patient tumor samples for MUC1 -SEA doma in with a proprietary immunohistochemistry assay. This assay is semi -quantitative in the form of H -Score for each sample. These H -Score can range from 0 to 300. Scores above 150 are typically considered as high to very high expression of the antigen.
As yo u can see, the H -Score in more than half the patients with late -stage ER -positive breast cancer and adenocarcinoma of the lung, which are two highly prevalent tumors, exceed the threshold of 150, particularly as it pertains to the circumferential expressio n pattern, which is relevant for ADC targeting.

In addition, ovarian and pancreatic cancers also have exceedingly high expression, although that expression tends to skew more towards apical

expression. Based on these data, we have designed our Phase 1 study to evaluate these tumors for evidence of efficacy.
Next slide. So this slide shows a representative dataset for the in vivo efficacy of SBO -154 in CDX mouse models. This particular experiment is done using the COLO -357 cell line, which is a pancreatic ca ncer model. The image on the left is with the same cell expressing MUC1 -SEA at a moderate level of 130 H -Score. SBO154, as you can see, is significantly superior in terms of tumor reduction compared to vehicle control, as well as MMAE conjugated to anti-HBV, which is an untargeted control. However, this activity is significantly improved in the model on the right, where the H -Score is higher than 200, where all eight animals saw a deep response in the form of tumor regression. Similar results were al so obtained with other models of breast and ovarian cance

r. This suggests that the activity of SBO -154 is closely linked to the level of MUC1 expression on the tumor cell. The H -Score of 200 is closer to the level of expression in human tumor samples that we observed. That's the data that I shared on the previous slide.
Moving to the next slide for the Phase 1 study. Coming on to the Phase 1 study design, as Anil had alluded to earlier, we initiated our Phase 1 study in July - August of 2025 in the US, in Aus tralia, and in India. This is a study design of the adaptive dose escalation and expansion trial. The Phase 1a dose escalation is currently ongoing, where we enroll all solid tumors, irrespective of their MUC1 expression.
We are collecting the tumor samples of these patients to retrospectively test for MUC1 expression and correlate with clinical activity. Once we are near completion of the dose escalation portion, we plan to enroll a backfill cohort of approximately 25 pati ents with high MUC1 expression.
The goal of the backfill cohort will be to get additional safety and PK at specific dose levels, but also to get early evidence of clinical activity, since the patients will all be pre -selected for high levels of antigen exp ression. Once we complete the part 1a, we expect to initiate 1b in three specific tumor types. These are expansion cohorts in estrogen receptor positive breast cancer, adenocarcinoma of the lung, and ovarian cancer. And the choice of these tumors is based on the high level of MUC1 expression known to be present in these patients.

Again, patients will be pre -selected for high MUC1 expression in this portion of the study. The study design will be an adaptive Simon's 2 stage, where we will initially enroll abo ut 13 to 15 patients in the first stage. Once we observe a sufficient level of threshold clinical activity, we will enroll an additional 15 or so patients in the second stage for a total of approximately 30 patients in each cohort. This will provide defini tive evidence of clinical activity that will enable us to proceed to the next phase of development.
Next slide. A quick update on the study status. The study is actively enrolling at 11 sites across multiple geographies. The first two dose cohorts have completed enrollment with no unexpected safety findings, allowing us to recently initiate the third dose escalation cohort. We expect to complete dosing to the highest dose cohort of 2.4 milligrams per kilogram by the end of the third quarter of this year.
This brings me to my final slide, which is highlighting the upcoming milestones, summarizing the upcoming milestones for this trial. We expect, as I mentioned, to expect to have the maximum tolerated dose, or the MTD, identified by the end of the third qua rter of 2026, allowing us to proceed with cohort expansion by the end of the calendar year, and a potential early clinical proof of concept by the second half of 2027.
I'll now hand it back to Anil for his concluding remarks before we take questions.
Anil Raghavan: Thank you, Sandeep. I'm not planning to go through this slide. This is just a

summary of the key drivers of our value that we have discussed through the
course of this presentation. Thank you again for joining in, and we're really
looking forward to the Q&A.
Moderator: Thank you very much. We will now begin the question -and-answer session.
As there are no questions from the participants, I'll now hand the conference
over to Mr. Jaydeep for closing comments. Thank you, and over to you.
Jaydeep Issr ani: Thank you, Ikra. Thank you, everyone, for joining the call today. If you may
have any questions, you can reach out to us on the details provided on our
website, and we'll answer your questions. Thank you again, and have a good
day. Bye -bye.

Moderator: Thank you. On behalf of SPARC, that concludes this conference. Thank you
all for joining us today. You may now di

sconnect you r lines.